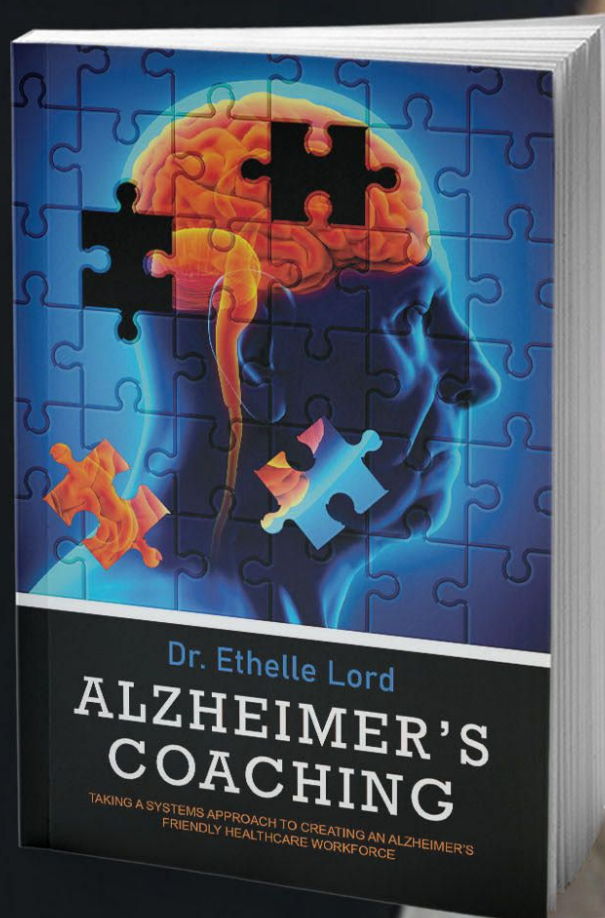


Dr. Ethelle Lord

ALZHEIMER'S COACHING



TAKING A SYSTEMS APPROACH TO CREATING AN ALZHEIMER'S
FRIENDLY HEALTHCARE WORKFORCE

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Legaia Books™
555 Fayetteville Street
Suite 201 Raleigh, NC 27601
www.legaiabooks.com
info@legaiabooks.com
Phone: (704) 216-4194

*Alzheimer's Coaching: Taking a Systems Approach to Creating an Alzheimer's
Friendly Healthcare Workforce*
Dr. Ethelle Lord

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Everything seems to be taken away with Alzheimer; the memories, the ability to speak and choose, respond—because the voice is silenced, but the heart and feelings are very much alive. Between stimulus and response, there is a large gap. Space with deafening silence and fear, waiting to be heard and valued again. In this silent gap remains a human being full of love and dreams.

Even after containing Alzheimer, as you will find out in this 2nd edition, there may still be a gap between a stimulus and a response.

—Éthelle G. Lord, M.Ed., DM, President
International Caregivers Association,
LLC Remembering 4 You, LLC
GSHS Practitioner



My Dearest Larry,

When you forget your words, I remember them.

When you cry, I cry your pain with you.

When you tremble in fear, I pray for you.

Whether in joy or sadness, I love you.

I remember our memories...

Your life's work, your dreams.

And when you drift away, I await your return.

A hand movement, eye contact, a word uttered.

Your hugs and kisses still move me.

I believe in our love and purpose together.

Nobel is your life purpose.

I understand Alzheimer's is a reality, not a destiny.

How joyful it is to see you reach out for me.

You allow me to continue to love you deeply.

You are the purest LOVE I'll ever know.

Forever yours,

Éthelle



To the students and healthcare workforce, professionals, and nonprofessionals, who provide care for individuals living with Alzheimer's and other dementias. Your devotion to your work is total, and your desire to find the best possible techniques, approaches, and understanding of what is required to perform this most demanding work is admirable. You work day after day unconditionally to provide both care and safety to the millions who require your services. I honor you.

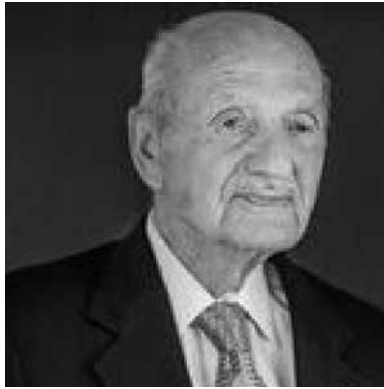
To my husband, Major Larry S. Potter, USAF Retired (twenty-one years of service) and a university professor in the field of small business education (fifteen years). After about a decade, I realized his diagnosis of Alzheimer's was much more than that. His dementia was contained in 2015 in a study with six other participants. I owe him an enormous debt and gratitude for introducing me to the connection between spirituality and dementia. He is one of millions of protected souls. Like all the others who are living with a diagnosis of Alzheimer's, he is protected from the evils of this world and

the common day-to-day challenges of living in a world of chaos, pain, and much sadness.

I also dedicate this book to my three grandsons whom I have not seen for several years—Tanyn, Quinn, and Leo—who now live in New Mexico. My wish is that I reunite with them when the time is right. To me, they represent the future and hope. I love them very much.

A special dedication of this body of work was dedicated to my loving mother-in-law, Pearl Potter. She has been grieving for her firstborn, Larry, for many years. She lost her husband, my father-in-law, in 2008 to vascular dementia. A mother should never have to watch her child suffer with dementia. (See “How to Tell Another Family Member about the Progression” in chapter 16.) Since the first edition, my mother-in-law died on February 8, 2017 at the age of 94.

This second edition is dedicated to the first ICA Chairman of the Board of Directors of the International Caregivers Association (ICA), Norman Duncan who died August 16, 2019 at the age of 100. He was actively working as the ICA Chair to the time of his death.



Norman Duncan (1919-2019)
ICA Chairman of the Board of Directors (2009-2019)

ACKNOWLEDGMENTS

Alzheimer's and Dementia. Coaching is based on my personal experience of a need to create a coaching training program to help everyone with dementia care. It is also based on a lot of research and several years of teaching the program to individuals and life coaches from all over the world.

This book would not have been possible without the support and encouragement of several individuals. First of all, I would like to acknowledge and thank A. Michael Bloom, author of *The Accidental Caregiver's Survival Guide: Your Roadmap to Caregiving Without Regret*, who served as my mentor and coach during the entire process. We spent many hours on the phone, e-mailing, discussing, and sharing.

I am especially grateful to Dr. Eilon Caspi, gerontologist and dementia behavior specialist. Dr. Caspi was the first to review my book and has always contributed ideas, support, and much needed encouragement for the field of dementia care.

I am also grateful to all my students in the International Caregivers Association (ICA) Alzheimer's/dementia coach certificate program who provided much insight into what needed to be included in this book while in their semester-

long study programs: Peter Gooley of Australia; Susan Manion, Debra Desrosiers, and Lee Dastur of the United States; and Carole Murphy and Jessica Gozlan of Canada.

Finally, I am grateful for Alzheimer's coming into our lives. At first, it almost drove me crazy requiring much time, research, and teaching other care providers on my part. But I found myself teaching family members, friends, and neighbors about my husband's condition. Then I found myself teaching every home care worker who came into our home to help, including Larry's doctors, as they did not know how to speak Alzheimer's. And again, when it was time to place Larry in a care facility, I found myself teaching management and staff about dementia care as well as teamwork. I started as Larry's caregiver, but ended up as his care *pack leader*.

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FOREWORD

In the first edition of her book *Alzheimer and Dementia Coaching*, Dr. Éthelle G. Lord reveals her heartfelt feelings born of her experience as family caregiver for her husband, Major Larry S. Potter, USAF Retired, a specialist in the field of small business education, who lives with the diagnosis of Alzheimer. “I owe him an enormous debt and gratitude for introducing me to the connection between spirituality and dementia,” she writes.

In her second edition, Dr. Éthelle G. Lord outlines how she and Dr. Richard Rankin completed a study that began in 2015. Drs. Lord and Rankin are the first researchers to discover how to contain dementia using energy medicine. Two case studies can be found in the Appendix section of this book. Results speak for themselves.

What she also writes in her book is the product of her career as a university professor and her experienced-based, disciplined understanding of the clinical, psychological, and spiritual value of dementia coaching, and of the need for a standard in dementia care education.

Dementia is not the name for a specific disease but for a group of a hundred or more brain disorders that seriously impair the individual's ability to live a normal life.

Individuals living with dementia may be unable to think well enough to carry out daily activities, such as getting dressed or eating. They may lose their abilities to make decisions. Their personalities may change. They may lose the ability to control their emotions. They may become prone to agitation and seeing things that no one else can see. They may lose their memories to the point which they no longer recognize their family members. They may even lose their language abilities.

Dementia challenges healthcare systems because of reliance on methods for classifying the dementias that have much to do with defining which healthcare professionals get paid for their services, as they do with understanding the similarities and differences of the individual disorders that dementia comprises. The result, too often, is uncertainty or even confusion in terminology, which creates particular difficulties for family caregivers.

Dementia research is dominated by the research industry. Its first priority is searching for the cure; with the question of cause waiting for a clear answer, and with care having a much lower, if any, priority.

Drugs are available to treat some of the brain disorders of dementia such as strokes. While drugs cannot cure all the disorders of dementia or repair brain damage associated with these, some drugs may reduce symptoms or slow progress of some disorders. But such drugs may also be abused in attempts to suppress undesirable behaviors of individuals living with dementia.

Alzheimer's disease, as physicians term it, is the most common form of dementia among older people. But it is not part of normal aging. In its early stages, though, it may be difficult to distinguish from the ordinary processes of aging.

It slowly leads to incurable, irreversible, and unstoppable loss of brain functions. It destroys memory and thinking skills and, eventually, even the ability to carry out the simplest tasks of daily living. It usually starts with symptoms that first appear after age sixty-five but can appear much earlier. It terrifies governments and bureaucrats because of alarming predictions of future prevalence that promise huge, costly challenges for healthcare systems. In response, healthcare systems rely more and more on family caregivers who, increasingly, face physical, psychological, and financial exhaustion.

Dr. Lord argues for, explains thoroughly, and instructs clearly on dementia coaching. She aims for an *Alzheimer and Dementia-Friendly Health Care Workforce* with family caregivers in the role she describes as *pack leaders*. A book like this is the start of changing the course of dementia care and avoiding many of the false hopes, blind alleys in dementia care, and useless detours that delay positive results or possible solutions in dementia care. It is a rich resource for those providing day-to-day dementia care to the millions of individuals diagnosed with a form of dementia, especially with Alzheimer's disease.

What she is seeking to create is hope—an emotion associated all too rarely with so many of the disorders of dementia and with the particular dementia physicians term

“Alzheimer's disease.”

—Gordon Atherley, MD

ABOUT DR. ATHERLEY

- Investigative radio journalist and social archivist.
- Holds the UK equivalents of the North American PhD and MD degrees, and LLD, *honoris causa*, from Canada's Simon Fraser University.
- Prior to retiring from medical practice, his medical specialties were occupational medicine and public health.
- Executive Producer of *Schizophrenia Community Radio*, Dr. Atherley broadcasted the first program for the Schizophrenia Societies of Canada on August 12, 2015. Formerly he was the host, founder, and owner of *Family Caregivers Unite* with nearly a million listeners. The International Caregivers Association is in an equal partnership with Dr. Atherley and will be hosting, producing, and broadcasting *Family Caregivers Unite*. This radio program is especially inspiring to family caregivers and informing for all professional care providers. *Family Caregivers Unite* is an Internet radio talk show that empowers family caregivers by amplifying their voice, spreading their vision, and publicizing their value.

Founder and owner of EQualitative Research that derives from his work for his earned doctorate in epidemiology and

ALZHEIMER'S COACHING

biostatistics, his work in large-scale information services as founding P/CEO of the continually successful Canadian Centre for Occupational Health and Safety, and his current, influential work with *Voice America* in radio journalism.



PREFACE

In this second edition readers will discover germinal research that discovered how to contain Alzheimer. You will also find information on “aggression” in dementia. I have never encountered someone living with dementia demonstrating “aggressive behavior”. However, I am aware that care providers are under-trained in dementia care which sometimes leads to extreme frustration and even residents killing another resident. Such behavior does not have to end in tragedy. Dementia training that is universal is one solution (this topic is discussed in this book).

It is to reveal the many aspects of dementia care from a coaching point of view. Every family and every person living with Alzheimer is yearning for a better level of care. Care providers yearn for easier and more effective ways of providing such specialized care. All information provided here is intended to improve the quality of life and care for everyone.

By 2060, it is estimated the number of dementia diagnoses will climb to 98 million, worldwide. Figures for hospice costs for these individuals was estimated at \$259 billion by 2017.

The World Health Organization (WHO) has a global action plan on the public health response to dementia 2017-2025. This is a comprehensive blueprint for action – for policy-makers, international, regional and national partners. The goal is to establish dementia as a public health priority; increasing dementia awareness and dementia-friendly initiatives; reducing the risks of developing dementia; treatment and care; information systems for dementia; support for dementia care providers; and research and innovation (<https://www.who.int/news-room/fact-sheets/detail/dementia>).

Alzheimer and Dementia Coaching grew out of my personal experience as a family caregiver and my need for knowledge to create a rewarding caregiving experience with better results. I wanted my husband to have the fullest life in spite of this condition. I found a way to keep him interested and interesting throughout the entire process which currently continues. He is now clearly in the advanced stage of his condition. I do not believe family caregivers need to burn out before the caregiving journey ends. I do not believe those living with Alzheimer need to be neglected and suffering. I do not believe an undertrained health care workforce can survive the challenges of dementia care without the most basic of effective dementia care training and coaching. There is a better way.

There are serious gaps in dementia care training, whether in a medical or nursing curricula or in-service programs offered for those already working in this field. This book was written with the serious student in gerontology and future Alzheimer and dementia coaches; for the benefit of all care providers at home and medical personnel, ambulance responders, and emergency room personnel; and for law enforcement personnel

and teachers who encounter students providing dementia care to a grandparent or a parent.

I also do not believe that facilities such as hospitals and long-term care should treat dementia care as though the patient or resident is sick. Alzheimer is not a sickness. I look at it as a condition that requires specialized care and skill set. Therefore, it is imperative that medical programs teach their doctors, nurses and aids on how to provide such care prior to entering the field of healthcare. If health care professionals are already in the field, then the Alzheimer and dementia coach has the knowledge and training to train them on the job and improve such care.

Everyone living with this condition suffers. It is part of the human experience. Everyone providing care struggles to find the best possible ways to communicate and provide services in a way that is less stressful for everyone. This book contains a way to find dementia solutions and techniques that work.

The information in this book is divided into several chapters or to make it easy for the reader for the student to quickly access the information needed. In its totality, this information is the most basic any Alzheimer and dementia coach and others will need to learn and apply in the field. Therefore, you will find that some of the information applies to the family setting, the organizational setting, and community or living facility. It is up to the reader to refer to relevant information needed and for a serious student to consider all categories as important in the field of dementia care.

It is rare to find someone born with a natural talent for caregiving, let alone for specialized care such as is needed with Alzheimer and other dementia. Even doctors and nurses do not know how to “speak Alzheimer”—the language of dignity.

And the large majority too often bypasses the family caregiver, who has important personal and professional knowledge of the person living with Alzheimer or another form of dementia.

Speaking Alzheimer, which I address in this book, is basic to providing any level of dementia care. Other topics discussed include the coaching process, healthcare regulations, stages of caregiving, forming effective partnerships, and other topics of interest from basic to advanced. The book contains lots of tips you can apply immediately.

After almost fifteen years of firsthand experience, my goal is to create an important germinal body of work useful for the field of gerontology and Alzheimer and dementia coaching. It is also an important reference for training in dementia care and coaching. It is especially practical for those responsible for training certified nursing assistants and aides in attendance and for family caregivers interested in finding their task a little more rewarding.

There is no known cause or cure for Alzheimer. What we do have is the ability to simply manage dementia care better and train our healthcare workforce.

By managing dementia care better, we are able to improve on the quality and delivery of services, improve on the quality of life of those living with this debilitating condition, and reduce the enormous high cost of dementia care for everyone. By training our health care workforce in Alzheimer, we are able to improve on the working conditions that also improve on the quality of life of those individuals requiring such specialized care.

Here are two examples that immediately and effectively improve care as well as reduce the high cost of care. There

is a simple technique of brushing the teeth of someone with dementia, which requires a special toothbrush and toothpaste. Speaking Alzheimer requires following only three guidelines. Sounds simple? These two skills require commitment and training. A well-trained health care workforce in Alzheimer will be in demand and be valued in the near future. Those individuals will be able to command a better income and be invaluable. By adopting a systems approach, we are able to create an Alzheimer/dementia-friendly healthcare workforce that is knowledgeable and ready to make a difference. The numerous benefits of partnerships in this endeavor include lowering the cost of doing business and affording the person living with Alzheimer a continuum of care second to none. It is a natural move away from the medical/rehab model of care, which is too expensive and ineffective in dementia care, to a biopsychosocial (or care/ social) model of care, the components of which are:

- biological (anatomic and physiologic)
- psychological (thoughts, memories, beliefs, attitudes, emotions, and feelings)
- social (interpersonal relationships)

There is only one thing that is worse than receiving a diagnosis of Alzheimer. The worse thing is a healthcare workforce that has not been trained to care for the millions who have this diagnosis. Alzheimer and dementia coaches will reference this body of work because of the broad spectrum of information it contains. Since they are responsible for large teams of interprofessional health care workers in the field—whether in hospitals, long-term care facilities, or assisted living facilities—this reference will be a constant companion.

INTRODUCTION

THE CHALLENGE OF ALZHEIMER'S

I feel fortunate and am grateful to have the opportunity to provide care for my husband. Every day I am with him I learn a new lesson. He truly is the expert in his own condition, which was first diagnosed in January of 2003 as Alzheimer, and then rediagnosed a few years later as vascular dementia. Alzheimer is responsible for the largest healthcare challenge in all history.

Among the many lessons my husband has been teaching me are patience, trust, love, and spirituality. In *Alzheimer and Dementia Coaching*, you are exposed to a variety of topics supporting the work of Alzheimer/dementia coaches. Trained as a management coach at Coach U, I have been able to use much of my training in supporting my husband's dementia care needs. These skills have been invaluable in changing the course of his care at home and now in long-term care.

There are many specialties in coaching. Some coaches specialize in personal finance coaching, life coaching, business coaching, career coaching, or family coaching.

But I was unable to find anyone specializing in Alzheimer/dementia coaching. This book was born from my need to specialize in Alzheimer/dementia coaching and my desire to

encourage others to do the same. The world is not only ready for this coaching specialty; it is greatly in need of it.

The healthcare workforce, which begins in the home with the primary family caregiver, or pack leader, is lacking the most basic skills and techniques for performing such specialized care. A workforce that is undertrained and overworked leads to poor care for the care receiver and increases the cost of care. It is in yearning to improve care and services, leading to an Alzheimer/dementia-friendly health care workforce, that I offer you this book.

Alzheimer and Dementia Coaching is primarily designed for those interested in dementia-care coaching, gerontology, and neurology, and anyone who wants to deepen their understanding of this fascinating topic.

A diagnose of dementia or Alzheimer, in particular, is both daunting because of its complexity and various stages and encouraging because there is a small window of opportunity when symptoms can often be reversed. Dementia has two main areas of concern. The first is the diagnosis itself and the family dynamics such a diagnosis entails. The second is the care and housing necessary to provide the best possible services to individuals and their families. In the first case, it is important for doctors to introduce the diagnosis and possible timeline associated with such specialized care. In the second instance, a unique environment must be designed that considers lighting, color, natural light, and access to the outside, as well as the primary caregiver being included in this concept for a long-term care journey, should it be required. The concept is referred to as a care/social model of care or community where a family member is welcome to stay and participate in the day-to-day care. This is a unique system that allows the circle of care to be

completed with dignity and respect for the individual living with dementia.

A few words about the semantics in this book: Generally speaking, most of the references to Alzheimer are in the possessive case (Alzheimer's) when doing a search or reading articles, books, and speeches. However, Dr. Lois Alzheimer, a German physician who was first to describe this condition/disorder in 1906, is never referred to as Dr. Alzheimer's (in the possessive case).

I'm not sure why most people choose the possessive case when we do not refer to cancer as cancer's (i.e., "He was suffering from cancer's" when most people will say "She was suffering from Alzheimer's." Hope this makes sense for my readers. I can understand Alzheimer being capitalized as it is Dr. Lois Alzheimer's name we are referring to here.

I agree with the author of *Surviving Alzheimer's: Practical Tips and Soul-Saving Wisdom for Caregivers* by Paula Spencer Scott who stated, "Don't we all wish his (Dr. Alzheimer) name had been shorter and easier to spell?" You will find Alzheimer referred to in the singular form and also the possessive form throughout this book. Both forms are used intermittently.

My next book, *Natural Rhythm of Dementia: A Metaphysical Healing Experience*, is based on a true story of hope, faith, and healing. Our time on earth is brief, and therefore needs to be glorious. Providing care for those living with Alzheimer is a unique and priceless journey where they are the teachers and we are their students. This book will be inspiring to everyone who reads it

Part 1



STATE OF THE STATE OF ALZHEIMER COACHING

“Change your thoughts and you can change your world.”

— Norman Vincent Peale

OVERVIEW OF ALZHEIMER/ DEMENTIA COACHING

The Call for Alzheimer/Dementia Coaching

Coaching has an interesting history. Management coaching dates back to the 1950s. In the 1980s, Thomas Leonard, whom many consider the father of coaching, popularized coaching as a career path, and life coaching dates back to that period when it was an extension of sports and business coaching. But coaching itself is perhaps as old as the history of mankind. Coaching is a useful tool and career for

- guiding others;
- establishing basic moral structure; and
- encouraging someone to leap from self-help into coaching.

A large number of coaching specialties has emerged since the '80s. They can be divided into the following major categories:

- Therapeutic coaching

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- Life coaching
- Intervention coaching
- Career coaching
- Command coaching (instructing, such as fitness coaching and guided Alzheimer/dementia [A/D] coaching)
- Time management coaching
- Business coaching
- Health coaching
- Alzheimer's/dementia coaching

Health coaching is taught at universities across North America and is often a career specialty chosen by nurses and doctors. I predict that Alzheimer/dementia coaching (A/DC) will keep growing along with the demand in the healthcare field for such a specialization.

My work as a pack leader for my husband, Larry, has been going on for more than fifteen years as of the date of this writing. He is doing extremely well in spite of the hurdles and problems encountered in the course of providing his care and protecting him in every way possible. Providing for, protecting, and supporting the person living with this condition is the main role of all caregivers, especially the pack leader.

The need for A/DC became apparent to me once my husband was diagnosed with Alzheimer in January of 2003 after having had triple bypass surgery in the fall of 1999. The bypass surgery left him with brain trauma from which he was never able to recover, and early onset of dementia was subsequently diagnosed.

This is a man who graduated from college and entered the United States Air Force service in 1966. He was a career military officer retired in 1985 as a Major after twenty-one

years of service. At the time of his retirement from the air force, he had his master of business administration (MBA) degree. He then obtained his master of education degree (M.Ed.) and began to teach business courses at the University of Maine at Presque Isle in Maine. He taught as an associate professor for fifteen years before he was forced to retire on disability due to his dementia.

I noticed that nothing much had changed in health care for persons living with dementia since the '80s. As a paralegal for Legal Services for the Elderly in northern Maine, I had the opportunity to visit a variety of homes and facilities where individuals whose cases I was involved in received such care as part of their activities of daily living (ADLs).

Appalled at the state of services in long-term care facilities for someone living with dementia, I recognized a growing need to advocate for personal assistance to ensure the quality of care. Unfortunately, these individuals, and more specifically those living with Alzheimer, are often left in residential settings without advocates. This results in substandard dementia care.

There is an enormous market for service improvement. Therefore, our responsibility to do good instead of to do harm is that much larger as well. Mercy Gilbert Hospital in Gilbert, Arizona, has a wonderful approach to patient care. Outside of each patient's room is a little sign that says Pause—Reflect—Heal. Every staff member is required to place their hand on the sign and repeat the mantra to themselves before entering a room. The idea is that when they enter, regardless of the reason, they are to concentrate on that patient alone and block out thoughts about what they have to do next or how busy their day is. From doctors to nurses, to aides, to housekeeping, everyone follows this process.

In 2019 it is difficult, if not impossible, to find a country in the world that is not affected and devastated by Alzheimer. In a 2010 study by researchers at RAND Corporation of Redland, California and the University of Michigan published in 2013, dementia costs surpassed the number of diagnosis of cancer and heart disease added together. ([http:// journals.lww.com/neurotodayonline/Fulltext/2013/05160/ Dementia_Care_Costs_Surpass_Heart_Disease_Cancer.2.aspx](http://journals.lww.com/neurotodayonline/Fulltext/2013/05160/Dementia_Care_Costs_Surpass_Heart_Disease_Cancer.2.aspx)). It is a predictable and progressive condition that largely affects older individuals (sixty-five and older), but which can also affect individuals in their twenties, thirties, forties, or fifties (early onset). Healthcare leaders are frantically looking for ways to lower the enormous cost of providing specialized care for these individuals, who may live eight or nine years with their Alzheimer. Some individuals live twenty years or longer after diagnosis.

Healthcare leaders need assistance to find the best possible solutions for the least amount of money and to enact the responsibilities given to them. Alzheimer/ dementia coaches can provide exceptional service to health care leaders and teams of professionals because they learn specifically about Alzheimer and the change process of this condition.

Who can the healthcare leader or executive go to for a confidential ear? How and to whom will this leader voice his/ her legacy plan? What do they do when the same dementia care management problems keep coming up, and when they have to deal with serious staffing problems?

These are perfect questions to bring to an Alzheimer/ dementia coach because they are the same questions the coach asks themselves when they begin a new coaching relationship.

These questions can challenge even the coach, especially when faced with today's Alzheimer health care crisis. When presented with these questions, an Alzheimer/ dementia coach is motivated to find answers and can motivate the leader to impressive results.

What Is Alzheimer/Dementia Coaching (Also Known as A/DC)?

The Alzheimer's Association estimates that by the year 2050 there will be two billion individuals living with Alzheimer or any other dementia. Leaders are stymied at the sheer number of daily diagnoses and the specialized care required for this population.

Families often find themselves burdened by the high cost of dementia care at home and the physical demands of around-the-clock specialized care. If the family caregiver is employed outside the home, they soon realize they have to request flextime to meet the demands of caregiving at home. Eventually many retire early or must leave gainful employment altogether in order to meet the daily needs of their loved one who is living with Alzheimer.

The essence of an Alzheimer/dementia coach is to be the hub of the healthcare wheel, coordinating a large number of people who are all part of the interprofessional team. This always includes—or should include—family caregivers even after in-home care is no longer provided.

Alzheimer/dementia coaches bring specialized training and a fresh outlook to the problem of in-home care and long-term care. The coach is aware of the high cost of providing this specialized care, whether in the home or in a care facility and helps the family make cost-effective decisions.

Alzheimer/dementia coaches typically

- contract with home care agencies to help families establish systems and deal with the stressors of providing such specialized care to a loved one;
- support family caregivers and families in understanding a diagnosis of Alzheimer's by leading a family meeting to discuss sharing the enormous responsibilities of providing dementia care in the home;
- share conceptual ideas, brainstorming, and metaphors (leading to laser coaching) with healthcare leaders and executives;
- lead healthcare leaders and executives in setting up their planning, visioning, and expectations in the face of Alzheimer's and other dementias invading their facilities;
- invite healthcare leaders and executives to look at their own legacy and short-term and long-term professional goals;
- support and encourage leadership capabilities in healthcare leaders and executives for better managing their own anxiety facing this very serious situation; and
- assist healthcare leaders and executives with teamwork skills in using existing interprofessional personnel and family caregivers in partnership to provide the best possible care with the least cost.

Who Is an Alzheimer's/Dementia Coach?

Many people who are interested in the field of Alzheimer's have themselves experienced caregiving or are healthcare professionals. For retired and semi-retired folks who want to devote their lives to serving this population, becoming an Alzheimer's/dementia coach is a perfect way to accomplish this personal goal. Alzheimer's/dementia coach training is ideally

suited for certified life and health coaches.¹ It is estimated that diagnoses of Alzheimer's will at least triple by 2050, and it takes three care providers per person living with dementia to care for them.² The numbers are truly staggering. Coaching is a wonderful vehicle to help families and organizations better provide the care necessary with this healthcare crisis of biblical proportions.

In order to begin to understand the role of an Alzheimer's/dementia coach, it is important to look at three main areas:

1. Alzheimer's itself from the point of view of the condition and the person living with the condition
2. Caregivers, beginning with family members and continuing into long-term care
3. Care models and shifting from a medical/rehab model to a care/social model

An Alzheimer's/dementia coach has adopted an appropriate coaching philosophy, is certified in an Alzheimer's/dementia coaching program which includes the use of a range of behavioral-based coaching models and best practices, and has received quality mentoring and support from an Alzheimer's/dementia coach. If the Alzheimer's/dementia coach lacks any of these qualities, the client or organization may not receive appropriate coaching, and in fact may suffer damages, as Alzheimer's is a serious problem that can cause a lot of human suffering.

Who Is a Caregiver?

All professional care providers are caregivers, as are those who provide care at home or in a homelike setting. There are five types of caregivers:

ALZHEIMER'S COACHING

1. Accidental caregivers (estimated at over 70 percent)
2. Reluctant caregivers (estimated at over 20 percent)
3. Natural caregivers (estimated at less than 5 percent)
4. From-the-couch (FTC) caregivers
5. Seagull caregivers

Caregivers generally fall into one of two major categories—nonprofessional and professional—which are further subdivided into a number of different classifications. Nonprofessional caregivers, or informal caregivers, include family caregivers, pack leaders, care partners, and friends of the family. Family caregivers are the frontline care providers. The large majority of family caregivers become caregivers accidentally; meaning they did not see it coming nor could they plan for this specialized responsibility that is both life-changing and exhausting.

It is possible for a family caregiver to start as an accidental caregiver and end up as a natural caregiver. The chances are much less that someone starting out as a reluctant caregiver will end up as a natural caregiver. A natural caregiver remains a natural caregiver to the end of the journey.

The differences between the five types of caregivers are the stages they go through and the length of time it takes to rehabilitate from their caregiving journey. (More will be said in a later chapter about each type of caregiver and their stages.)

Caregivers in facilities such as assisted-living facilities and long-term care facilities, or formal caregivers, also fall into one of the five types listed above; however, they do have one benefit that a family caregiver does not—they have a larger team or coworkers and management, ongoing training offers to support them, and they usually have access to a social

worker who is able to help them process the end-of-life issues that may come up in the course of their responsibilities as a care provider or attendant.

CarePartners is a term coined by the International Caregivers Association Training Institute or ICA Training Institute, formerly known as the Remembering 4 You Training Institute. The ICA calls professionals trained in its Caregiver Partnership Agreement Program (CPAP) CarePartners. The other term coined by the ICA is “pack leader.” The CPAP teams all care providers from the physician to the family caregiver together as a healthcare team of interprofessional experts. The main purposes of forming such a large interprofessional team are to augment the quality of care, reduce the cost of that care for those individuals living with Alzheimer’s, and speed up services while reducing worker stress.³

Characteristics of an Alzheimer’s/Dementia Coach

Qualities of a successful Alzheimer’s/dementia coach include the following:

- Observant versus indifferent
- Patient versus reactive
- An active listener versus a passive listener

While each of these qualities is important, the most important has to be an *active listening*. Active listening requires *fully concentrating* on what is being said and the tone of voice being used rather than just *hearing* what is being said, and it includes observing gestures and body language to create a complete picture of communication. By practicing active listening, a coach can create an environment that is comfortable, inviting, trusting, and healing. In research

conducted by Korn Ferry International, active listening is considered a *compensator* for other qualities.⁴ In the home and in long-term care, active listening to someone living with Alzheimer's is a major component of the four essential elements of care: attentiveness, responsibility, competence, and responsiveness.⁵ To be responsive, one must be willing and able to actively listen to another.

Requirements of A/DC Training

At the ICA, A/DC training requires a minimum of an associate's degree along with an accredited coaching training program or equivalent training and experience. Additionally, each candidate must demonstrate their writing skills and possess the ability to function using their right brain function, which may include imagination, authenticity, reality-based feature, intuitive, understanding, and empathic traits—among many other traits.

Trainees can become institutional or independent Alzheimer's/dementia coaches and should demonstrate a willingness to help create a more comprehensive, holistic vision of caregiving. Individuals who complete an A/ DC training program gain a greater understanding and appreciation of their own personal growth and healing and the important role they play in affecting change.

Here are some important questions to contemplate while considering A/DC training:

- How is dementia care changing the world we live in?
- Why?
- What does this mean to families, management, and the healthcare industry?

- How do we adapt to this health care crisis?
- What if we changed how we view ourselves and those living with dementia?

In 2006, author Daniel H. Pink proposed using a right brain technique for responding to communication clues that is revolutionizing science and medicine while yielding a much deeper understanding of a wide range of human experience, from dyslexia to Alzheimer's. This requires developing the six senses, an ability any coach needs to provide effective coaching.

Physiognomy, along with body language, is another coaching tool that can communicate more information in a shorter period of time than any other strategy; information that is extremely helpful in all cases in which language, due to autism or Alzheimer's, to give only two examples, is a problem. (For more information about physiognomy and communication, go to <http://www.amazingfacereading.com>.)

Alzheimer's Coaching: Top Ten Myths

The following article by the author first appeared in The Canadian on May 8, 2013.

Alzheimer's coaching is a term used loosely by many, even those in the Alzheimer's Association and life coaches who want to get a piece of the action or strike while the iron is hot. Alzheimer's care is a multibillion-dollar industry. The money keeps pouring in at several large organizations and pharmaceutical companies that are doing research to find a cure. Despite the fact that there is no clearly known cause or cure for Alzheimer's, many keep promoting these established organizations in hopes of turning the Alzheimer's health care tide around.

First, what is Alzheimer's coaching? It is a new niche in coaching that I pioneered out of a serious need for someone to provide the help I so desperately needed after my husband was diagnosed with his Alzheimer's (vascular dementia) in 2003. Neither of us had any close family nearby; I found myself completely isolated and he found himself fighting for his very life.

Second, who needs a certified Alzheimer's coach? I soon found out that not only did I need a certified Alzheimer's coach to cope better, feel supported and provide my husband the best home instead cares but hospitals, nursing centers, and assisted living facilities (to name a few) need certified Alzheimer's coaches on staff. These organizations provide services to individuals living with Alzheimer's and interact with family caregivers every day but have little-to-no knowledge of Alzheimer's other than what they have been told by such organizations as the Alzheimer's Association (usually outdated information and raw statistics).

As with any new classification or niche in professional coaching, there can be more confusion than clarity regarding the definition of this new vocation. Here is a list of myths about certified Alzheimer's coaching and the associated facts:

Myth #1: The Alzheimer's Association trains Alzheimer's coaches who can provide all the help organizations, and especially family caregivers, may need.

Fact: The limited Alzheimer's coach training curriculum of the Alzheimer's Association is intended to continue serving this organization's fundraising goals and assist the public in a minimal way. Life coaches who promote being certified Alzheimer's coaches have usually not received any special or advanced training such as that offered by the Remembering

4 You (R4Y) Training Institute which pioneered the field of certified Alzheimer's coaching. Because anyone can hang up an "Alzheimer's Coach" shingle, many do who are not trained in that area of expertise. R4Y's training is 120 hours with two internships and lasts twelve weeks. I strongly recommend hiring a certified Alzheimer's coach who has successfully completed an International Coach Federation (ICF) accredited program such as this one at R4Y, because the ICF is the professional body that provides independent certification considered the benchmark for the professional coaching industry.

Myth #2: Alzheimer's coaching is an easy way to make a good living.

Fact: Coaching has never been an easy way to make a living for several reasons. The most prevalent reason is that most coaches are unable to market themselves and their talents. They simply do not have marketing in their DNA and must acquire those skills. Many never do, and therefore a very large percentage of coaching practices fold within months after opening. Coaches trained and certified as Alzheimer's coaches learn how to drive sales, be creative, and promote their talents as certified Alzheimer's coaches. Topics included in the business module include marketing to management and interprofessional health care teams, family caregivers and children of those living with Alzheimer's, law enforcement personnel, hotel/motel management, to name give you a few examples.

Although statistics are not yet available in this new coaching specialty, the ability of certified Alzheimer's coaches to increase the quality of services and reduce the high cost of providing care is promising, and job satisfaction and quality of life for individuals living with Alzheimer's are expected to increase. In

the area of Alzheimer's care, Alzheimer's coaching is perceived as the hub of the health care wheel and essential to the entire healthcare system approach.

Myth #3: Certified Alzheimer's coaches can only provide information on the stages of Alzheimer's and the facts and figures of Alzheimer's, encourage family caregivers to find time for themselves so they can provide family caregiving even longer, and empathize with those who have lost their loved ones to Alzheimer's.

Fact: A trained and certified Alzheimer's coach is a true expert in the field of Alzheimer's care because they know what works and what does not work. Many reports having been there in the trenches with family members. They are able to promote teamwork in an interprofessional health care team, manage and maintain an active partnership between a facility and families, and train others in becoming care partners who can provide great quality of life rather than a life that is less than acceptable. Long-term care facilities largely cater to Alzheimer's and dementia residents/patients, and they are truly unprepared to deal with this healthcare problem. Balancing a well-trained workforce with a solid work-life balance is critical in effectively coping with the daily demands of this specialized work. Alzheimer's is a crisis; there is no need to also have a crisis in our workforce due to lack of proper knowledge, training, teamwork, and support.

Myth #4: Alzheimer's coaching is for "crisis intervention" in facilities.

Fact: Coaching, in general, is regarded as a life-saver by employees who are about to be kicked out of their jobs or, at the very least, demoted. The large majority of facilities and their management do in fact manage by crisis. Not so with

Alzheimer's coaching. Certified Alzheimer's coaching is for management personnel who find it impossible to stay ahead of the Alzheimer's crisis, train and support their staff in carrying out their responsibilities effectively, increase family communications in order to add to the team's knowledge rather than ignore such a rich resource, and finally are refusing to continue to manage by crisis.

Certified Alzheimer's coaching focuses on managers' strengths and legacy to help them formulate a program that will improve and strengthen the delivery of care to persons living with Alzheimer's and dementia. It focuses on employee talent, training, and teamwork to alleviate the burden of delivering care to these individuals. Family caregivers have a true advocate in an Alzheimer's coach who is there for them, supportive, and informative.

Myth #5: Alzheimer's coaching takes too much time—time we don't have.

Fact: Certified Alzheimer's coaches are trained in a specific coaching method that includes laser coaching. Both the method and laser coaching take as little as a few minutes to twenty minutes per coaching session.

They are also trained in Alzheimer's programs that provide tips and ideas that can be applied immediately in the workplace upon leaving training. They have a wide spectrum of coaching skills and techniques that make delivery of care to individuals living with Alzheimer's and dementias more efficient. Certified Alzheimer's coach training is designed to augment basic accredited coach training programs and is considered advanced coach training.

Myth #6: Alzheimer's coaches are like luxury staff or good friends with whom you can share ideas or secrets and who will keep you motivated, no matter what.

Fact: Your certified Alzheimer's coach may be very empathic but is not there to befriend you. Many organizations such as hospitals, nursing centers, and assisted living facilities want a certified Alzheimer's coach on staff to be the hub of their healthcare organization. An Alzheimer's coach can create an Alzheimer's café, manage and maintain a number of partnerships, assess living needs of individuals living with Alzheimer's or dementia, and provide a wealth of information for management and staff.

Because a certified Alzheimer's coach wants to alleviate the burden of care, he/she advocates for family caregivers because these caregivers are largely isolated, often depressed, and welcome such a trained professional. The coach is trained to ask questions and allow you time to develop your own answers, always guiding you towards your goal but keeping in mind that there is an individual living with Alzheimer's for which there is not necessarily a cause or cure at the moment. A certified Alzheimer's coach is trained in a systems approach, whether it is an organizational system, a family system, or public system that affects the welfare and care of those living with Alzheimer's and other dementias.

Myth #7: Alzheimer's coaching only benefits family caregivers.

Fact: When family caregivers need facts and figures, free advice, or the kind of basic information found on the Internet, they can contact the Alzheimer's Association which freely provides printed material, training on family caregiving such as the SAVVY program, and free counseling. Although

this information is not always satisfactory for many family caregivers, it is free to them.

For family caregivers who really want to promote better caregiving and find easier, more effective caregiving tips and techniques along with the most recent research and findings which may improve the lives of their loved ones, a certified Alzheimer's coach is the answer.

For organizations and managers who truly want to find out what will work best in their workplace, save them money, and help cope better with the high demands of Alzheimer's and dementia care, a certified Alzheimer's coach is the answer. A well-trained workforce can do more, faster, for less, and with a higher degree of confidence. Creating an Alzheimer's friendly healthcare workforce is extremely important not only to attract more customers, but to keep customers happy, costs down, and employees enthusiastic about coming to work every day.

Myth #8: Alzheimer's coaches tell their clients what to do, how to do it, and when to do it.

Fact: Certified Alzheimer's coaches learn a *guided* coaching method designed to slowly introduce clients to coaching. Coaching is only a small portion of a certified Alzheimer's coach's practice (internal or external to an organization). Certified Alzheimer's coaches, like other professional coaches, do not give advice. They are trained to ask appropriate questions that lead to discovery and desired change. They can assess problem areas, design specialized training, establish programs to support the workforce resulting in improved levels of care and workplace satisfaction, knowledge of team formation and teamwork, and address many other issues concerning Alzheimer's care in institutions or at home.

Myth #9: Alzheimer's coaching is expensive.

Fact: Alzheimer's health care costs keep climbing, and family caregiving is not getting any easier or less expensive for families. Without the assistance of certified Alzheimer's coaches, managers and individuals keep repeating the same procedures and techniques over and over again, even the same mistakes. Many techniques are less than effective, and most are extremely costly. A certified Alzheimer's coach can be retained for as little as \$500 to \$3,000 a month depending on the contractual agreement, training and experience of the coach. Organizations and individuals are getting value in return for this investment. The International Coach Federation (ICF) has reported individual clients who claim a median return-on-investment (ROI) of 3.44 times their investment in coaching. The ROI is even greater for organizations, if only in reducing staff turnover, a line item directly linked to improved job satisfaction.

Alzheimer's coaching is worthless for organizations that want to maintain the status quo and continue to deliver sublevel services to individuals living with Alzheimer's and dementias, paying more for less and keeping their workforces at their current level—overworked and underpaid with high turnover ratios.

How can you put a price on a loved one who responds better to the care received and experiences improved memory as a result of their caregiver receiving coaching? In the words of Thomas Leonard in *The Portable Coach*, “Why do you think top-echelon coaches in professional sports now make almost as much money as the athletes (p.108)?” Great question, isn't it?

Myth #10: Certified Alzheimer's coaching is designed to replace other similar services in the field.

Fact: Alzheimer's and dementias affects 34.6 million individuals around the world today (even more that have not yet been diagnosed). The thinking that certified Alzheimer's coaches are greedy and believe they are the only ones who can make a difference is erroneous. Unlike the Alzheimer's Association, which is not only harnessing a great deal of money but claims the cornerstone on Alzheimer's, certified Alzheimer's coaches are not interested in the same kind of power and responsibility. Their primary focus is the actual care for the individual living with Alzheimer's or dementia, whether that care is classified professional or non-professional care, and all care partners.

Certified Alzheimer's/dementia coach training demands solid training, lots of reading and writing, and successfully completing two internships. As time goes on, more certified Alzheimer's programs will eventually be available on the market.

Hope often comes from the grassroots effort such as Dr. Mary T. Newport who provides important research information in her recently published book (2013) entitled *Alzheimer's Disease: What If There Is a Cure?* This book is a must-read for anyone in the business of providing any type of care. I am grateful to Dr. Newport for exposing the Alzheimer's Association and why it is difficult for anyone outside of that association to get its support in offering an alternative to what they already offer.

ALZHEIMER, DEMENTIA, AND THE COACH

Alzheimer

When a person receives a diagnosis of Alzheimer's, at least two people's lives are changed forever: the person diagnosed and the family member who becomes the caregiver. It is generally recognized that Alzheimer has three principle stages: mild, moderate, and severe or advanced.

There are distinct symptoms of Alzheimer that are easy to identify. It is always recommended to get medical advice because other health care conditions can mimic Alzheimer's. The ten symptoms are:

1. Memory loss that disrupts daily tasks of living
2. Sudden appearance of challenges in planning or solving problems
3. Difficulty completing familiar tasks at home, at work, or socially
4. Confusion with time and place orientation
5. Trouble understanding visual images and spatial relationships with increased risk of falling
6. New problems with words in speaking or in writing

7. Misplacing things and losing the ability to retrace steps
8. Decreased or poor judgment and reasoning
9. Withdrawal from work and/or social activities
10. Changes in mood and personality

Dementia can be compared to cancer in that there are many types of cancers, but the word “cancer” is not specific to any type, such as breast cancer, brain cancer, colon cancer, etc. The word “dementia” is not specific but rather describes a wide range of symptoms and diagnoses, such as Alzheimer, vascular dementia, Parkinson’s dementia, Lewy bodies, etc. However, all have in common a decline in memory and/or thinking skills sufficiently severe to reduce an individual’s ability to perform daily tasks of living.

According to the Alzheimer’s Association, Alzheimer’s accounts for up to 80 percent of all dementia cases/ diagnoses, while vascular dementia follows as the second most common type of dementia. Alzheimer has predictable stages and progresses over time. Some sufferers live only weeks or months after diagnosis, while others live for decades.

There are no known causes; therefore, no cures for Alzheimer. A large number of supplements appear to be effective in reversing the symptoms of Alzheimer and improving the quality of life for someone living with Alzheimer.

(For more information about supplements, see Newport, Mary, *Alzheimer’s Disease: What If There Was a Cure?* Basic Health Publications, 2nd Edition, 2013; Peggy Sarlin, *Awakening from Alzheimer’s: How 9 Maverick Doctors Are Reversing Alzheimer’s, Dementia and Memory Loss*, Online Publishing and Marketing, LLC, Lexington, VA. 2012; and Whitehouse, PJ and George, D., *The Myth of Alzheimer’s: What*

You Aren't Being Told about Today's Most Dreaded Diagnosis, St. Martin's Press; New York, NY. 2008)

Standard prescription drugs for the treatment of Alzheimer do not benefit everyone and have a short benefit period of only a few months, and individuals can find their condition worsened while taking such medications. Appropriate medication needs to be evaluated on a case-by-case basis by a medical doctor and the help of the primary caregiver.

Alois Alzheimer

The word “Alzheimer” comes from Alois Alzheimer (1864–1915), a German physician who earned great success as a professor of psychology in Breslau, Germany.

His primary interest was histopathology, which is the study of changes in tissues caused by disease. Alzheimer was also the founding father of neuropathology, which is the study of the nervous system tissue.

On November 4th, 1906, Alois Alzheimer gave his first lecture about a type of dementia he had observed in a fifty-one-year-old woman, Auguste D., from Frankfurt, who had been admitted to the Frankfurt Hospital with a cluster of symptoms: reduced comprehension and memory, aphasia, disorientation, unpredictable behavior, paranoia, auditory hallucinations, and pronounced psychosocial impairment. Alzheimer was able to show the progression of the signs of dementia in this patient.

Major Alzheimer's Milestones

The following milestones published at Alzheimers.net are based on US history and do not include milestones from other parts of the world.

There is admittedly a lot we don't know about Alzheimer's. 100 years after Dr. Lois Alzheimer's lecture, we still don't know exactly what causes this neurodegenerative disease and we do not have a cure. However, it is important not to lose sight of how far we have come in the last 100 years and noted the unique symptoms.

1906: Alzheimer's Disease is first described by Dr. Alois Alzheimer in his patient known only as Auguste D. The patient experienced memory loss, paranoia, and psychological changes. Dr. Alzheimer noted in the autopsy that there was shrinkage in and around nerve cells in her brain.

1931: The electron microscope is invented by Max Knoll and Ernst Ruska, allowing magnification up to 1 million times. This invention allowed scientists to study brain cells with greater detail.

1968: Cognitive measurement scales are created which allows researchers to measure impairment and estimate the volume of damaged brain tissue.

1974: Congress establishes the National Institute on Aging (NIA). To this day, the NIA supports Alzheimer's research.

1983: November of 1983 was declared the first National Alzheimer's Disease Month indicating a greater awareness of the disease.

1984: The NIA begins to fund Alzheimer's Disease Centers and establishes a nationwide network for Alzheimer's research.

1993: The Food & Drug Administration (FDA) approves the first Alzheimer's drug, Cognex. The drug targets memory loss and dementia symptoms. Today, there are a total of five drugs approved to treat Alzheimer's.

1994: Former President of the United States Ronald Reagan announces that he has been diagnosed with Alzheimer's Disease. This leads to greater awareness about the disease.

2003: The NIA begins a National Alzheimer's Disease Genetic Study to hopefully identify risk genes for the disease.

2010: Alzheimer's becomes the sixth leading cause of death in the United States.

2011: President Barack Obama signs the National Alzheimer's Project Act which provides a national framework to support and fund Alzheimer's research.

2013: The G8 Dementia Summit in the United Kingdom launches an international effort to fight Alzheimer's and find a cure by 2025.”¹

2014: The International Caregivers Association, LLC (ICA) was established with a home office in the United States. Founding President, É. G. Lord, DM, launches an international effort to change the course of dementia care and create a standard in dementia care by 2017.

Facts and Figures

The Alzheimer's Association estimates that by the year 2050 there will be 13.8 million (or more) diagnoses of Alzheimer's in the United States.² In the same report, the US Census suggested this number could rise as high as sixteen million.

Normally, Alzheimer's is diagnosed in individuals sixty-five years old or older. The genetic link in this population is more complex than it is in younger individuals.³ When Alzheimer's is diagnosed in individuals younger than sixty-five (some as early as in their twenties), it is considered early-onset or familial Alzheimer's (FAD). The percentage of familial Alzheimer's,

commonly referred to as genetic Alzheimer's, is estimated at less than 1 percent. FAD has been traced to mutations in three genes.

- About twelve families worldwide have a genetic fault on chromosome 21 in a gene called amyloid precursor protein (APP), which affects the production of the protein amyloid. Amyloid buildup in the brain has been linked to Alzheimer's.
- A slightly larger number of families carry a fault on chromosome 14 in the presenilin-1 (PS-1) gene, causing early-onset familial Alzheimer's. Mutations in PS-1 accounted for the majority of FAD cases, and more than seventy mutations have been described.
- A very small group of families (seven members of kin with FAD were described) carries a fault on chromosome 14 in the presenilin-2 (PS-2) gene.

The PS-2 gene is generally considered responsible for a variable penetrant clinical phenotype.⁴

In Columbia, where there are a number of families with a genetic mutation for Alzheimer, a hundred-million-dollar drug trial is underway that may reveal important clues in finding a cure for Alzheimer. This study, if a cure is found, will immediately benefit the family's future. The larger interest is for the rest of the world.⁵ For further information on the study of human memory, please go to <http://www.human-memory.net/>.

Types of Dementia

There are several types of dementia. The following descriptions of ten types of dementia were taken from the Alzheimer's Association, "Types of Dementia," 2013, <http://www.alz.org/dementia/types-of-dementia.asp>.

The most common type, Alzheimer, accounts for up to 80 percent of all dementia diagnoses.¹⁵ Individuals diagnosed with Alzheimer have difficulty remembering names and recent events. They experience apathy and depression, impaired judgment, disorientation, and confusion. Behavioral changes and difficulty speaking, swallowing, and walking often become apparent to others first until a diagnosis is finally made by a family doctor or other medically qualified professional. As of this writing, more than five million people in the United States live with Alzheimer. It quietly develops for years before diagnosis. Some believe it can take up to twenty years to manifest. The mortality rate is 5.7 years for women and 4.2 years for men.

Alzheimer affects new cells that are vital for memory and mental abilities. The connection becomes disrupted because of a lower than normal or necessary level of chemicals necessary to carry messages from one nerve cell to another. Memory, judgment, thinking, and reasoning are all affected, leading to the inability to handle day-to-day activities of living. Teepa Snow has recorded a DVD entitled *The Senior Gems*, which is available free of charge at <http://teepasnow.com/pac-partners/senior-gems/>. In this video, she describes how each part of the brain is affected and the impact of Alzheimer's and dementia on the individual.

The second most common type of dementia is vascular dementia, which was previously known as multi-infarct or poststroke dementia. A substantial percentage of bypass surgery patients experience vascular dementia.⁶ These individuals demonstrate impaired judgment and impaired ability to plan steps needed to complete a task due to brain trauma caused by microscopic bleeding and blood vessel blockage as result of the surgery. Brain imaging can detect the blood vessel problems.

The third major type of dementia is dementia with Lewy bodies. These individuals often have the memory loss and thinking problems common in Alzheimer's but also experience sleep disturbances with well-formed visual hallucinations and muscle rigidity or other parkinsonian movement difficulties, which usually present earlier than in people with Alzheimer's.

The fourth major type is mixed dementia, which is more than one type of dementia, occurring simultaneously in the brain of the individual.

The fifth major type is Parkinson's disease, which is often the result of progressive dementia similar to dementia with Lewy bodies or Alzheimer's. Problems with movement are the first symptoms, and as it progresses, the symptoms are similar to those of dementia with Lewy bodies.

The sixth major type is frontotemporal dementia (FTD), which includes behavioral variant frontotemporal dementia (by FTD), primary progressive aphasia, Pick's disease, and progressive supranuclear palsy. The symptoms include changes in personality and behavior with language difficulties. In this type of dementia, the nerve cells in the front and side regions of the brain are especially affected.

The seventh major type is Creutzfeldt-Jakob disease, which is the most common human form of a group of rare, fatal brain disorders affecting people and certain other mammals. Variant Creutzfeldt-Jakob disease, or mad cow disease, occurs in cattle and has been transmitted to humans under certain circumstances.

The eighth major type is normal pressure hydrocephalus, which is detected when someone presents with difficulty walking, memory loss, and inability to control urination.

The ninth major type is Huntington's disease, which is a progressive brain disorder caused by a single defective gene on chromosome 4. Symptoms include abnormal involuntary movements, a severe decline in thinking and reasoning skills, irritability, depression, and other mood changes.

The tenth major type is Wernicke-Korsakoff syndrome, which is a chronic memory disorder caused by severe deficiency of thiamine (vitamin B1). The most common cause is alcohol misuse or abuse. The memory problems may be strikingly severe while other thinking and social skills seem relatively unaffected.

Causes and Cures for Alzheimer's

At present, there is no known cause and therefore no effective treatment for Alzheimer.⁷ Research is ongoing, and care is our only hope for alleviating the fear and suffering these individuals and their families have to endure. Included in this second edition are two case studies documenting the containment of dementia (see the "USA Case and France Case — Containing Dementia" in Appendix 1). Containment means the progression of the dementia is arrested and much cognitive/physical abilities can be regained by the person living with dementia.

Medications called cholinesterase inhibitors were available on the market in early 2000, but they have proven to have limited benefits for this condition. They may prevent the breakdown of acetylcholine, a brain chemical affecting memory and thinking. As Alzheimer's progresses on its course with destiny, less and less acetylcholine is produced in the brain. However, there are too many misdiagnoses of Alzheimer's, and patients have freely prescribed these drugs which may prove to be

more detrimental in such cases than giving nothing or taking over-the-counter supplements such as phosphatidylserine (commonly referred to as PS), lion's mane, and antioxidants.

Currently available prescription medications can mask the progression of Alzheimer for about six to twelve months. They may help control agitation in the severe stage of Alzheimer in some individuals. Medications prescribed include Razadyne (galantamine), Exelon (rivastigmine), Aricept (donepezil), and sometimes Cognex (tacrine), a popular prescription at one time but rarely prescribed now due to safety concerns.

(For more information about such prescription drugs, see: Bell, S. H., Zhang, S., Czaja, S. J., Burns, R., and Schulz, R, "Use of Cognitive Enhancement Medication in Persons with Alzheimer's Disease Who Have a Family Caregiver: Results from the Resources for Enhancing Alzheimer's Caregiver Health (REACH) Project," 2004, *The American Journal of Geriatric Psychiatry*, 12(3), pp. 250–257.

Hartz, S., Getsios, D., Tao, S., Blume, S., and MacLaine, G., "Evaluating the Cost Effectiveness of Donepezil in the Treatment of Alzheimer's Disease in Germany Using Discrete Event Simulation," 2012, *BMC Neurology*, 12(1), pp. 1–12.

Figiel, G. and Sadowsky, C., "A Systematic Review of the Effectiveness of Rivastigmine for the Treatment of Behavioral Disturbances in Dementia and Other Neurological Disorders," 2008, *Current Medical Research and Opinion*, 24(1), pp. 157–166.

Moreira, T., "How to Investigate the Temporalities of Health," 2007, *Qualitative Social Research*, 8(1), pp. 1–23.)

Before anyone is prescribed one of these medications, it is important to get a thorough medical exam and a second opinion to eliminate the possibility of other medical conditions that mimic Alzheimer, such as delirium caused by a bladder infection and an inner ear problem that affects walking.

Care Models

Currently, the model used and supported for dementia care is the medical/rehab model of care. Since Alzheimer has no known cause or cure, it is technically not a disease; however, it is being treated as a disease by the healthcare community. The medical/rehab model of care for Alzheimer's and other dementias is ineffective on many levels, and contributes to raising the extremely high cost of dementia care.

Reform is in order when it comes to quality of care and cost of care for Alzheimer and other dementias. The Patient Protection and Affordable Care Act of 2010 (PPACA) proposed such reform because a single focus, such as cost reduction, is truly not enough. (For more information about the PPACA, see <http://www.nursingworld.org>.)

The history of progress in bringing changes in the United States is moving slowly but making progress year by year due to personal efforts of individuals who are living with dementia, public pressure on government officials, and several advocacy groups. Here are the highlights of some of the growth in public awareness from 2000 to 2017:

2017

Historic funding increasing to \$400 million granted to the Alzheimer's Association for research was signed into law. This brings annual funding to \$1.4 million.

2015

The Alzheimer's Association led the fight for a revolutionary law that allows scientists to submit an annual research budget directly to Congress.

2014

In December, Congress passed and the President signed into law the Fiscal Year (FY) 2015 Omnibus Appropriations Bill, incorporating the entirety of the Alzheimer's Accountability Act into the legislation. This legislation calls for the National Institutes of Health (NIH) to submit a Professional Judgment Budget for Alzheimer's disease research each year until 2025 to achieve annual research milestones established under the National Alzheimer's Plan.

2014

Legislation is passed and signed which provided an additional \$122 million in the FY14 budget for Alzheimer's research, education, outreach and caregiver support. This included \$100 million for research, \$20 million more than was in the President's FY14 budget request.

2014

The 35th state in the country published a state Alzheimer's disease plan.

2014

The 40th state enacted the Uniform Adult Guardianship and Protective Proceedings Jurisdiction Act (UAGPPJA) which establishes a framework for courts in different states to communicate about adult guardianship cases.

ALZHEIMER'S COACHING

2013

The Director of the National Institutes of Health (NIH), Dr. Francis Collins provided \$40 million from the Director's budget for additional Alzheimer's research.

2013

The Federal District Court of Vermont approved the proposed settlement agreement in the Jimmo v. Sebelius class action lawsuit which sought to end the application of the "improvement standard" as the basis for Medicare coverage decisions for skilled therapies.

2012

The first-ever National Alzheimer's Plan was released by the administration in May 2012. In connection with the plan's release, this included an additional \$50 million in federal Alzheimer research funding during the fiscal year.

2011

The National Alzheimer's Project Act (S.3036), was signed into law.

2010

The two-year waiting period for Social Security Disability (SSDI) for those diagnosed with younger or early onset Alzheimer's disease is eliminated. This change provides expedited access to SSDI and Supplemental Security Income (SSI).

2010

Beginning January 2011, advanced a new Medicare benefit that includes the detection of cognitive impairment as part of the annual wellness visit.

2010

For the first time, the decennial Health People Report included objectives for Alzheimer and other dementias underscoring the growing public health threat they pose to the nation.

2010

Ensured that the Patient Protection and Affordable Care Act contained provisions to help people with Alzheimer, their families and their care providers including a federal long-term care insurance program.

2009

Secured an additional \$10 billion for the National Institutes of Health (NIH) in the American Recovery and Reinvestment Act (“stimulus bill”), \$5 billion awarded so far, the National Institutes of Health (NIH) who invested \$77 million in Alzheimer’s grants. The Alzheimer’s Disease Neuroimaging Initiative was among the studies to receive such funding.

2005

Established the leading standard of quality for nursing homes and assisted living by offering practice recommendations and training for professional providers, as part of our Quality Care Campaign.

ALZHEIMER'S COACHING

2004

Gained inclusion of Alzheimer's medicines in Medicare's Drug Formulary (covered drugs).

2003

Passage of Medicare's Part D Drug Benefit, which went into effect in 2006.

2001

Secured rehabilitation benefits for Medicare beneficiaries with Alzheimer's disease so individuals were no longer denied this coverage.

2000

Passage of the Family Caregiver Support Program which is included within the Older Americans Act and specifically to support a number of community-based programs such as respite care, adult day care, counseling services and caregiver training. (<http://www.alz.org/advocacy/victories.asp>)

The ICA proposes a care/social model of care that encourages inter-professionality as well as a different sort of care facility. Inter-professionality among primary care clinicians, hospitals, specialists, nurses, all care professionals, and family caregivers to form a solid health care team improves the quality of care, reduces the cost of care, and improves working conditions for all caregivers.

A/DC as a Career Path

A/DC is a new career track and coaching specialty to meet the rapidly growing needs of an Alzheimer/dementia-friendly healthcare workforce throughout the world. This health care workforce must be able to provide adequate services, short

and long term, to a growing population of individuals living with Alzheimer and other dementias and their families, care providers, and emergency services.

At the time of this writing, it is estimated that there are over forty-seven million diagnosed cases of Alzheimer in the world, and it is safe to estimate that there are another ten million undiagnosed cases, or 60 percent since it takes over two decades for symptoms to become apparent. There are many people living with Alzheimer who are not aware that they have the condition.

As the pioneer in A/DC, I founded Remembering 4 You (R4Y) in November of 2011 to meet an important need following a decade of providing care for my husband, Larry. Remembering 4 You was the result of having to literally remember for him. Every morning, Larry would ask me, “What are we doing today?” If there were appointments or activities, I needed to remember for him. If there were holidays or special celebrations in the family, I had to remember for him—on top of what I had to remember for myself. I felt like I was living two lives simultaneously.

I also realized that information and care services were grossly inadequate and that much of the information available was actually inaccurate. That further stressed me out and increased my level of isolation. In time I became totally isolated and desperate and considered ending it all for both of us, as the future was bleak. My experience as a caregiver, or pack leader, as I now refer to myself, is far from unique. In fact, it is similar to that of the majority of family caregivers and pack leaders.

I was inspired to create a unique training program to meet the needs of all aspects of dementia care—the A/DC training

program. I saw a pressing and urgent need. I wanted to find a solution to this enormous problem for millions of families.

Here are just three of the problems I hoped to address.

- It was and continues to be obvious that doctors are not equipped to “speak Alzheimer” to those who suffer from it, and they often don’t know how to interact effectively with family caregivers.
- Researchers need more information about this condition before they advertise that they are near to finding a cure.
- The healthcare team must include a family member because they have unique insights into the problems of that individual. Caregiving is largely intuitive until standardized dementia care can be adopted by everyone.

While providing care at home and in assisted living, I pursued a doctorate in management (DM) in organizational leadership degree at the University of Phoenix (2005–2010). I already possessed my master of education degree (M.Ed.) in counseling from the University of Maine (1989–1992) and my bachelor of arts (BA) degree in behavioral sciences from the University of Maine at Presque Isle (1984–1987). My doctoral research is entitled *A Quantitative Study on Executive Coaching from a Learning Transfer Perspective* and was published at ProQuest, UMI 3436667.

Alzheimer and other dementias require not only medical attention but that those responsible have some knowledge of elder law, home care support (in the mild and moderate stages), long-term financial planning, food preparation, pantry design, and much more that is covered later in this book.

Families must move quickly to protect themselves from financial misfortune and failure to provide specialized care

during the mild and moderate stages, which can result in costly outcomes within a short period of time after diagnosis. They need to hold family meetings to discuss the condition and how best to approach the day-to-day care that can sometimes last decades.

Family members are the best people to provide care after diagnosis and are the front line of support. They must quickly establish and practice habits that both maximize a positive experience for them and provide a good quality of life for the care receiver.

Group support, although helpful, can often create a smoke screen hiding serious family issues lurking in the home. The quality of care is directly related to the quality of life for the person living with Alzheimer and their primary caregiver. The two are like two sides of the same coin.

Large organizations such as the Alzheimer's Association and the Alzheimer's Society are found in most countries around the world along with aging organizations such as the Area Agency on Aging AARP in the United States and aging societies in the United Kingdom, Japan, and China. All provide free literature and often free phone-counseling sessions. However, they are not always the best source of information and help in the marketplace.

A trained Alzheimer/dementia coach provides the best possible help available, and one can do an Internet search or call one's local hospital for a referral source for a trained, certified Alzheimer/dementia coach (CA/DC). CA/DCs are available via phone, Internet, Skype, and any other electronic tool available. A list of CA/DCs is available at the ICA at www.internationalcaregiversassociation.com or www.icareassoc.com.

A/DC at the Pioneering Level

Practical and proven tools and techniques are the best for jump-starting a CA/DC program. Recommended reading is the first order to introduce students to current research in this field.

The objective of an introductory module such as ICAs is to review the overall content of the program and the tools and methods they will use in the field as practitioners. Alzheimer/dementia coaches are ultimately able to coach individuals and groups within the larger interprofessional health care team. The goal is to create an Alzheimer/ dementia-friendly healthcare workforce of which the Alzheimer/dementia coach is an integral part. The ICA's delivery system, which will be introduced in a later chapter, is the CPAP. Ideally, this program is formed and managed by a CA/DC.

PHILOSOPHY OF AN A/DC PROGRAM

The ICA adopted a holistic philosophy regarding its A/DC training program. Alzheimer/dementia coaches facilitate life changes in those who provide care for someone with Alzheimer. Using questions rather than giving advice, the coach is responsible for keeping the client's goals in mind and being mindful of the physical, intellectual, emotional, energetic, and spiritual aspects of the life of the individual they are coaching.

Alzheimer/dementia coaches do not pass judgment on others. They meet people wherever they currently find themselves in their life and bridge the gap between where they are and where they want to be.

Alzheimer/dementia coaches do not coach the person diagnosed with Alzheimer's; instead, they work with individuals and long-term care management teams—the teams of caregivers, family members, and individual family caregivers. They can make referrals and facilitate the assembly of an optimal team of professionals. The training offers the working knowledge and skills to work from within or outside of existing organizations.

ALZHEIMER'S COACHING

Alzheimer/dementia coaches are trained to work in a multitude of practice settings and delivery models, including hospitals, nursing centers or nursing homes, assisted living facilities, and privately with clients via phone, in person, or using the Internet. While they work in collaboration with a variety of professionals in all disciplines, they do not duplicate the work of social workers, counselors, case managers, or spiritual directors. Other trained and certified coaches such as life coaches and management coaches have expertise that overlaps that of an Alzheimer/dementia coach, but they do not have the full range of special training and capabilities of Alzheimer/dementia coaches. Through the empowerment, support, and education of clients, Alzheimer/dementia coaches facilitate the evolution of our health care system all over the world.

In addition to completing the certification program, an ICA Alzheimer/dementia coach candidate must complete a demanding, full-year study program (September to May).

Part 2



INDIVIDUALS AND A/DC

“There is only one corner of the universe you can be certain of improving, and that’s your own self.”

— Aldous Huxley

APPROACHES IN A/DC

A coach's philosophy reflects their moral standards and integrity and allows them to map the coaching process. It also affects their perceptions of how an individual makes decisions, how an individual behaves and why, and how an individual demonstrates consequences to actions taken.

While a philosophical approach may be abstract, ethics are much easier to understand and follow. Examples of ethical conduct in coaching are maintaining confidentiality, upholding applicable professional standards such as those of the International Coach Federation (ICF), providing services that are competent and appropriate, only claiming qualifications for which the coach holds a certification, and so on.

Coaches in general usually follow one of the following philosophical approaches:

- transactional analysis (TA)
- neuro-linguistic programming (NLP)
- Adler
- faith-based

- trait factor
- eclectic
- holistic

A coach's philosophy guides the coaching process. It is how the coach gives value to each coaching situation. It is important that the coach is aware of their preferred philosophical approach or approaches and that they make them known to their clients.

Coaches should understand

- a coach's philosophy impacts many other people;
- the importance of developing a sound coaching philosophy;
- the need for their own personal and professional self-awareness for developing a sound coaching philosophy;
- the need to clearly identify and clarify their coaching objectives when developing a sound coaching philosophy; and
- coaching philosophies need to be evaluated and modified over time.¹

The coaching dialogue in A/DC is an interactive process for inviting others to participate in a sustainable, thinking and behaving dialogue with action. The process includes the guided A/DC method, in addition to the standard five coaching steps:

1. guided A/DC method
2. asking for meaning
3. building a new perspective
4. creating a bridge
5. developing action

Coaches usually have a particular coaching style acquired during their training. Coach training can take up to two years of classes and preferably includes mentoring with a seasoned coach during the entire training process. There are different styles of coaching. Here are just a few examples of styles you might find:

- autocratic
- democratic
- holistic
- 3-D
- Vision
- solution-focused
- Zen, etc.

(For more information about coaching styles, see <http://www.lifecoach-directory.org.uk/content/coaching-styles.html>).

A coaching style affords the coach an overall direction in the coaching session. Selection of a style of coaching is dependent on outcomes and how the client wishes to get from where they are at the beginning of coaching to where they want to be at the end of coaching.

Thomas Leonard founded Coach U in 1992, a virtual coach training university which quickly became a leading school in the nation with thousands of coaches around the world, and also coined the phrase “laser coaching.” Leonard is also known as the father of coaching. He was a brilliant visionary with unlimited amounts of creative energy who took the world by surprise and made coaching an everyday service for individuals and management alike.

The ICA's Alzheimer/dementia coach training supports two studies done by W. Michael Cox and Richard Alm at the Federal Reserve Bank in Dallas, Texas, USA.³ These researchers examined ten years of employment data and found that the largest gains in the workplace required *people skills and emotional intelligence* (for registered nurses and lawyers) and *imagination and creativity* (for designers and dementia caregivers). People skills and emotional intelligence (EI) have typically been regarded as low scores under the emotional realm of those profiled by the A New Standard in Relating (ANSIR) personality profiler.⁴ EI and spiritual intelligence may well be bedfellows. In dementia care, both are needed in order to improve the quality of life for these individuals. By-products of improving on their quality of life are a high quality of work and work satisfaction on the part of the coach and the caregiver.

Freshman and Rubino classified EI as a core competency for healthcare administrators in 2002.⁵ The term “emotional intelligence” was coined in 1990 by Mayer and Salovey to describe the compassion, empathy, trust, and interpersonal skills found in self-knowledge, observation, and contemplation.⁶ Daniel Goleman includes “social intelligence” and “ecological intelligence” in EI and is responsible for popularizing it in his book *Emotional Intelligence* in 1995.

One of the main roles of the Alzheimer/dementia coach is to invite healthcare leaders, managers, and supervisors to adopt the concept of EI in the workplace in the form of management style and dementia-care training style. The ICA has adopted a unique dementia-care program that includes EI, right-brain capabilities, and inquiry-based learning. It is ICA's hope that tens of thousands of dementia-care programs worldwide will

adopt the same. *Harvard Business Review* has referred to EI as a “groundbreaking, paradigm-shattering idea,” and it’s just what dementia-care training needs.

DECISION-MAKING IN A/DC

The ability to make good decisions is critical in dementia care. Each decision affects a number of individuals and systems. This is the most important skill for an Alzheimer/ dementia coach to develop. Most people understand how they arrive at a decision, so it is extremely valuable for the Alzheimer/dementia coach to know more about this process.

Movement from A to B— “What do I do next?”— is the decision-making process in coaching. This applies to individuals, environments, and systems. All options are examined and, one by one, eliminated until the right option or best option is left. This is the only way to bring decision-making into your conscious awareness.

Guided coaching questions are designed to help reach decisions that are life-changing and consider the system as a whole. The coach uses guided questions— “what” and “how” questions— during at least the first four sessions to establish rapport with the client. Guided coaching is just what it sounds like; it is guided by the coach who has a specific purpose in asking specific guided questions. There is one distinctive difference between A/DC and other types of coaching in that the

client is usually entering coaching based on a recommendation from family members or healthcare professionals within an organization.

Guided questions help achieve the following:

1. Create a “wheel of life”.
2. Establish a legacy profile.
3. Identify three changes desired in day-to-day care.
4. Identify three problem areas.
5. Identify personal and professional goals.

Using a coaching tool such as the wheel of life to assess a client or members of a group is helpful and valuable for everyone concerned. It is especially useful for those who are introverted and tend to turn inward, but both introverted and extroverted individuals benefit. The coach gains important knowledge that leads to profiling the client's desired legacy. (See Appendix 1 for details about the wheel of life tool and its use in coaching.)

The following is an example of a guided A/DC session with an executive:

First session:

A/D coach: I want to use the next two or three meetings with you to complete the wheel of life and Dementia Coaching gather more information about the work that you do here at Mary's Healthcare Facility. Is that okay with you?

Joe, the executive: Perfectly fine with me. Go ahead. We have a lot of problems here, and I welcome this opportunity to improve our operation and service

delivery for our residents living with Alzheimer's and other dementias.

Coach: Wonderful, Joe I do like your positive energy in the face of such grueling work and responsibility. Let's take a moment and review your wheel of life results... In our next session, I want to focus on what you want your legacy to be after your work here is completed.

Second session:

Coach: In our last session, we examined and discussed your wheel of life results and determined that the area you want to focus on is... In this session, I would like to know what you want your legacy to be after you leave this job or position. What three things would you like to improve on in the direct day-to-day care of your residents who are living with Alzheimer's or another dementia?

Third session:

Coach: In our last session, we covered your legacy and were able to identify three things you would like to improve on in-direct care during our last session. Now, I am interested in what three things seem to create problems and could be improved on in regard to families and their relationships with either your staff or the residents.

Fourth session:

The coach begins using "what" and "how" questions to determine what Joe wants to accomplish through coaching.

Kevin McAlpin and Hans Vaagenes developed a series of coaching questions that are helpful in decision-making:

ALZHEIMER'S COACHING

- Where am I now?
- Where is my goal?
- How do I get there? What options do I have?
- What resources do I need and how committed am I?¹

McAlpin and Vaagenes tell us that a strategic decision is one that can significantly redirect the system. Such decisions have the power to reorganize an entire system. One requirement is that the individual must be able to take a step outside the system to ensure a broader view from both internal and external aspects of the system. For example, using a systems approach to change the course of dementia care is considered a strategic decision. Another example is making the decision to elevate the value of the work of certified nurse assistants and home care workers by providing them proper dementia-care training. Another example is finding the answer to the question: “How does our system allow for such a mistake or mishap?”

PERSONALITIES AND STYLES

Neuro-Linguistic Programming (NLP)

Using NLP as an approach to communication and personal development in coaching is both quick and easy with the NLP assessment provided in Appendix 2. (See Appendix 3 for calculating results of the NLP assessment and Appendix 4 for the NLP matrix.)

NLP is used to determine the individual's preferred way of communicating and relating to others. It also teaches that when speaking to a group, it is best to ask the same question in different ways in order to relate to everyone in the audience.

NLP can optimize individual and organizational performance. As an important component in modern psychology, it is an extremely powerful concept because it is accessible and positive. NLP is beneficial for self-development and for businesses and organizations to improve communications and get more of what they want instead of less. NLP helps the individual achieve better self-awareness and self-control, increases their ability to appreciate another person's feelings and behavioral style, and encourages empathy and cooperation among people.

NLP can improve one's ability to understand one-on-one communication and increase emotional intelligence (EI), an important aspect of multiple intelligence theory. Furthermore, it is helpful in stress management and developing self-belief, assertiveness, and self-confidence. (For more information about NLP, go to <http://www.businessballs.com>.)

If an individual's preferred way of communicating is known, an exact question can be formulated or expressions used to be readily understood. It is easy to teach this method of communicating to management and healthcare workers. Simply present the assessment, allow participants to score their own results, and then have a discussion about the results.

There is no right or wrong style of communicating. It is simply different.

This article is reprinted from www.nlp.com:

The Eyes Have It!

[Have] you ever wondered how some people do well at poker? No? Well, listen anyway.

I have a friend; we'll call him Bob (since it's the only name he responds to) whom I play poker with. Bob's a nice guy but he's not much of a poker player. If he's got a full house, I know it. If he's sitting there with the world's worst hand, I can tell. Do I cheat?

No, not in this instance.

Bob follows patterns. For instance, if he's leaning back and looking upward like he's imagining how he's going to spend his vast fortune in the Bahamas and his body is relatively still, I know it's time to fold and cut my losses. If, on the other hand,

Bob is upright, taps his fingers, fidgets and is generally active, it's time to bet the Christmas fund.

Bob is not subtle, that's obvious. But all people exhibit patterns like these in response to a given stimulus (full house). How could your Dad (Mom, teacher, spouse, bookie, etc.) know that you were lying just by looking at you?

They learned your patterns. Maybe it was the look in your eye, your shuffling feet, the cookie crumbs on your chin or something not quite definable, but they knew.

NLP looks for patterns and one of the earliest patterns noticed in humans was a tendency for the eyes to look in a certain direction when thinking in a certain way.

More specifically, people generally look up and to your right when thinking about a remembered image (Aunt Martha), up and left when constructing an image (Aunt Martha sitting on top of a flagpole), directly right for a remembered sound (Aunt Martha's gravelly voice), directly left for a constructed sound (Aunt Martha saying she's included you in her will), down and to your right when having an internal dialog with themselves ("Boy, I hate Aunt Martha") and down and left when experiencing feelings (experiencing how you really feel about Aunt Martha).

These only provide information about how someone is thinking not what they're thinking, an important distinction. So, what good is this? Good question, listen.

Example: If you are trying to convince your teenager to clean his room and every time he refers to his lack of interest in the subject, he looks up and to his right, he's probably constructing an image, possibly an unpleasant image of himself cleaning his room. If you continue telling him how nice it would feel

to have the room clean, you're not likely to get far. You're trying to appeal to how it feels to have a solution when all he's seeing is a problem. You're not speaking the same language representationally. If you were to recognize this difference and tell him how nice his room would look with everything appearing neat you would have a better chance of getting some action out of him (were he not a teenager).

Recognizing whether someone is seeing, hearing or feeling, allows you to match the sensory system they're using and communicate with them much more easily.

More importantly, if you play poker with the same people again and again, you might begin noticing some patterns that might prove profitable.

By the way, it's wise to check to see if anyone at the table is NLP so you can abruptly switch your patterns (all's fair in love and poker).

Note: Watching someone's eyes and then referring to a chart often makes others nervous (at least in my experience), so I would suggest finding a friend and making an eye-accessing chart. Have them hold it in front of their chest and as they move their eyes around, call out the sensory system they're using. Getting the movements down usually takes only a few minutes.¹ (For more information about reading another's eyes, go to: <http://www.ncbi.nlm.nih.gov/pmc/articles/> (and enter PMC3394779) in the search bar.

How to Become a Wise Guy

Gather together the following items:

- A few minutes in a quiet place
- The directions for this experience
- Your brain (essential)

Now, let's begin.

Think of a time when you were having an unpleasant conflict with someone else (if you've never had one, call me and I'll yell at you). This can be your husband, child, business associate or anyone.

Re-experience this situation (I know, I know but we're going to make it better soon). Notice the other person's reactions, their physical posture, gestures etc.

Clear that image away and now, re-experience the situation as the other person. Using the information you gathered in the first stage, step into their shoes and imagine as you progress through the experience, what their feelings and attitudes are.

After you have processed any new knowledge gained through that experience, you can clear that image away and now, re-experience the experience from no one's point of view. In other words, watch the both of you from a neutral space. Notice any new information gained by this new perspective.

Now what? Well, the information you've gathered from these different points of view have enormous value. You can now perceive how others see you and how an entire situation can be viewed from a neutral position. Instead of one way of experiencing the world, you have three and are better able to empathize with others and make more objective decisions. I

believe that having all this information readily available is the basis for wisdom.

Warning: While this is a very useful way to experience and gather information about the world, realize that these are our guesses as to how others are feeling and one should test the waters before jumping in.²

Physiognomy: Face Reading

Physiognomy, or face reading, is centuries old. In the seventeenth and eighteenth centuries, the age of enlightenment, physiognomy was a topic of empirical investigation and scientific attention. Today, Mac Fulfer, JD, is making face reading his life passion. He teaches face reading in the hope of inviting people to open their hearts to understanding and connect better with every person they meet. For more information about his work, visit [www. amazingfacereading. com](http://www.amazingfacereading.com).

According to Fulfer, there are features on every person's face that have meaning for all to see and appreciate. With forty-three muscles or facial nerves that register emotions and life events, it is possible to read not only feelings but also a person's decision-making style, openness, sensitivity, how they listen, and the effects of their life story. In dementia care, it is about preferences for how best to communicate with them, activities that are most suitable, and a care plan that is truly person-centered.



Physiognomy has nothing to do with mind reading. It has everything to do with the inner being. It focuses on the individual, their history, and their needs and preferences. ICA is including face reading in all coach training programs and dementia care educational programs. Because it is possible to read any changes due to thinking or feeling, face reading allows the care provider to connect intimately and stay connected with the person who is living with dementia.

In the words of Mac Fulfer, face reading “is a gift that enables you to deepen your communication (and therefore the connection) with every person you meet. To see, understand and accept another individual at face value is a privilege and honor to be respected” (Fulfer, M. 2011, ‘What is your face really saying? *The TCU Magazine*, Summer 2011, pp.38–47).

For all care providers, face reading will not only speed up the process of getting to know the person living with dementia, it will also have an immediate effect on the quality of care and life. Going forward, the practice of face reading will be part of any Alzheimer/dementia coach training programs, dementia care

educational programs, and a key to any provider of services to these individuals. Fulfer's legacy is to improve the way people communicate, to find in each and every individual something special that recognizes their uniqueness, talent, and trials. Isn't that the definition of person-centered care?

Belbin and Team Roles

The following article was retrieved September 1, 2008, from MindTools.com, http://www.mindtools.com/pages/article/newLDR_83.htm

Belbin's Team Roles: How Understanding Team Roles Can Improve Team Performance

When a team is performing at its best, you'll usually find that each team member has clear responsibilities. Just as importantly, you'll normally see that every role needed to achieve the team's goal is being performed fully and well.

But often, despite clear roles and responsibilities, a team will fall short of its full potential.

How often does this happen in the teams you work with? Perhaps some team members don't complete what you expect them to do. Perhaps some team members are not quite flexible enough, so things "fall between the cracks." Maybe someone who is valued for their expert input fails to see the wider picture, and so misses out tasks or steps that others would expect. Or perhaps one team member becomes frustrated because he or she disagrees with the approach of another team member.

Dr. Meredith Belbin studied team-work for many years, and he famously observed that people in teams tend to assume different "team roles." He defines a "team

role” as “a tendency to behave, contribute and interrelate with others in a particular way” and named nine such team roles that underlie team success.

(See Appendix 5 for the Belbin Team Role Summary Descriptions.)

Creating More Balanced Teams

Belbin suggests that, by understanding your team role within a particular team, you can develop your strengths and manage your weaknesses as a team member, and so improve how you contribute to the team.

Team leaders and team development practitioners often use the Belbin model to help create more balanced teams.

Teams can become unbalanced if all team members have similar styles of behavior or team roles. If team members have similar weakness, the team as a whole may tend to have that weakness. If team members have similar team-work strengths, they may tend to compete (rather than co-operate) for the team tasks and responsibilities that best suit their natural styles. Knowing this, you can use the model with your team to help ensure that necessary team roles are covered and that potential behavioral tensions or weaknesses among the team members is addressed.

Tip

Belbin’s “team-roles” are based on observed behavior and interpersonal styles.

Whilst Belbin suggests that people tend to adopt a particular team-role, bear in mind that your behavior and interpersonal style within a team is to some extent

dependent on the situation: It relates not only to your own natural working style, but also to your interrelationships with others, and the work being done.

Be careful: You, and the people you work with, may behave and interact quite differently in different teams or when the membership or work of the team changes.

Also, be aware that there are other approaches in use, some of which complement this model, some of which conflict with it. By all means, use this approach as a guide, however, do not put too much reliance on it, and temper any conclusions with common sense.

Understanding Belbin's Team Roles Model

Belbin identified nine team roles and he categorized those roles into three groups: Action Oriented, People Oriented, and Thought Oriented. Each team role is associated with typical behavioral and interpersonal strengths.

Belbin also defined characteristic weaknesses that tend to accompany the team-role. He called the characteristic weaknesses of team-roles the “allowable” weaknesses; as for any behavioral weakness, these are areas to be aware of and potentially improve.

How to Use the Tool:

The Belbin Team Roles Model can be used in several ways: You can use it to think about team balance before a project starts, you can use it to highlight and so manage interpersonal differences within an existing team, and you can use it to develop yourself as a team player.

The tool below helps you analyze team membership, using the Belbin team roles as checks for potential strengths and weakness.

Use Belbin's model to analyze your team, and as a guide, as you develop your team's strengths, and manage its weaknesses:

1. Over a period of time, observe the individual members of your team, and see how they behave, contribute and behave within the team.
2. Now list the members of the team, and for each person write down the key strengths and characteristics you have observed. (You may also want to note down any observed weaknesses).
3. Compare each person's listed strengths and weaknesses with the Belbin's descriptions of team roles, and note the one that most accurately describes that person.
4. Once you have done this for each team member, consider the following questions:
 - Which team roles are missing from your team? And from this, ask yourself which strengths are likely to be missing from the team overall?
 - Is there a prevalent team role that many of the team members share?

Tip

Among teams of people that do the same job, a few team roles often prevail. For example, within a research department, the team roles of Specialist and Plant may prevail. A team of business consultants may mainly comprise Team Workers and Shapers. Such teams may be unbalanced, in that they may be missing key approaches and outlooks.

If the team is unbalanced, first identify any team weakness that is not naturally covered by any of the team members. Then identify any potential areas of

conflict. For example, too many Shapers can weaken a team if each Shaper wants to pull the team in a different direction.

5. Once you have identified potential weaknesses, areas of conflict and missing strengths, consider the options you have to improve and change this. Consider:

Whether an existing team member could compensate by purposefully adopting a different a team role. With awareness and intention, this is sometimes possible.

- Whether one or more team members could improve how they work together and with others to avoid potential conflict of their natural styles.
- Whether new skills need to be brought onto the team to cover weaknesses.

Tip

Remember not to depend too heavily on this idea when structuring your team—this is only one of many, many factors that are important in getting a team to perform at its best.

That said, just knowing about the Belbin Team Roles model can bring more harmony to your team, as team members learn that there are different approaches that are important in different circumstances and that no one approach is best all of the time.

Sides of You:

Unlocking the Way You Think, Work, and Love

You are encouraged to read *3 Sides of You: Unlocking the Way You Think, Work, and Love* by S. Seich. It provides a rare and

unique view of the combination of personalities and styles in all societies that Alzheimer/dementia coaches can apply to their interprofessional care teams. Unfortunately, the assessment is no longer available online. ANSIR stands for A New Standard in Relating. The ANSIR community closed when Seich died in the arms of her beloved husband, Bill, following a long battle with breast cancer on December 24, 2009. Seich was a genius who developed a system to improve communication, understanding, and harmony unlike any other in the world—no matter the nationality or language spoken.

Seich spent nine months in the state of Maine in the mid-1990s working with me. I held the position of director of ANSIR training and certification for a period of about ten years. Both experiences were extremely valuable and unforgettable. Her system of personality styles will live on (See Appendix 6 for the ANSIR Matrix of Styles).

Her entire body of work is totally original, more accurate in determining personality types than any other found on the market today, and superior in value to each and every person self-profiled.

At the time of her premature death in 2009, Seich's research focused on style compatibility. She referred to this as “match/mismatch” in *3 Sides of You*. Her focus was to increase the chances of meeting someone with a personality style compatible with your own and to learn *why* you are compatible.

May her soul rest as one of the many bright stars in our universe and continue to inspire all of us.

Part 3



INTRODUCTION TO A/DC TRAINING

“If you don’t like something, change it; if you can’t change it,
change the way you think about it.”

— Mary Engelgreit

WHY A/DC IN HEALTH CARE ORGANIZATIONS?

Without sounding critical, I think the time has come to take a serious look at health care organizations responsible for dementia care. Simply taking a quick look at analytics in such organizations, it is easy to see that dementia care is prominent. Yet few, if any, organizations have considered the time consumed by dementia care on the part of their workforce. Starting with doctors, they still spend only a limited time (perhaps five to fifteen minutes per patient in their office) to assess the health care needs of those living with Alzheimer or another dementia. Without speaking Alzheimer, it is impossible for a doctor to get a good assessment of a dementia patient.

Emergency rooms (ER) in hospitals are other places visited by dementia patients. Yet the medical staff in an ER cannot even speak Alzheimer. Dementia care requires an interprofessional team that includes the pack leader in order to obtain the necessary details, to develop a proper assessment, and to make the correct diagnosis in either an emergency situation or during a regular doctor's visit.

I see teamwork and interprofessional healthcare teams as the first order of concern. Second, analytics is rarely considered

in dementia care or even in other serious health conditions in many assisted living and care facilities. This leads to burnout of their workforces, simply because they do not have the expertise or the number of employees needed to do the work, and both residents and patients suffer needlessly. Finally, a system review and a system approach to finding solutions assure the best possible outcomes for everyone.

Dementia care is challenging some of the best healthcare systems in the world. It challenges care facilities that offer assisted living services and long-term care, in particular. The lack of proper dementia-care training is grossly lagging behind. One reason is that these facilities don't want to invest in their workforces by adding such training. Many use DVDs and free information provided by organizations such as the Alzheimer's Association, the Area Agency on Aging, AARP, and anything they can find on the Internet. Most of these materials are free or almost free to the user.

Free is good for the bottom line but extremely costly in all other areas. There have been dementia-care training programs adopted by many government agencies and endorsed for care facilities. The first that comes to mind is the Best Friends™ program as a model of care in dementia care that was designed to improve outcomes and increase the quality of life of those living with dementia, which failed miserably. Then came the Savvy Caregiver Program, which is also falling extremely short of what family caregivers need. More studies and efforts need to be done to create and adopt better caregiving programs to support the millions of families who are struggling. It is important that long-term dementia care in the home be treated like a job—a job inside the home.

Until governments and organizations realize it takes more than a calculator to become an accountant, we will continue to cause the cost of dementia care to increase through mistakes and lack of information. We will continue to have extremely high turnover in the care workforce due to a lack of skills and proper tools to do the work. We will eventually have to close some of these businesses that cannot provide basic services in dementia care but continue to charge high monthly fees (averaging \$7,500 to \$15,000 at the time of this writing), and yet struggle daily to do the work they promise to their customers.

I hope that the millions of baby boomers, now entering the stage in their lives at which they might need care or become caregivers, will be much more demanding for the dollars they have to pay for such inadequate, substandard, and unacceptable care. The time has come to revisit assisted living services and long-term care facilities to ask one question: How can this be better, and how can my needs be met?

Because individuals in assisted living facilities are living with an average of five serious healthcare conditions at once (such as cancer survivor, diabetes survivor, stroke survivor, dementia, mobility issues, etc.), workers and staff need to know basic care. Yet workers in these facilities receive little to no training to deal with dementia care and some of the other conditions listed above. Why?

Healthcare staff in long-term care encounters close to 80 percent or more residents living with dementia as well as some of the same conditions found in those in assisted living facilities. Yet a certified nurse assistant receives minimal dementia-care training (mostly on DVD or free material provided by organizations or found on the Internet) and just weeks of nurse-

assistant training at a community college or distance-learning certified program, and are given extraordinary responsibilities in providing day-to-day care for at least ten to twelve residents, or twenty to twenty-four residents, if working as a team of two. The sheer number at their charge seems unethical, yet we tolerate that. Why?

Creating dementia-care systems that include a solid training program for healthcare workers is extremely important, beginning with the physicians who need to learn to speak Alzheimer's all the way down to the aides who provide the day-to-day services needed for the care receiver to enjoy the best possible quality of life. It is estimated that over four million cases of neglect and abuse are reported to state government agencies yearly in the United States alone. It is difficult to find a care facility that has not had one complaint of neglect or abuse against their management team. It is time to look at our healthcare systems and ask this question: How did we set it up that way?

Taking a systems approach to solving the crisis in dementia care today is the best way to assure standardized dementia-care programs and services, minimize burnout of the health care workforce by training them to save time and stress in this important but demanding work, and improve on the overall delivery of dementia-care services for everyone. Families need support as they are the frontline workers in dementia care and must remain an important part of the overall interprofessional health care team to the very end.

Incentives for owners and operators of assisted living facilities must become, or at least remain, profitable in order to meet the rapidly growing needs of baby boomers living in such facilities. In 2006, baby boomers started turning sixty,

and this population alone will demand efficiency and quality for those who can no longer live independently. Care/social communities that combine assisted living and nursing services make more sense to baby boomers than today's nursing home model. Such a higher-quality product appeals to the generation of baby boomers who have money to pay for their preferences or have invested into long-term care insurance.¹

It is hoped that the US federal government finds a way to subsidize long-term care for everyone. Because of an unprecedentedly large number of long-term care residents or family members responsible for someone entering long-term care, baby boomers may be the tipping point to change this system that is struggling to adopt the person-centered approach to care. Alzheimer/dementia coaches will play an important role in coaching both management and families through this historical change that is needed in order to eliminate the physical and organizational trappings of what we have known as nursing homes.

CHALLENGES AND OPPORTUNITIES

A/DC training is divided into three main sections: introduction, fundamentals, and advanced. Furthermore, it is possible for a family caregiver or any other interested individual working with persons living with dementia to obtain a certificate in just the fundamentals of A/DC as opposed to full certification. The exception is that these individuals get one less hour of instruction per week and are not obligated to complete any of the internship requirements, which take a full month in the field. However, they are obligated to attend the training and submit fieldwork for the entire yearlong training.

Individuals and professionals desiring the full certificate program to become an Alzheimer/dementia coach must identify a suitable internship site, attend every class, submit all fieldwork, and complete several internships in the semester-long program. The introduction segment of the A/DC program includes general information and discussion of what will be covered in the other two sections: fundamentals and advanced sections of the training.

Fundamentals of A/DC consist of the following areas of study:

- A. History of long-term care to understand the culture of such institutions/facilities.
- B. Elements of care and the importance of developing talent in care.
- C. Types of caregivers and stages of caregiving.
- D. Who cares for the family caregivers and the ABCX formula and double ABCX model.
- E. Stages of Alzheimer's and progression/changes in the brain/behavior.

A. History of Long-Term Care

A conservative estimate of persons living with dementia and eventually requiring long-term care is at 50 percent.¹ This is sometimes attributed to family caregiver fatigue, death of the caring spouse, or care needs of the person living with Alzheimer increasing to the point at which their needs can no longer be met at home. While long-term care, residential care, and nursing homes— all terms referring to care facilities— have become a standard form of care in the twenty-first century, problems continue to abound in such facilities and people need to be diligent after placement of a loved one in such a care facility.²



Caregiving is made of two words: care and giving. It takes the four elements of care to make it into one word. Without those four elements, they should be considered two words with two separate functions. Ideally, they become one word with one function.

Care is what a person does who provides direct care requiring skills, smarts, and confidence in their ability to make a difference to meet the needs of a care receiver. *Giving* is providing a great deal of emotional support, compassion, and empathy. This requires the ability to listen actively, counsel and comfort another, and provide companionship. The caregiver must remain vigilant to what is going on and what the care receiver needs, comfort the individual, and be aware of body language and tone of voice.

B. Elements of Care

The terms *care*, *nursing*, and *health care* are intertwined when speaking about dementia care. Feeling and exhibiting concern, compassion, and empathy for persons living with Alzheimer are necessary and encouraged in the field of A/DC.

The elements of care for a family caregiver are the same as for a professional care provider. Family caregivers are often the first line of care for the person living with Alzheimer unless the person is the last member in their family or nobody comes forward to provide care.

Caregiving for those at home must be treated as a full-time job requiring a specific job description, since each person living with Alzheimer has different needs and requirements. It is indeed a specialized form of care unlike any other. The main reason is that the care receiver is often hard to understand in the way they express themselves, which can sometimes be in total silence.

Then there are the professional care providers including management and leaders, doctors and nurses, and aides. All work long and difficult hours to meet the needs of their patients and residents. The care of anyone living with a dementia is always considered specialized care, and therefore all health care workers, from doctors to aides, need the basic training, mentoring, and refresher courses to keep up with this important work.

Alzheimer is considered by many to be the greatest health care crisis since the beginning of time. The Alzheimer's Association estimated there is a new diagnosis of Alzheimer every sixty-eight minutes in the United States. This is unlike

any other condition known to mankind, and all of us are rushing to find the best possible solutions to Alzheimer's.

Elements of Care in a Home

This section is about the care provided at home, whether by the pack leader, family members, volunteers, or home care workers. The elements are the same whether a person is at home or in a care facility. In *Moral Boundaries*, Joan Tronto identified four elements of care:

1. Attentiveness
2. Responsibility
3. Competence
4. Responsiveness³

Attentiveness is the care provider's ability to detect pain or joy of the care receiver. It is as if the care provider is standing in their shoes to feel for them. This requires a great deal of compassion and empathy on the part of the care provider; however, they are necessary to better understand the basic needs of the care receiver.

Examples are to assure nothing is in the way to either confuse or cause someone to accidentally trip on an area rug or run into a piece of furniture. Lighting is important at all times. Perhaps a salt lamp left on twenty-four hours a day helps shed light on all objects in a room, especially at dawn and at night. If the person screams, are they in pain or in fear?

Responsibility means taking charge of the care receiver's needs. It is about knowing what to do for those individuals, when it needs to be done, whether we like it or not, and finding

someone else to do it when the primary caregiver is simply exhausted.

At the very least, each care provider has a responsibility to be supportive of the care receiver and to learn how to minimize resistance on the part of the care receiver.

1. Tone of voice
2. Emotional reading of care provider
3. Appropriate touch (must be timely and individually appropriate)

Competence refers to the ability of the care provider to do the work of dementia care. This requires basic training such as speaking Alzheimer's, setting up systems that track services and appointments, and providing the best care possible within the limitations of the home and the individual medical needs of the care receiver.

Such basic care must consider the individual's natural ability and personality traits. Always be respectful of their condition as it progresses and their abilities are changing. Adaptation and keeping up with developments in dementia care at home is important in maximizing the experience of care in the home. It is simply referred to as good care.

Responsiveness is all about the level of sensitivity the care provider demonstrates to the care receiver. It is also about respecting the vulnerability of the individual living with Alzheimer's and their changing needs or condition. For example, if they are experiencing pain and the same treatment does not seem to work—responsiveness requires that the care provider consult with the physician to reassess the condition and treatment. As the condition progresses, responsiveness requires adaptation to the changes in order to provide the best

possible dementia care. An example is when the individual used to enjoy a certain activity such as counting coins, and now that the condition is progressing and eyesight is diminished, responsiveness means a new activity needs to be found to take the place of counting coins as a form of engagement.

Elements of Care in a Care Facility

This includes group homes, assisted living facilities, hospitals, and care facilities offering long-term care to their residents. Tronto identified four elements of care.

1. Attentiveness
2. Responsibility
3. Competence
4. Responsiveness⁴

Attentiveness by professional healthcare individuals is about the ability to detect the needs of both the care receiver and the family. If the professional fails to recognize the needs of the care receiver, the family is not experiencing caring for their loved one. By taking a receptive position with respect to the care receiver, they are challenged to step out of their personal preference systems (comfort zones) in order to take up that of the care receiver to better understand their needs.

Examples of lack of attentiveness are a worker ignoring the cries of a care receiver who is feeling cold or when a care receiver is screaming in pain—the worker simply continues down the hall and ignores the screams. Another example is that of the doctor who does not know how to speak Alzheimer's and asks inappropriate questions of the care receiver. This serves to raise the resistance level and does nothing to evaluate the patient.

All professional healthcare personnel have the *responsibility* to care for the care receiver in their place of work (Code of Ethics for Nurses, ANA, 2001). There is no exception to this element of care whether you are a physician or an aide. Responsibility is not about whether or not the professional care provider has a responsibility to provide the care, it is more about the extent and scope of the care provided. At the minimum, it is expected that the care provider has the responsibility to be supportive of the care receiver in their care to the best of their abilities.

Attendants must learn and adopt these basic skills:

1. Tone of voice
2. Emotional reading of care provider
3. Appropriate touch (must be timely and individually appropriate)

Competence affects the previous element of care, responsibility. It is impossible to provide the dementia care necessary without basic training in dementia care (e.g., speaking Alzheimer's, fall prevention, priming for cooperation, etc.). Competence is about the professional's ability to do the work that is required. For example, if a medical nurse hands out a pain management medication that is no longer effective for the care receiver, either due to lack of knowledge or organizational protocol, then the nurse is seen as uncaring. It is the responsibility of management to provide the medical nurse with a pain management protocol that addresses such situations. A medical nurse has a responsibility to update competence continuously. The American Nurse Association states "Continual professional growth, particularly in knowledge and skill, requires a commitment to lifelong learning." Good care and individualized dementia care comes

from competence. That means the ability to give care that is based on the physical, psychological, cultural, and spiritual needs of the care receiver and their family.

The final element, *responsiveness*, refers to the sensitivity demonstrated by the care provider. It refers to the sensitivity demonstrated by staff to both the care receiver and the family of the care receiver. A care receiver and their family members are vulnerable to each and every action or lack thereof. A simple smile, an offer of a hot cup of tea or coffee, and a simple but friendly conversation go a very long way.

In some situations, the care receiver lacks responsiveness due to analgesia (treatment to control pain) and has not been reassessed to identify an alternative pain medication. It can also occur when food or drink is not provided often enough to control diabetes and the care receiver goes into crisis or a seizure requiring hospitalization and sometimes resulting in death. It is important to constantly and diligently verify that responsiveness exists and caring needs are met.

(For more information about the elements of care, see Lachman, V. D., "Ethics, Law, and Policy," 2012, *Medsurg Nursing*, 21(2); pp. 112–116; Van Leare, L. and Gastmans, C., "A Personalistic Approach to Care Ethics," 2011; *Nursing Ethics*, 18, pp. 161–173; and Watson, Jean, "Jean Watson: Theory of Human Caring," in M.E. Parker (Ed.),

Nursing Theories and Nursing Practice, pp. 342–354.)

C. Types of Caregivers

The following terms are examples of terms used throughout the world to refer to a person providing care to someone: caregiver or care giver, care, caretaker, care provider, protector, care attendant, care partner, provider of care, and aid-in-

attendance. After much observation and listening to caregivers, I have concluded there are five major types of caregivers:

- Accidental caregiver (up to 75 percent)
- Reluctant caregiver (up to 15 percent)
- Natural caregiver (up to 5 percent)
- From the couch caregiver (less than 2 percent)
- Seagull caregiver (about 1 percent)

About 70–75 percent of caregivers, both family and professional, are accidental caregivers. These are individuals who did not plan or even expect to become caregivers. A diagnosis such as Alzheimer's was received and then someone had to become that person's caregiver. Another scenario is a person who did not have a clear career goal after high school. They can say they accidentally chose a career as a caregiver to be able to get a job in the workplace and make a living. Accidental caregivers have no preparation for such responsibilities and all their training is usually on the job. It is not a career or job they planned for or trained to perform. Rather it often falls onto them suddenly and unexpectedly.

One of the largest demands in employment at the moment is in healthcare and many who have as little as a high school diploma or GED choose this career path because it is a way of earning a living, feeding their family, and finding productive work. It is not something they idealized about, but that they feel they need to do simply to survive and provide. An example is someone caring for a parent who is suddenly diagnosed with Alzheimer's, Parkinson's, or vascular dementia and is in need of care on a daily basis.

Another 10–15 percent of caregivers, both family and professional, are reluctant caregivers. These individuals have suffered either physical, emotional, or psychological torment, or all three, at the hands of the person(s) for whom they now must provide care either at home or at a care facility. In the case of a family caregiver, this person may be the only logical person to provide such care in the family and therefore they have been *appointed* to do so out of necessity and obligation, but not by choice. Many reluctant caregivers are employed in larger facilities, again because the demand for caregivers in health care is high, and it's fairly easy to barely make a living in this field.

Examples include someone caring for a mother-in-law who was never welcoming or accepting, or a grandfather who was sexually abusive or was a serious alcoholic, or an elderly parent who showed favoritism to another sibling and/or regularly abused the caregiver when they were younger. The caregiver, who in spite of some horrible past circumstance, simply cannot walk away from the care receiver's needs—perhaps in hopes of pleasing them in some way or to be accepted by them. These caregivers walk around with broken hearts and broken spirits.

Only about 2–5 percent of all caregivers, both family and professional, are natural caregivers. It feels natural for them to simply provide the care, and they know exactly what needs to be done at the time it needs to be done. They remind me of water on a duck's back; it simply seems effortless, and they always have a positive attitude no matter what.

These individuals are rare, but it is always inspiring to see them in action because they have such love and spirit for life that nothing seems to dampen their determination to make a positive difference. They instinctively know when

to take a break and do not feel any guilt about leaving their loved one with a hired caregiver, but often prefer that another family member take their place if at all possible. They have an instinctive concern for the care receiver's needs and don't relocate the care receiver if at all possible. Other traits they have are organization and planning. It is part of their natural makeup and nature to plan everything and be super organized. *Routine* is just another word for *ordinary* to them.

Natural caregivers are also found in the service of others in facilities such as assisted living and long-term care facilities. They generally smile at everyone they meet and readily offer to help team members with care of residents or patients without being asked.

Examples include the spouse in a long-standing relationship who is now a full-time caregiver, a grateful daughter or son of a loving parent suddenly in need of daily care, and a caregiver who genuinely understands that their path in life is to provide support to others whether it is physical, emotional, or spiritual.

From-the-couch caregivers estimated at less than 2 percent (also known as FTC caregivers) are known for providing caregiving from a distance. They do not physically provide direction for their loved one, but always from a distance, rarely in person, and with detached emotions. Others provide the most important care such as the emotional, physical, and medical needs of the individual for the caregiver who is absent.

One characteristic of the FTC caregiver is a reluctance to accept the reality of the care needed or the degree of progression in the condition of the care receiver. They often live in denial for months or years, only to awaken and suddenly realize the seriousness of the condition after the person living with the condition has died. The caregiver might be heard saying, "I

did not realize how sick she was, and nobody told me she was dying.”

Seagull caregivers about 1 percent of millions of caregivers are just what the word implies: they fly in once in a while, s—— all over everyone, including the care receiver, and fly back out. They leave a nasty taste in the mouths of everyone concerned and are not at all effective, supportive, or encouraging. They are often waiting and hoping the care receiver will die sooner than later so they will be free to travel as they please without having to make a pit stop.

The seagull caregiver creates both drama and additional work for those left with the hands-on care. However, this caregiver wants control of every decision and pursues every possible medical control through legal means such as a power of attorney or guardianship of the person living with Alzheimer's. They like to make an impression, but it is rarely a good one or a helpful one for the person living with Alzheimer's.

They have limited knowledge of the condition and rarely read research to become informed in order to make meaningful decisions. Instead, they either pretend they do or make a lot of noise to keep others at a distance so as not to be discovered. These individuals are preoccupied with vanity, prestige, power, and personal adequacy. They lack empathy and compassion.

All five types of caregivers are found in homes providing family caregiving to someone. They are also found in other settings such as assisted living facilities, long-term care facilities, hospitals, etc. They all experience the stages of caregiving but at different degrees and for different lengths of time in each stage of caregiving.

Family caregivers have three major roles: carer/caregiver/pack leader, provider, and protector of the person living with Alzheimer's. These responsibilities often begin even before the diagnosis and end, at the time of death. Professional care providers have two major roles: carer and protector.

In this context, *care* refers to the reciprocal relationship between the care provider and the care receiver. Care must always meet the care needs of the care receiver without abuse of power or neglect occurring, especially paternalism.

Provider refers to someone meeting the needs of the care receiver in all that they may need including food, lodging, clothing, hygiene, medical attention, socializing, and engagement activities, according to their abilities.

Each person living with Alzheimer's or dementia needs protection from those who ridicule their behavior and from a society that is not yet Alzheimer's friendly. Bullying is not as rare as you might think when someone is confined in a care facility and living with dementia. It is the responsibility of the staff to protect each resident and that of the family caregiver to report any such behavior.

Some facts about care receivers taken from "Women and Caregiving: Facts and Figures":

- Sixty-five percent rely on family and friends for their care
- Thirty percent of families will require outside help such as home care workers
- Zero percent of elders will require long-term care
- Seven percent of elders have long-term care insurance⁵

Women provide the majority, up to 75 percent, of informal care to spouses, parents, parents-in-law, friends, companions,

and grandparents, whether at home or at a care facility, and are on average forty-six years of age.⁶

According to Michael Bloom, it is possible to overcome the conflict of finding yourself an accidental or reluctant caregiver, which comprise the large majority of caregivers, and find a way to reach the level of acceptance.⁷ Fear-driven care is never effective or healthy for either the caregiver or the care receiver. Bloom encourages caregivers to learn ways to release their fear, sharpen their caregiving expertise, communicate more effectively, and begin the process of caring without regret.

Depending on the type of caregiver, the stages of caregiving vary in intensity. In the case of the natural caregiver, the stages are shorter than for others. However, it is important for each type of caregiver to develop coping and even healing techniques that are effective. This is necessary in order to not only to survive caregiving but to thrive.

The different types of caregivers have different experiences of the stages of caregiving. They also experience the Kübler-Ross model of the five stages of grief differently.

1. Denial (serves to buffer the shock)
2. Anger (intense emotion expressed at others including the care receiver and even the medical community)
3. Bargaining (making a deal with God or a higher power to fix the situation)
4. Depression (sadness, regrets, and fear of eventual death of the care receiver)
5. Acceptance (allowed to feel the grief of this tremendous and impending loss)⁸

It is emotionally difficult when the care receiver is a parent, spouse, or child. The process of grieving the loss begins early

and can last for several years. For many, it is not at all unusual to begin the grieving process at the time of the diagnosis and experience it throughout the entire caregiving experience.

In addition to the support from family, friends, and neighbors, it is important for family members to attend support groups in their community in order to hear about services and tips that might help in providing better care. Support groups are often sponsored by hospitals or the Area Agency on Aging and have the effect of a first aid kit. Because caregiving progresses along with the progression of the condition, it is important to keep adding tips and ideas to better cope.

What Is a Pack Leader?

The term *pack leader* was coined by myself, the author of this book, when I founded Remembering 4 You, LLC, on November 11, 2011. It is a term that encompasses the leadership role of family caregiving within an interprofessional healthcare team. The pack leader remains in that role until the end of the life of the care receiver or until the end of their own life should they die first.

It is the most important team role within the larger interprofessional healthcare team because of personal knowledge, experience providing dementia care, and natural leadership qualities that help support decisions along the way. Such decisions, as described earlier, can include a combination or selection in the following areas: medical, legal, financial, spiritual, and social.

Ideally, a pack leader possesses or acquires Tronto's elements of care discussed earlier.

- Attentiveness (can detect needs of care receiver/family)

ALZHEIMER'S COACHING

- Responsibility (provide the care needed according to the stages of Alzheimer's and assist team members)
- Competence (if something is not helping, address problems as they arise and keep learning)
- Responsiveness (level of sensitivity to care receiver / family)

Traits of a pack leader include some of the following:

1. Nonjudgmental
2. Forgiving
3. Compassionate
4. Simple
5. Accepting
6. Loving
7. Respectful
8. Hard-working
9. Thinks independently
10. Positive attitude

Both guilt and resentment often surface when in the role of pack leader or caregiver. It is proof that we are human! To identify these emotions, it is important to accept our humanness and learn forgiveness. It is the only way to move on and continue being effective at serving the needs of a care receiver. We all must learn to let go.

Dr. Harold H. Bloomfield, a psychiatrist, developed the forgiveness processes below. They are proven methods, easy to learn, and speed up the process. Say this out loud and as often as needed in order to actually feel the release of what it is that bothers you at that moment. Never leave work or home

without first forgiving the other person and the judgment you made at the moment you were angry or resentful toward another person.

These following forgiveness processes are reprinted from *Making Peace with Yourself: Turning Your Weaknesses into Strengths* by Dr. Harold H. Bloomfield and Dr. Leonard Felder:

Forgive the Other Person

- Whenever you can, as soon as you can, forgive the other person.
- You do this not for the other person, you do this for yourself—your peace of mind and the quality of your future relationships.
- John-Roger taught a simple, but remarkably effective technique of forgiveness. (It's a good idea to surround yourself with Goodness and Light before beginning this process.)
- First say, "I forgive _____ (the person, event or thing that caused your loss) for _____ (what they did to cause the loss)." That's the first part of forgiveness.
- Then say, "I forgive myself for judging _____ (same person, event or thing) for _____ (same transgression)."
- The second part of forgiveness—forgiving yourself for judging another—is important, but often overlooked. Your *judgment* of the other person's action is what hurt you emotionally. When you forgive his or her action, you must also forgive *your judgment* of his or her action.
- It may take many repetitions of the above sentences to until the many layers of transgressions and judgments—but keep at it. You can, and will, be free. Forgive Yourself
- Whenever you can, as soon as you can, forgive yourself.

- Whatever errors, transgressions, failings, weaknesses, infractions or mistakes you feel you made to cause the loss—real or imagined—forgive yourself for those.
- The process is the same as forgiving another. Surround yourself with Goodness and light and say, “I forgive myself for _____ (the failing).” Then add, “I forgive myself for judging myself for _____ (the same failing).”
- Again, the most powerful part of the process is forgiving yourself for having judged yourself for whatever you did (or didn't do). Who (besides parents, teachers, society and nearly everyone else) said you must be perfect?
- As John-Roger pointed out, “We're not perfect. We're human.”
- Forgive yourself for being human, forgive yourself for judging your humanness, and move on.⁹

Limitations of a Pack Leader at a Distance (Often Referred to as a Long-Distance Caregiver)

These terms, *pack leader at a distance* and *long-distance caregiver*, are truly misnomers when it comes to dementia care. Many pack leaders attempt to provide caregiving at a distance. Some are as far away as the East Coast of the USA from the West Coast, or even in another country. Some are only a few hours' drive away. In either situation, this presents tremendous challenges for both the pack leader and the care receiver.

There are many reasons for the need to provide pack leadership from a distance. It may be that the pack leader is the only child or close relative but has a family of their own and a well-established professional career in another area distant from the care receiver. It may be that the pack leader is

emotionally unable to move closer and take full charge. In any case, it is a reality of life for many families today.

This presents serious limitations that need to be addressed. The very safety and care of the care receiver are necessarily compromised because of the nature of the specialized needs of any dementia-care receiver. Daily supervision, intervention, and monitoring become impossible for the pack leader at a distance, which only increases the frustrations and stress of caregiving. Furthermore, the care receiver might not feel as supported and protected as they need to feel. It is very much a case-by-case problem that needs closer examination and study.

When money is no object, dementia care can be purchased, but only a few well-trained caregivers exist in the world. Most have received on-the-job training for generalized care in existing assisted living facilities, nursing homes, or rehabilitation facilities, but not specifically for dementia care. It is therefore critical for the pack leader at a distance to be able to interview carefully and discriminate between the trained dementia-care professional and others. Here are questions they may want to consider in an interview for a home care situation:

Where did you receive dementia-care training?

How many hours was the dementia-care training program?

Did you receive a certificate of completion, and when?

Tell me more about the classes you had to take.

Do you know how to speak Alzheimer's?

Here is a daily list of tasks I would like you to perform. How would you satisfy these needs?

What is the best way for me to reach you in an emergency?

Can I call you anytime to get a report on the care being given?

What legal documentation do you need from me so you can attend to medical emergencies?

Another scenario may be a facility advertising dementia care to their residents or to their patients. This is common today as the demand is high in dementia care. Families are often desperate to find the best possible care solution for their loved one without taking the time to ask these important and life-changing questions.

In situations where a facility or organization promotes that they offer dementia care, the following sample questions should be considered by the family (and any others that are suitable to each and every person considering assisted living, long-term care placement or hospitalization):

What do you mean by dementia care?

Where did your staff receive dementia care training?

Are the management and staff certified in dementia care and from where?

Do you welcome the primary caregiver as part of your interprofessional health care team concerning treatment and activities?

What techniques are used in the case of anxiety or "difficult behavior" on the part of the person who has dementia?

Answers to these sample questions will quickly raise red flags if the person or agency is not the proper fit. Caregivers should not be surprised if the search is time-consuming or feels exhausting. The very life of their loved one is at stake, and everything they can do to assure a smooth level of dementia

care will help them sleep better at night knowing they have done all that they can.

An area that most often challenges family care providers and professional care providers is the demonstration of aggressive behavior, verbal or physical, by the person living with dementia. I would like to refer the reader to Dr. Eilon Caspi, a gerontologist and dementia behavior specialist at <http://dementiabeaviorconsulting.com> for more information. This is a critical area in dementia care not to be ignored or discounted in any way.

D. Who Cares for the Family Caregiver?

Every family caregiver should become familiar with the “Caregiver Fatigue Timeline” (See Appendix 7). Without knowledge of the timeline, a caregiver is at a 20 percent risk of dying before the person for whom they provide care. Dementia care is considered specialized care, and it is important for the family caregiver to remember the following:

- Dementia care is a full-time job. Caregivers should take regular breaks, develop a job description, and accept outside help for this 24-7 responsibility.
- Caregivers need respite.
- Caregivers need tips and training in dementia care to do well.
- Caregivers need to feel accepted and eventually rehabilitate back into society.

Family Dynamics

It is difficult to maintain stability and cohesion in a family unit once a crisis such as a diagnosis of Alzheimer is introduced

to a family system. Relationship issues can occur, especially after one or two members have been given power of attorney or other legal powers by the care receiver—or on behalf of the care receiver—also referred to as *sibling rivalry* or *family dynamics*.

Sometimes, the fact that a family member has been diagnosed with Alzheimer becomes the way the family unit maintains stability. Whatever the dynamics in a family, the pack leader is often the target of blame by other family members, especially if there is a perception that something is not being done correctly or too much money is being spent.

An Alzheimer/dementia coach assesses family dynamics early in the coaching plan. When the pack leader works a full-time job and provides care during all other hours, it can create a work/family conflict that eventually produces its own set of health and financial problems.

1. A family member is diagnosed with Alzheimer.
2. Increased tension disrupts the family system.
3. Unresolved tension results in anger and depression.
4. Failure to dialogue results in a family feud and estrangement of family members.

A family meeting should be called as soon as possible after the initial diagnosis. An Alzheimer/dementia coach can assist with the meeting, help family members plan ahead during this meeting, and have important conversations about what legacy they want to leave. Discussions at a family meeting should include enacting a power of attorney, a durable power of attorney, a living will, and estate planning with an elder law attorney as early as possible while the care receiver is still able to speak well.

Mediation is an option for a family in crisis, and the Alzheimer's/dementia coach can also lead that discussion, coach the family as a group or individually, and even act as liaison to the different parties.

An Alzheimer's/dementia coach usually follows a model for evaluating a family in crisis in order to better guide the family including these elements provided by P. Dass-Brailsford in *A Practical Approach to Trauma: Empowering Interventions*:

- The event precipitating the crisis is perceived as threatening.
- There is an apparent inability to modify or reduce the impact of stressful events.
- There is increased fear, tension, and/or confusion.
- There is a high level of subjective discomfort.
- A state of disequilibrium is followed by the rapid transition to an active state of crisis.¹⁰

Putting ideas and wishes on paper is often one of the best ways to create a plan of action for the family. Everyone gets to speak, only one person speaks at a time, and money issues get discussed along with all the other important areas. Everything becomes transparent, and the care receiver's wishes can be made known and hopefully honored by the entire family.

E. Stages of Alzheimer's

Although we still don't know how the Alzheimer process begins, it seems likely that damage to the brain starts a decade or more before symptoms become evident. During the preclinical stage of Alzheimer, people are free of symptoms but toxic changes are taking place in the brain. Abnormal deposits of proteins form amyloid plaques and tau tangles throughout

the brain, and once-healthy neurons begin to work less efficiently. Over time, neurons lose their ability to function and communicate with each other, and eventually die. Before long, the damage spreads to a nearby structure in the brain called the hippocampus, which is essential in forming memories. As more neurons die, affected brain regions begin to shrink. By the severe stage of Alzheimer, the damage is widespread and brain tissue has shrunk significantly.

Very Early Signs and Symptoms of Alzheimer's

Memory problems are typically one of the first warning signs of cognitive loss that can be due to the development of Alzheimer. Some people with memory problems have a condition called amnesic mild cognitive impairment (MCI). People with this condition have more memory problems than normal for people their age, but their symptoms are not as severe as those seen in people with Alzheimer. Other recent studies have found links between some movement difficulties and MCI. Researchers also have seen links between MCI and problems with the sense of smell. The ability of people with MCI to perform normal daily activities is not significantly impaired. However, compared to those without MCI, much older people with MCI go on to develop Alzheimer.

A decline in other aspects of cognition such as word-finding, vision/spatial issues, and impaired reasoning or judgment can signal the very early stages of Alzheimer. Scientists are investigating whether brain imaging and biomarker studies, for example, of people with MCI and those with a family history of Alzheimer can detect early changes in the brain like those seen in Alzheimer. Initial studies indicate that early detection using imaging and biomarkers may be possible, but findings need to

be confirmed by other studies before these techniques can be used to help with diagnosis in everyday medical practice.

These and other studies offer hope that someday we may have tools that could help detect Alzheimer early, track the course of the disease, and monitor response to treatments. The following are the stages of Alzheimer identified by the Alzheimer's Association:

Mild Alzheimer

As Alzheimer progresses, memory loss worsens, and changes in other cognitive abilities are evident. Problems can include, for example, getting lost, having trouble handling money and paying bills, repeating questions, taking longer to complete normal daily tasks, using poor judgment, and experiencing mood and personality changes. People are often diagnosed in this stage.

Moderate Alzheimer

In this stage, damage occurs in areas of the brain that control language, reasoning, sensory processing, and conscious thought. Memory loss and confusion grow worse, and people begin to have problems recognizing family and friends. They may be unable to learn new things, carry out tasks that involve multiple steps (such as getting dressed), or cope with new situations. They may have hallucinations, delusions, and paranoia, and may behave impulsively.

Severe Alzheimer's

By the severe stage, plaques and tangles have spread throughout the brain, and brain tissue has shrunk significantly. People with severe Alzheimer cannot communicate and are

completely dependent on others for their care. Near the end, the person may be in bed most or all of the time as the body shuts down.

What Causes Alzheimer?

Scientists don't yet fully understand what causes Alzheimer, but it has become increasingly clear that it develops because of a complex series of events that take place in the brain over a long period of time. It is likely that the causes include some mix of genetic, environmental, and lifestyle factors. Because people differ in their genetic make-up and lifestyle, the importance of any one of these factors in increasing or decreasing the risk of developing Alzheimer may differ from person to person.

The Basics of Alzheimer

Scientists are conducting important studies to learn more about plaques, tangles, and other features of Alzheimer. They can now visualize beta-amyloid associated with plaques by imaging the brains of living individuals. Scientists are also exploring the very early steps in the disease process. Findings from these studies will help them understand the causes of Alzheimer.

One of the great mysteries of Alzheimer is why it largely strikes older adults. Research on how the brain changes normally with age is shedding light on the answer to this question. For example, scientists are learning how age-related changes in the brain can harm neurons and contribute to Alzheimer damage. These age-related changes include atrophy (shrinking) of certain parts of the brain, inflammation, the production of unstable molecules called free radicals, and mitochondrial dysfunction (a breakdown of energy production within a cell).

Genetics

Early-onset Alzheimer is a rare form of the condition. It occurs in people aged thirty to sixty-five and represents less than 5 percent of all people who have Alzheimer. Most cases of early-onset Alzheimer are of familial Alzheimer, caused by changes in one of three known genes inherited from a parent.

Most people with Alzheimer have late-onset Alzheimer's, which usually develops after age sixty-five. Many studies have linked the apolipoprotein E (APOE) gene to late-onset Alzheimer. This gene has several forms. One of them, APOE 4, seems to increase a person's risk of getting the disease. However, carrying the APOE 4 form of the gene does not necessarily mean that a person will develop Alzheimer, and people who do not carry APOE 4 can develop the disease.

Most experts believe that additional genes may influence the development of late-onset Alzheimer. Scientists around the world are searching for these genes and have identified a number of common genes in addition to APOE 4 that may increase a person's risk for late-onset Alzheimer.

Environmental/Lifestyle Factors

Research suggests that a host of factors beyond basic genetics may play a role in the development and course of Alzheimer. There is a great deal of interest, for example, in associations between cognitive decline and vascular and metabolic conditions such as heart disease, stroke, high blood pressure, diabetes, and obesity. Understanding these relationships and testing them in clinical trials will help us understand whether reducing risk factors for these conditions can help with Alzheimer as well.

A nutritious diet, physical activity, social engagement, and mentally stimulating pursuits can all help people stay healthy as they age. New research suggests the possibility that these and other factors also might help reduce the risk of cognitive decline and Alzheimer. Clinical trials of specific interventions are underway to test some of these possibilities. (Please read *Grain Brain* by Dr. David Perlmutter to get more information on environmental causes of Alzheimer.)

Diagnosing Alzheimer

Alzheimer may be diagnosed only after death by linking clinical measures with an examination of brain tissue and pathology in an autopsy, but not always. But doctors now have several methods and tools to help them determine fairly accurately whether a person who is having memory problems has *possible Alzheimer's dementia* (dementia may be due to another cause) or *probable Alzheimer's dementia* (no other cause for dementia can be found).

To diagnose Alzheimer doctors may want to run tests. (For more on testing, see Lord, E. G., *How in the World... And Now What Do I Do? A Primer for Alzheimer's: 12 Major Points for Coping Better*, 2013, Remembering 4 You, LLC, Mapleton, ME; contact the author for a copy of the primer, lordethelle@gmail.com) These tests may be repeated to give doctors information about how the person's memory is changing over time.

Early, accurate diagnosis is beneficial for several reasons. It can tell people whether their symptoms are from Alzheimer or another cause such as stroke, tumor, Parkinson's disease, sleep disturbances, side effects of medications, or other conditions that may be treatable and possibly reversible.

Beginning treatment early in the disease process can help preserve function for some time even though the progression of Alzheimer cannot be changed. Having an early diagnosis also helps families plan for the future, make living arrangements, take care of financial and legal matters, and develop support networks and systems.

In addition, an early diagnosis provides opportunities for people to get involved in clinical trials if they so desire. In a typical clinical trial, scientists test a drug or treatment to see if that intervention is effective and for whom it would work best. There are many benefits from such clinical trials in actually improving quality of life.

Participating in Clinical Trials

People with Alzheimer, MCI, a family history of Alzheimer, and healthy people with no memory problems and no family history of the disease may be able to take part in clinical trials. Participants in clinical trials for Alzheimer help scientists learn about the brain in healthy aging as well as what happens in Alzheimer. Results of clinical trials can lead to improved prevention and treatment approaches. Volunteering to participate in clinical trials is one way to help in the fight against Alzheimer.

The National Institute on Aging (NIA), part of the National Institutes of Health (NIH), leads the U.S. federal government's research efforts on Alzheimer's. NIA-supported Alzheimer's Disease Centers located throughout the United States conduct many clinical trials and carry out a wide range of research including studies of the causes, diagnosis, and management of Alzheimer's. NIA also sponsors the Alzheimer's Disease Cooperative Study (ADCS), a consortium of leading

researchers throughout the U.S. and Canada who conduct clinical trials on promising Alzheimer treatments.

To find out more about Alzheimer clinical trials, talk to your healthcare provider or contact NIA's ADEAR Center at 1-800-438-4380. Or visit the ADEAR Center clinical trials database. You can sign up for e-mail alerts that let you know when new clinical trials are added to the database. More information about clinical trials is available at www.ClinicalTrials.gov. (Also see "For More Information on Clinical Trials and Support" below.)

The latest clinical trial concerning the benefits of coconut oil for Alzheimer sufferers is presently being conducted at the University of South Florida with Dr. Jill Smith heading this important pilot. For more information on this pilot, contact jsmith10@health.usf.edu or visit <http://www.health.usf.edu/byrd/enrollingstudies.htm>.

Treating Alzheimer

Reality is that Alzheimer is a complex condition or disease, and it is unlikely that any single intervention will be found to delay, prevent, or cure it within the foreseeable future. That's why current approaches in treatment and research focus on several different aspects including helping people maintain mental function, managing behavioral symptoms, and slowing or delaying symptoms:

Maintaining Mental Function

Four medications are approved for treating Alzheimer's by the U.S. Food and Drug Administration. Donepezil (Aricept), rivastigmine (Exelon), and galantamine (Razadyne) are used to treat mild to moderate Alzheimer (donepezil can be used

for severe Alzheimer as well). Memantine (Namenda) is used to treat moderate to severe Alzheimer. These drugs work by regulating neurotransmitters (the chemicals that transmit messages between neurons). They can help maintain thinking, memory, and speaking skills, and certain behavioral problems. However, these drugs don't change the underlying disease process, are effective for some but not all people, and may help only for a limited time.

Managing Behavioral Symptoms

Common behavioral symptoms of Alzheimer include sleeplessness, agitation, wandering, anxiety, anger, and depression. Scientists are learning why these symptoms occur and are studying new treatments— drug and non-drug— to manage them. Treating behavioral symptoms often makes people with Alzheimer more comfortable and makes their care easier for caregivers.

Slowing, Delaying, or Preventing Alzheimer Alzheimer's research has developed to a point at which scientists can look beyond treating symptoms to think about addressing underlying disease processes. In ongoing clinical trials, scientists are looking at many possible interventions such as immunization therapy, cognitive training, physical activity, antioxidants, and cardiovascular and diabetes treatments.

Reversing the Symptoms of Alzheimer

There have been success stories reported on reversing the symptoms of Alzheimer. It is possible to reverse for a period of time symptoms such as degraded speech, understanding, and movement. The best source of information on this is *Awakening from Alzheimer's* by Peggy Sarlin. Coconut oil, Bacopa tincture,

phosphatidylserine (PS), lion's mane, Prevagen, and many other supplements can help. Nothing can reverse the actual condition, which eventually destroys all hope of survival.

Supporting Families and Caregivers

Caring for a person with Alzheimer can have high physical, emotional, and financial costs. The demands of day-to-day care, changing family roles, and difficult decisions about placement in a care facility can be hard to handle. Researchers have learned much about Alzheimer care, and studies are helping develop new ways to support dementia care.

Becoming well-informed about Alzheimer is an important long-term strategy. Programs that teach families about the various stages of Alzheimer and about flexible and practical strategies for dealing with difficult caregiving situations provide vital help to those who care for people with Alzheimer's.

Developing good coping skills and a strong support network of family and friends are important ways that caregivers can help themselves handle the stresses of caring for a loved one with Alzheimer. For example, staying physically active provides physical and emotional benefits.

Some Alzheimer caregivers have found that participating in a support group is a critical lifeline. Support groups allow caregivers to find respite, express concerns, share experiences, get tips, and receive emotional comfort. Many organizations such as those listed in "For More Information on Clinical Trials and Support" sponsor in-person and online support groups across the country. There are a growing number of groups for people in the mild stage of Alzheimer and their families. Support networks can be especially valuable when caregivers face the difficult decision of whether and when to

place a loved one in a nursing home or assisted living facility. (For more information about at-home caregiving, see *Caring for a Person with Alzheimer's Disease: Your Easy-to-Use Guide* from the National Institute on Aging.)

The Myth of Alzheimer's

Dr. Peter Whitehouse and Dr. Daniel George have written a book, called *The Myth of Alzheimer's: What You Aren't Being Told About Today's Most Dreaded Diagnosis*, which is also available as an audiobook. The authors argue that Alzheimer's is "scientifically unsound and socially disruptive," among other revelations.

For More Information on Clinical Trials and Support

The National Institute on Aging's ADEAR Center offers information and publications for families, caregivers, and professionals about diagnosis, treatment, patient care, caregiver needs, long-term care, education and training, and research related to Alzheimer's. Staff members answer the telephone, e-mail, and written requests and make referrals to local and national resources. The ADEAR website provides free online publications in English and Spanish, e-mail alerts, an Alzheimer clinical trials database, the Alzheimer's Disease Library database, and more. (For more information about clinical trials, go to <http://www.nia.nih.gov/alzheimers>.)

COACHING INDIVIDUALS

Go confidently in the direction of your dreams!
Live the life you've imagined. As you simplify your life,
the laws of the universe will be simpler.

—Henry David Thoreau

There has probably been no greater need for Alzheimer/dementia coaches than there is today. Care facilities need to create desirable living conditions in which “Residents are healthy and happy. Families are involved. Team members are engaged. Resident turnover is low. Staff turnover is low and occupancy is high.”¹

Management, staff, and even family members have a need for coaching to be able to sustain the demanding task of providing care for someone who is living with Alzheimer. A/DC often includes group coaching, especially for organizations. However, many prefer individual coaching that allows them to explore problem areas not directly related to caregiving that can also be dealt with during A/DC sessions.

Issues that can be dealt with in coaching include: learning to speak Alzheimer, adopting systems that work best for

dementia care, one's legacy, caregiving issues that can lead to regrets after the person is gone, family crisis and conflicts, and engagement with the person living with this condition. A/DC is much the same as life coaching in that it involves the formation of an alliance between the coach and the client(s). The coaching relationship is designed to give all the power back to the client. It is about asking the right coaching questions at the right time. The distinction between life coaching and A/DC is simply that often the A/DC also becomes a consultant in order to introduce new information or guide the client to a source of information.

A/DCs are trained to use a great variety of tools and techniques to empower their clients to find answers within themselves or at a reliable source. Getting everyone to cooperate and form teams around those in need is a very large task for coaches. It requires basic coaching training, a certificate in Alzheimer/dementia coaching, and a will to be part of changing the course in dementia care.

Beginning with families and family meetings, coaches must be able to guide family members in determining what they want for their loved one, who is living with Alzheimer. Caregiving without regret is possible.² All caregivers want to provide the best care possible. Often caregiving begins with lots of anger and resentment for the responsibility at hand. Some have difficulty moving from that point to caregiving without regret.

In chapter 8, I described the five types of caregivers. Both family caregivers (informal care providers) and professional caregivers (formal care providers) fall into these categories. The Alzheimer/dementia coach has the tools to identify and coach people in each of these categories of caregiving.

Dementia care increases in intensity as the condition progresses. In her doctoral research, C. M. Gallogly identified seven stressors that gradually increase in intensity for caregivers, leading to a feeling of hurt and depression:

1. Lack of communication between spouses.
2. Stressful medical interactions.
3. Care receiver dependency.
4. The need for assistance with activities of daily living (ADLs).
5. The situational stress of the external environment.
6. Care receiver health.
7. Loss of intimacy.

Gallogly further identified four axial codes leading to the *tipping point* in caregivers:

1. Caregiver's stress.
2. Caregiver's need to be alone.
3. Caregiver's anger.
4. Caregiver's health.³

There are five levels associated with caregiving. They vary in duration according to the individual caregiver. Counseling is necessary for a caregiver to be able to reenter society and become free of all effects of caregiving. Coaching helps shorten each level and effectively find a resolution. In the end, the goal is to be rehabilitated from one's journey as a caregiver in order to assume some form of normalcy within society. Levels of caregiving:

- Level I—Getting started as a caregiver

- Level II—Finding help with caregiving
- Level III—Heavy care and coping
- Level IV—Letting go
- Level V—Rehabilitation

Level I—Getting Started

Family caregivers must recognize the responsibility they were given and begin to look at adopting systems that simplify that responsibility. The keys to success are routine and minimizing the stress of caregiving. Staying calm in the midst of a hurricane is probably the most important trait to develop. This allows the caregiver to stay the course and reassure the care receiver that all is in control (when in fact it is never the case with dementia care). Relationships with other family members change as caregiving begins. The key is to share information and stay transparent so others can also accept the changes that are sure to come with the progression of Alzheimer.

Professional caregivers must recognize the impact of caregiving on their profession and on their lives outside of work, learn how to be an effective professional caregiver, and discover personal and professional information about the resident who needs their care. Forming an alliance and inviting family caregivers into the larger interprofessional team is critical to the success of care.

Level II: Finding Help

Everyone goes through some hard times at some point. Life isn't easy. Those who are the strongest are usually the most sensitive. Those who exhibit the most kindness are the first to get mistreated. Those who take care of others all the time are usually those who need it the most. The three hardest things to

say are “I love you,” “I’m sorry,” and “Help me.” Sometimes you have to look past someone’s smile and happy demeanor to see how much pain they are in.

An estimated 75 percent of all care receivers need long-term care placement due to the pack leader becoming exhausted, ill, or dying first. Other family members and professional caregivers need to share useful information in order to provide person-centered care. The pack leader has both a need and a responsibility to be present in the life of the care receiver until death.

Professional caregivers, therefore, can learn a lot from the family caregiver. Such information leads to quicker adaptation, better quality of life and care, and assuring person-centered care. Getting to know a new resident is important for everyone involved but most of all, is critical for the care receiver.

All caregivers should learn about how best to get help for themselves. Remember that individuals with Alzheimer have basic needs: physical, emotional, and spiritual. How these individuals get to know their caregivers is through their emotions, so the energy the caregiver brings into their space is what they react to. One question every caregiver needs to ask daily is: “Did I please the care receiver today?” and strive to fulfill that need for all care receivers.

Level III—Heavy Care

Burnout and injury can occur while providing heavy-duty care that requires lifting and should be prevented. Safety is first. Find ways to minimize falls and injuries.

Caregivers should work smartly by anticipating problems and finding solutions that solve the problems as they arise. Experimentation is common for family caregivers who have

absolutely no dementia-care training. They should proceed with caution, get their rest, take regular breaks, take at least one weekend off a month and a week off every six months, and treat caregiving like a job and not a dead-end journey.

For every problem that a professional caregiver uncovers, they should provide at least one solution to their team or to management. They should learn to be a team member within the larger interprofessional healthcare team. The family caregiver can be their best friend and not their enemy.

Caregivers should practice a relaxation exercise daily, or as often as possible. Go to <http://web.stanford.edu/group/falun/eng/start.htm> for information about a wonderful exercise called Falun Dafa and then watch this video: https://www.youtube.com/watch?v=bYdk-lNcj_w. They might want to meditate daily by reading or praying. Breathing deeply and using essential oils such as lavender helps calm anxiety and reduce stress.

Caregivers should learn to forgive themselves and others on a daily basis. This allows them to move on from one difficult moment to another without the burden of guilt or anger toward others. (See Bloomfield and Felder's forgiveness exercises in "Making Peace with Yourself: Turning Your Weaknesses into Strengths" in chapter 8.)

Level IV—Letting Go

Grief begins the first day the diagnosis is made and continues after the person dies. A grief that lasts so long is not referred to as complicated grief. Instead, it is a form of symbiosis that creates emotional problems due to long-term isolation and sadness.

Find a good therapist who is aware of *commensal symbiosis*, defined as “A type of relationship where one (person) benefits greatly from the symbiosis. The other is not helped but is not harmed or damaged from the relationship. In other words, this is a one-sided symbiotic relationship.”⁴ (See Appendix 8 for the stages of symbiosis.) If you are a professional aide or caregiver, allow yourself to grieve when a resident dies, care for yourself, and have a plan for sharing your feelings with a group of other professionals at work. Every facility has at least one licensed social worker trained in group work.

In spite of all the stressors, increased responsibilities, and changing nature of such care, relationships between family caregivers and their loved ones remain meaningful to the end. Caregivers should remember to ask for what they want, say how they feel, and not do what they don't want to do. (More on this in Part IV under “Theories and Transactional Analysis”).

Stage V—Rehabilitation

After months or years of isolation providing specialized care at home, rehabilitation into society is necessary for a family caregiver. Rehabilitation helps prevent further isolation from society. The family caregiver can find they have coped by either losing or gaining weight due to the gruesome physical and emotional demands of daily caregiving. An exercise program, as simple as walks in nature, can satisfy both physical and emotional needs. It is a good idea for caregivers to join a gym and schedule time for themselves.

Therapy or coaching helps promote a healthy and successful rehabilitation process. Caregivers taking responsibility for their own well-being has never been more important and necessary. They may discover that selflessness is the best possible way

to recovery. In her groundbreaking qualitative study, C. M. Gallogly used the term *dementia couple* to describe the symbiotic relationship imposed by the nature of caregiving.⁵

The rule of thumb on the duration of rehabilitation is that it should last the same amount of time the caregiver devoted to full-time caregiving. If the care receiver is placed into a long-term care facility or their life ends, the family caregiver can begin the rehabilitation stage at this time. For example, if the person has provided three years of full-time caregiving, it will take three years to complete rehabilitation and reenter society in a healthy way.

Part 4



FUNDAMENTALS OF A/DC

“Nurture your mind with great thoughts, for you will never go any higher than you think.”

— Benjamin Disraeli

FUNDAMENTALS

After providing a great deal of information in the introductory module of the ICA's A/DC training program, the following modules go deeper into the fundamentals, widening the scope of dementia care and coaching. The first internship of two occurs at the end of the fundamentals when students have secured a site at which to practice and have sufficient basic information to feel confident about recording their observations in the field.

Earlier in this book, we looked at some basic dementia-care needs and the skills necessary to reduce stress for the caregiver and improve the quality of life of the care receiver. At present, care in the home provided either by family caregivers or home care workers does not meet basic standards of dementia-care training, such as speaking Alzheimer, taking a systems approach to dementia care, understanding the legal responsibilities ahead for a family caregiver, and so much more.

The same problem is evident in assisted living facilities and long-term care facilities in that staff lack basic dementia-care training and skills. A/DCs are trained to create dementia-care training programs that meet the organization's needs. They can

coach management and teams in doing better by providing adequate care to those living with dementia.

Internships and fieldwork are invaluable in developing confidence and experience while studying to become an A/DC. To that end, the program includes two internships. The first requires that students document their observations of the selected site and complete other specific requirements. The second and third internships include actual application of techniques learned in the program, documenting the results, coaching an individual or a group of individuals, recording the coaching sessions for evaluation, and successfully completing all the requirements of the A/DC course of study.

A CLOSER LOOK

It is possible, provided there are dementia-care skills in place, to be hard-pressed to witness behaviors caused by dementia. By speaking Alzheimer's alone, stress for caregivers and resistance on the part of care receivers can be eliminated. A method invented by Penelope (Peggy) Garner, described in *Contented Dementia: 24-Hour Wraparound Care for Lifelong Well-Being* by Oliver James, needs to be a large part of any training program. *Contented Dementia* is an indispensable handbook for training A/DCs.

Garner determined that there are three important caregiver guidelines in speaking Alzheimer's:

1. Stop asking questions (the facts may not have been stored and therefore not readily accessible).
2. Never disagree or interrupt the individual (what they choose to use to express themselves is important at that moment, and their questions are highly significant containing important clues for the care provider).
3. The person living with Alzheimer's is the expert in the field of dementia (they live day in and day out with the condition with mild to serious levels of confusion or disorientation).¹

Furthermore, Garner recommends the following guidelines to the person living with Alzheimer as soon as it is possible for them to adopt or put in place:

1. From now on, don't worry—stay with what you already know and love.
2. Appoint the person you most trust as your advocate and ask them to read *Contented Dementia* while you get on with your life.
3. Forget all about the diagnosis—you're very good at forgetting now, so use this skill to forget about dementia and get on with enjoying your life once more.²

Coaches learn how to effectively eliminate the stress caused by repetitive questions from the care receiver. They also learn how to engage the person by using a method called *ping pong*. Finally, they learn how to find enjoyment out of living with dementia by discovering repetitive activities and routines that are truly enjoyable for that individual. James's work changes the way care is provided and recommends a structure that works for everyone.

Garner's SPECAL sense (SPECAL stands for Specialized Early Care for Alzheimer) is a positive, simple, 24-7 method of managing dementia rooted in person-centered care. The use of photograph albums is ingenious in staying up to date with the progression of this condition, but one must be sure to remove photos that contain images that the person with dementia will not recognize. These are known as *red* photos in dementia care. Red photos increase confusion and even resistance in the person living with dementia. They remind them that they are forgetting important information. *Green* photos are like green lights— they contain images that are familiar and do not cause pause or puzzlement.

SPECAL sense is so general that it applies to any person living with dementia. The key is to move from general to specific in order to adapt the program to a person-centered approach. A family caregiver can then easily take it from home into long-term care to make the transfer seamless. (For a more detailed PowerPoint presentation about this important and groundbreaking method, go to [http:// www.chc.brookes.ac.uk/images/events/dementia-studyday/penny-garner_lifelong-wellbeing-using-the-specalmethod.pdf](http://www.chc.brookes.ac.uk/images/events/dementia-studyday/penny-garner_lifelong-wellbeing-using-the-specalmethod.pdf).)

When symptoms of Alzheimer have been reversed, the person uses the same expressions and feelings as they did before the diagnosis and has a greater understanding of his or her surroundings. Anyone can provide SPECAL care in their home and in long-term care—if they keep an open mind. An A/DC can coach caregivers to use this method effectively and effortlessly. The goal in dementia care must always be to provide for and protect that individual. It is never to demean or ridicule them by laughing at their behavior or what they say.

Since Alzheimer is not well understood yet and is an incurable condition, we must do all we can to improve the quality of life and care of these individuals. Training, education, and management of care are here to stay. We also must be willing to consider using natural supplements that can reverse some of the symptoms or at least improve the quality of life. There are also healing techniques that complement natural supplements to further support the needs of these individuals and their families. Because there is not a known cause or cure, it is important to continue our quest to improve the life standards of these individuals, because by improving their quality of life, we are in fact improving the quality of work for all those providing such important care.

ALZHEIMER'S COACHING

Perhaps it is no coincidence that the word “dementia” is misspelled in this image. It is used here to demonstrate the distinction between “dementia” (umbrella) and Alzheimer and dementias that fall under the dementia umbrella. It is similar to the word “cancer” (umbrella) and breast cancer and colon cancer, etc. that all fall under the cancer umbrella. Nevertheless, SPECAL sense is easily adapted to any form of dementia.

CARE BEWARE

Beginning with family caregivers, support and training for the frontline *warriors*, as they are known in Australia, are essential to protect the work involved and even to protect the life of the caregiver. Tension headaches, frustrations, and desperation are often the result of the lack of knowledge that we all have when learning to provide such care. There are so little information and training available that it is as if caregivers are being fed to the Alzheimer's wolf. Simply learning how to get the person to take a shower or brush their teeth is easy, yet many caregivers argue and insist until they themselves let go out of sheer frustration.

The History of Caregiving

This is the age of enlightenment in dementia, shared with the age of Alzheimer. Yet not enough has changed in the care of our elderly, especially in dementia care. We cannot provide the same care for Alzheimer as we traditionally done for other long-term conditions. Although there is now a curriculum in elder care and a specialty of gerontology, we still do not have a dementia care curriculum in schools and colleges, or a universal standard for dementia care.

Dr. Alzheimer's work with senility was the beginning of dementia care in the nineteenth century. The term "Alzheimer's disease" (AD) was adopted in 1912. In 1954, psychologists began to make a distinction between the normal aging process and the physiology of dementia. In 1955, the association between changes in the brain and senile dementia was established.

It is difficult to understand how our healthcare system continues to treat these individuals living with dementia in inhumane ways by not adapting to the needs of this very large population and their families. The medical community views Alzheimer as "a process of cognitive loss...considered a normal and untreatable process of aging."¹

It is impossible to know what goes on in homes and home care, but it is safe to say that there are millions of sufferers with caregivers who are undertrained, overtired, and stressed and/or depressed. Too many people living with dementia are living with substandard care and less than acceptable levels of communication, even in our institutions. Once in long-term care, individuals living with Alzheimer or another dementia are often told they can never go back home. This does not help minimize resistance. In fact, it increases behaviors of agitation and even aggression—all because of a lack of dementia-care training. Treating those living with Alzheimer or another dementia as though they were children is totally unacceptable.

Commonsense dictates that education can strengthen dementia care, lower stress, increase the quality of life and care, and save healthcare dollars by providing better services, faster, and with less effort. We all need an Alzheimer-friendly approach to treatment and care. The inhumane treatment of these individuals needs to end now. Public awareness and dementia care training works. Let this need start with the

ICA dementia care training, Teepa Snow's Positive Approach™ to dementia care, and the UK's Purple Angel movement cofounded by Norman McNamara, to give only three examples.

(For more information about the history of Alzheimer, see McKenzie, M., "Dementia and Humane Eldercare: A History of Dementia Care in the Age of Alzheimer's," *American Educational History Journal*, 2004, 31(2), pp. 195–201.)

It is important to adopt *dementia-care mapping* to know when and how to access services and specialties as the condition progresses while continuing to learn about the condition itself to provide better care.

(For more information about dementia-care mapping, see Hallberg, I. R. et al., "Dementia Care in Eight European Countries: Developing a Mapping System to Explore Systems," 2013, *Journal of Nursing Scholarship*, 45(4), pp. 412–424; Halek, M. et al., "The Effects of Dementia Care Mapping on Nursing Home Residents' Quality of Life and Staff Attitudes: Design of the Quasi-Experimental Study Leben-QD II," 2013; *BMC Geriatrics*, 13(53), pp. 1–10.)

There is a special method, toothpaste, and toothbrush for brushing the teeth of a person living with dementia. A video is available to learn this at [youtube.com/watch?v=vc4hG_8t9nA&t=178s](https://www.youtube.com/watch?v=vc4hG_8t9nA&t=178s) by Dr. Natalie Archer of Toronto, Canada. And the 3rd sentence is fine. There is also a special method for getting someone into the shower that Teepa Snow teaches in one of her DVDs available at <http://www.teepasnow.com>.

Webinars are a great platform for dementia-care training because participants can ask questions and actively participate. Although the ICA no longer offers dementia care training

certification programs, Teepa Snow offers a certification dementia care program, Positive Approach to Care (more at teepasnow.com/course/trainer-certification).

Establishing a routine and sticking to it saves time, money, and energy. It is important to know how to set up a care plan, with the help of other family members, in such a way as to reduce worry and time. This includes meal planning such as freezing ready-to-serve meals for when the caregiver is simply too tired to cook. By keeping lists and charts, when visiting the doctor's office or going shopping, it is less of a worry to recall the details. Hiring someone to come in at least once a week to do light housekeeping duties is a tremendous plus. All these, and much more, are things an A/DC is trained to assist with.

More women than men are living with Alzheimer's because women live longer than men and also because women who carry the APOE-E4 gene are at increased risk of developing Alzheimer (Stanford University School of Medicine). Women are expected, if they turned sixty-five in the year 2000, to live an additional nineteen years, to age eighty-four. In 1997, 70 percent of people over age seventy-five who needed assistance with activities of daily living were women.²

Men provide as good caregiving to their spouse, parent, or grandparent as women do. However, we are now seeing more and more children becoming caregivers as the number of dementia cases rises and people live longer. It is estimated that more than 1.3 million people in the United States between ages eight and eighteen provide care to a sick family member.³

When considering a home care agency, look for the following characteristics:

1. Dementia-care training received.

2. Ability to work with the family caregiver as a team member.
3. Willingness to actively listen and offer solutions to problems.

Family caregivers have the right to examine the home care help they will receive to determine if it will alleviate their already-demanding task or add yet another layer to an already extremely exhausting set of responsibilities. Family caregivers must take an active role in the kind of help they recruit and get the help that allows them the respite they need.

Part 5



ADVANCED A/DC TRAINING

“Believe that life if worth living and your belief will help create the fact.”

— William James

THEORIES

Bernard Bass is considered the expert in the field of leadership, and *The Bass Handbook of Leadership: Theory, Research, and Managerial Applications* is considered a “bible” for every serious student of leadership. He was a distinguished professor of management at the State University of New York in Binghamton, New York, and an author. He passed away at the age of eighty-two on October 11, 2007.

The systems approach, according to Bass, “looks at the leader as someone embedded in a system with multiple inputs from the environment, the organization, the immediate work group supervised, the task, the leader’s behavior, and his or her relationship with subordinates and outputs in terms of effective performance.”¹

Transactional Analysis

Transactional analysis (TA) is an idea theory useful in the management of dementia care, both for adaptability and ease of learning. This theory, developed by Dr. Eric Berne, has proven effective in improving communication and encouraging engagement of the care receiver.

Dr. Berne's extensive study of personality and group dynamics led him to identify three distinct personality realms, or ego states, found in each individual: parent, adult, and child. By understanding the role and dynamics of each state, it is possible to resolve most crises and disputes. TA is also used in negotiating hostage situations.

I have found some of the basic communication skills of TA exceptional for getting the attention of and respectfully engaging someone living with dementia. Although I am not a certified TA practitioner, I have been a student of TA since 1992 and continue to be a paid member of the International Transactional Analysis Association (ITAA). Dr. William Krieger and Emily Ruppert, MSW, were two of my TA teachers. I used TA in my private practice as a mental health counselor, in teaching business at several universities, and now in my A/DC practice and as a trainer.

TA is easy to understand and has many wonderful applications in relationships, business, and dementia-care communications. Ian Stewart and Vann Joines published a training book called *TA Today* that is enormously practical for self-directed study. It offers the basic information necessary to begin to understand and use TA and contains exercises.

TA has allowed me to make changes in my life, in my businesses, and now in having the great responsibility of being my husband's pack leader within a larger interprofessional healthcare team. Without the knowledge I gained studying and practicing TA, I would not have been able to be as effective as a family caregiver and pack leader as I have been. I am very grateful to Bill Krieger of Albuquerque, New Mexico, who introduced me to TA through a speech he gave in Maine, USA,

in 1991, and especially to the late Emily Ruppert, who had a heart of gold and demonstrated using TA in groups.

Systems Theory

Francis Heylighen and Cliff Joslyn define systems theory as a “transdisciplinary study of the abstract organization of phenomena, independent of their substance, type, or spatial or temporal scale of existence. It investigates both the principles common to all complex entities and the (usual mathematical) models which can be used to describe them.”²

In a family unit, a systems approach means that each family member has a role to play and rules to respect. Family members respond to one another in a certain way and according to their roles, which is how patterns develop. Family systems theory was introduced by Dr. Murray Bowen, who suggested that “individuals cannot be understood in isolation from one another, but rather as a part of their family, as the family is an emotional unit.”³

Within a healthcare environment such as an assisted living or long-term care facility, individuals are interconnected and interdependent. No activity or problem can be understood in isolation from the system. Leaders are embedded in the system and therefore part of the system. The system must assume the responsibility for something good happening or something bad happening. The systems approach requires asking, “How did the system set it up for this to occur?” to fully understand and resolve problems or celebrate positive outcomes.

As healthcare management begins to be influenced by baby boomers, many of whom have been exposed to TA and use it regularly, systems thinking, tools, and theory are becoming part of many of these healthcare organizations. It’s the most

effective way to guide change in health care in the twenty-first century. In a high-functioning system, people freely exchange feedback without fear of reprimands, accusations, finger-pointing, or scapegoating. Instead, everyone ensures first and foremost that the needs of the system are satisfied.

Adopting a systems approach to dementia care is golden. Much needs to be learned about managing dementia care properly, both at home and in care facilities. The goal is to maintain open communication between all parties and keep improving the system of care. No system is perfect; however, some systems are better than others, and dementia care can be better.

One leader in the field of systems thinking is Peter Senge, who described *personal mastery* as “continually clarifying and deepening our personal vision, of focusing our energies, of developing patience, and of seeing reality objectively.”⁴ One metaphor Senge used to describe developing personal mastery is to “step back” in order to “see the forest for the trees,” but qualified that most leaders and managers have difficulty doing that.⁵ However, in a systems approach to dementia care, considering the entire interprofessional care team requires that everyone has a say, and this provides leaders with a unique vision of their forest.

A simple way to explain how systems theory can work within your organization is to look at your own body. When a lifestyle change is made that affects one organ, the entire system is affected, shifts, or breaks down completely or takes a 160-degree turn for the better. Decisions and changes within an organization must be carefully made with the entire organization, and how such changes affect each and every part of the organization, in mind. The dynamics of each decision results in an outcome for all stakeholders.

USE OF METAPHORS AND LASER COACHING

Metaphor is as ultimate as speech itself, and speech
as ultimate as thought... Metaphor appears as the
instinctive and necessary act of the mind exploring
reality and ordering experience.

— John Murry, *Countries
of the Mind*, 1931

We use metaphors as a way of speaking and expressing ourselves in everyday speech. A metaphor is a figure of speech that uses an image, story, or tangible thing to make a point quickly and clearly. For example, “Her eyes were glistening jewels” and “Lost in a sea of forgetfulness” (the title of a book by Gary LeBlanc). A metaphor can also be used as a rhetorical figure of speech for special effect via association, comparison, or resemblance. For example, “That movie was the sleeper of the summer” and “Winning the tournament was a cakewalk for him.”

The metaphors of coaching are unique, especially in Alzheimer coaching, because they invite the magical side of our personality instead of the logical thinking patterns that most people engage to approach a coaching session. The use of

metaphors in coaching is inspiring and speeds up the process. The International Coach Federation's core competency 7.5 (see Appendix 10 for the core competencies) is used by credentialed coaches to promote direct communication with clients. Since coaching is all about taking the client from point A to point B in the most expedient way, this competency provides the coach with a descriptive tool to motivate the client forward.

The main difference between similes and metaphors is that similes use comparison words such as “as” and “like” (for example, “My heart is like an open highway”). Metaphors do not use “as” or “like” (for example, “My heart is a highway”). Metaphors are a little more subtle. Here are a few examples of metaphors:

- The heart of the city
- The foot of the mountain/bed/stairs
- To give a hand
- To be snowed under
- A warm welcome
- A hail of abuse
- To see red
- A white lie
- To give somebody the green light
- Familiarity breeds contempt

For information about metaphor sets, go to “Metaphor Sets in Turn of the Screw” by Michael Kimmel at http://www.academia.edu/1607109/Metaphor_Sets_in_Turn_of_The_Screw_What_conceptual_metaphors_reveal_about_narrative_functions

The following information is from “Exploring Metaphors in the Classroom” published by Teaching English in 2004:

Improving Knowledge of ‘Chunks’, many metaphors occur not as isolated words but in ‘chunks’ of language. Some of these ‘chunks’ are idioms that cannot really be varied. Some examples are:

- to be ‘down in the dumps’
- to ‘fight like cats and dogs’
- Other ‘chunks’ can be varied but generally, occur as collocations in fairly limited combinations. Some examples are:
 - a ‘fatal mistake/decision’
 - to ‘waste time/money’

When teaching metaphors, we should encourage learners to note them down and learn them as ‘chunks’— this will help learners to remember them better and use them appropriately.

We can revise learners’ knowledge of these chunks by writing a list of chunks on the board with important words missing, e.g. “fatal” in “fatal decision”, or “cat” in “to fight like cat and dog”. Working in teams, learners should then fill in the missing words and write sentences using the chunks.

Using English creatively

As we have seen, many metaphors in English form part of the ordinary repertoire of the native speaker. We can help learners to learn some of these fixed metaphors while simultaneously encouraging them to play creatively with language. One way is to ask learners to write short poems with one of the following titles:

- Weather metaphors
 - A sunny smile
 - An icy look
 - A stormy relationship
- People metaphors
 - A chip off the old block
 - A rough diamond
 - A shoulder to cry on
 - An ugly duckling
 - A fairy godmother
- Parts of proverbs
 - A new broom
 - Early birds
 - Birds of a feather
 - Silver linings
 - A rolling stone¹

Metaphors help the coach frame meaning and provide a unique way to represent the issues. The concept of metaphors in Alzheimer/dementia coaching is gaining support as an important way to lead into laser coaching. (More about laser coaching later in this chapter.)

The use of metaphors helps the A/DC describe, visualize, and make sense of the world of dementia care. Dementia care is extremely complex and specialized— many consider this area of healthcare as exceptionally demanding emotionally, psychologically, and physically. It is complex, and metaphors offer a wonderful way to create visual images so people can gain a greater understanding of a situation or problem. An octopus evokes the image of many arms reaching out to the multitude of demands put on a caregiver at work or in the home. The metaphor of a pack leader creates the image of

someone everyone else wants to follow because of their wide range of responsibilities and knowledge of the care receiver. Metaphors are a wonderful way to portray your understanding of others.

Individuals learn differently, and therefore, it is important to use all three main learning styles: auditory, visual, and kinesthetic styles of teaching/learning. Stating a metaphor would be considered auditory while visualizing the metaphor is considered visual, like creating an image or scene for the listener. Using metaphors successfully leads to kinesthetic learning as the learner becomes totally engaged in the experience and transformation in others, like an aha moment in making a powerful connection with another individual. Success in using metaphors in coaching also largely depends on the learning style of the individual. To learn more about learning style, see the NLP (Appendix 2).

Metaphors help the coach focus on their own coaching style. By studying their coaching metaphors, a coach can reflect on the methods and practices that accompany particular metaphors and how they impact their clients. The introduction of a different metaphor can reveal a new coaching style as an effective way to implement change and shift a client.

Klaus Krippendorff's academic essay entitled "Major Metaphors of Communication and Some Constructivist Reflections on Their Use" starts with a brief theory of metaphor and then goes on to outline the six most pervasive metaphors in human communication. Here is part of Professor Krippendorff's writing on the history of metaphor:

When English speakers talk about some kind of failure of communication, they might say:

- Communication *broke down*.
- He didn't *come across as well*.
- Her thoughts were *locked in* cryptic verse.
- The message *got lost* in the process.
- You just don't *understand*.
- It didn't *compute*.
- She was screaming *against a brick wall*. There was no *chemistry*.

All of these expressions rely on metaphors and it seems one can hardly think about communication without them. Notions of metaphor have ancient roots. In his *Topica*, Aristotle urged a distinction between definitions and metaphors, arguing that one should be wary about the latter's ambiguity (Ortony, 1975:3). "Metaphor," he wrote in his *Poetics*, "consists in giving a thing a name that belongs to something else" (Sontag, 1988:5). Aristotle's suspicion that there is something devious in the use of metaphors has survived in the contemporary distinction between literal language, preferred in scientific and technical discourse, and metaphorical language with aesthetic qualities are of interest largely to literary connoisseurs and poets. This distinction privileges an observer-independent external reality as the ultimate arbiter of what can be said and how... Thus, metaphors are not hidden comparisons as Aristotle suggested and many contemporary literary analysts continue to maintain. Unlike analogies, metaphors are fundamentally asymmetrical. They are the linguistic vehicles through which something new is constructed. In Lakoff and Johnson's (1980) definition of metaphor as understanding and experiencing one kind of

thing in terms of another... It is “the terms” that create the “thing” experienced. Thus, metaphors are not mere poetic embellishments in language, they affect their users’ perceptions and actions. Metaphors are also far from ambiguous and vague. Their entailments can be traced with considerable certainty and in as much detail as desirable, leading to a reality equally clear and obvious to the user of the metaphor as to its analyst. The 18th century philosopher Gambatisto Vico already recognized metaphor as the most important manifestation of human creativity.

References: Ortony, Andrew. Metaphor: A Multidimensional Problem. Pages 1–16 in his (Ed.) Metaphor and Thought. Cambridge: Cambridge University Press, 1979.

Sontag, Susan. AIDS and Its Metaphors- New York:

Farrar, Straus and Giroux, 1988.

Lakoff, George and Mark Johnson. Metaphors We Live

By. Chicago IL: University of Chicago Press, 1980.²

Metaphors in Coaching

Here are some examples of metaphors that fit coaching goals. People often complain of a lack of movement in their lives or at work, both metaphorically and literally. Common expressions for how a person feels that reflect this lack of movement are:

I don’t feel like I’m going anywhere.

I’m stuck in a rut.

I feel like I’m going around in circles.

I’m not making any progress at all.

I'm not moving forward.
It's like I'm up to my neck in quicksand.
I feel like I'm going backwards.
It's like I'm running up against a brick wall.

And to express lack of direction:

I don't know where I'm going.
I don't know which way to turn.
I feel lost.
I lack direction in life.
I feel disoriented.
I feel like I'm going the wrong way in life.
I don't know the right way forward.

In each of these situations, “how,” “what,” and “where,” coaching questions can be applied to get movement toward a life goal. Watch this presentation by Andrew Austin for more information about the use of metaphors in coaching: <http://www.youtube.com/watch?v=BYroenA5tEI>

Symbolic Modeling

Metaphors in Mind by James Lawley and Penny Thompkins was the first comprehensive guide to *symbolic modeling* using the “clean language” of David Grove. An annotated training DVD called *A Strange and Strong Sensation* demonstrates their work in a live session. This excerpt is from a profile of The Developing Company at CleanLanguage.co.uk:

Penny Thompkins: I am an American, but I have lived most of my life in Britain. For 18 years I was a director of a company manufacturing oil field equipment, with sales offices world-wide. Attendance at a 12-step program unexpectedly triggered a train of events that resulted in me leaving

the company, leaving my marriage, and embarking on a journey of intense self-discovery.

James Lawley: Meanwhile, I was enjoying the benefits of being a senior manager in the British telecommunications industry until the day I realized that becoming a more successful businessman was not my life purpose. I left business and set off on a year's travel which included joining an expedition to look for undiscovered Mayan temples in the jungle of Belize! To my surprise I discovered a long-hidden sense of my spiritual self.

Coaching with the use of metaphors by Penny Tompkins and James Lawley, supervise neurolinguistic psychotherapists—registered with the United Kingdom Council for Psychotherapy since 1993— coaches in business, certified NLP trainers, and founders of *The Developing Company* in the United Kingdom. They have provided consultancy to organizations as diverse as GlaxoSmithKline, Yale University Child Study Center, NASA Goddard Space Center and the Findhorn Spiritual Community in Northern Scotland.

Tompkins and Lawley asked the following question:

Are you aware that your clients use metaphor *several times a minute*? And that your clients reason and act in ways that are consistent with their metaphors? And that the nature of metaphor makes it ideal for working with out-of-the-ordinary problems and high-level goals? And that Clean Language keeps coaches' (unconscious) metaphors out of the coaching process and facilitates clients' metaphors change— and as they do, so do their perceptions,

decisions and actions? If not, you need to read this article.³

Laser Coaching

Laser coaching is about getting to the heart or root of the problem as deeply and efficiently as possible so that the change for the client is permanent vs. temporary. It is about helping the client discern the truth vs. their perception which is keeping them stuck, conflicted, or struggling.

—Marion Franklin, www.lifecoachinggroup.com

Laser coaching is traditionally a short and focused coaching session of five to twenty minutes (coaching sessions are usually forty-five to sixty minutes). It can be for one client or a group of participants. It's usually very, very focused on one particular aspect of the training or coaching goal.

Laser coaching is a special coaching technique that originated with Thomas Leonard in the 1990s. It is currently taught by Marion Franklin and others in the field of coaching. It is a process by which powerful questions are asked targeting core values. When limited beliefs are revealed, the coach requests that action steps be taken for the purpose of aligning with core beliefs.

Laser-focused coaching is perfect for clients who prefer to get to the point and are willing to go deep at a fast pace. It is not unusual for a coach to interrupt the coachee in a laser coaching session because time and focus are of the essence. Sue Pearson at suepearson.ca says, "It's about identifying the change needed quickly, digging down into the heart of the

issue in a fast, meaningful and focused way. It's about hitting the bottom line right out of the gates and sticking to it till the race is done. Intense— brisk— fast and focused. Insight on speed as it were.”

Many individuals and groups do not have three to six months available for A/DC but nevertheless, want to improve on their ability to deliver the best possible dementia-care services. Furthermore, many individuals are new to coaching, the A/DC usually provides the first session at no charge. If the coach is working within an organization, he/she might quickly move into the more efficient laser-coaching approach as soon as possible after the initial session. Laser coaching is designed to be extremely effective in both speed and results.

In the words of Sue Pearson in “What Is Laser Coaching?” laser coaching might be a good fit for you, if

- you have a narrowly defined and clear focus/goal for the call;
- you know what the obstacle is but are unsure how to break up the logjam;
- you have above-average communication skills;
- you have an appreciation of the coaching process;
- you can get to the heart of the matter without too much *backstory* or explanation;
- you have an idea of the direction you want to take in the call and you know your desired outcome;
- you have had experience of life coaching and have an understanding of the coaching process;
- you have worked with me for three months or more at some time in the past. Having a previous coaching relationship with me offers added *traction* to a laser call

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because we can generally go *deeper, faster* and can rely on our past experience and connection to ground our interaction; and

- you want to experience coaching but are not ready (or able) to commit for a three- or six-month package.⁴

A twenty-minute laser coaching session can help the coachee.

- free up some energy around an issue that feels stuck and rigid;
- discover how your mood or perspective may be limiting what you see;
- gain some insight into the nature of the problem that is vexing you;
- explore options or possibilities that may be available to you and try them on for *size or fit*;
- widen your view or look at something from a totally different angle or viewpoint;
- explore the impact of assessments— your own or other people's; and
- use metaphor to creatively get a handle on a situation, emotional tone or conflict.⁵

Prevention to Intervention

Recent clinical research provides a strong evidential basis for the preferential use of lifestyle interventions as first-line therapy. This research is moving lifestyle from prevention on, to include treatment—from an intervention used to prevent disease to an intervention used to treat disease.

—American College of Lifestyle Medicine

Laser coaching is an ideal coaching technique for organizations that want to get pointed in the right direction concerning dementia management and care and save time and money. It is ideal for getting a workforce trained and motivated to provide better dementia care, achieve balance, and do more, more effortlessly and with more fun— yes, even when providing dementia care.

INTERNAL/ EXTERNAL ALZHEIMER/ DEMENTIA

Coach

Every healthcare organization and facility offering care to persons living with Alzheimer and other dementias would benefit enormously to include an A/DC as the “hub” of their dementia care management team. The coach is the hub of the healthcare wheel, who holds all the other positions in place around the wheel. The rim of the wheel is where you find the care receivers.

The business of an A/DC is similar to any other coaching business in that the coach, after certification, can be an internal coach within an organization or an external coach in a private practice. Private practices can include more than one coach or be one-coach private practice.

In an organization, a coach encounters individual coaching with employees and management, group coaching, and coaching people’s careers and mergers affecting the workforce. They can deal with any issues concerning dementia care and possess the tools to make proper references and recommendations. The

same is true of a coach in private practice when accepting single clients or groups of individuals.

Organizations often contract with an external A/ DC because of the unique ability of an external coach to take larger liberties in their coaching approach. External coaches to an organization are not limited to organizational guidelines or limitations. But an internal coach has the advantage of being an employee of the organization and therefore privy to all personnel documentation, regulations, and much more. This often simplifies and shortens the coaching contract because the coach's discovery phase is much shorter.

Since, according to WHO, there are already an estimated seventeen million known Alzheimer diagnoses in the world, the need for this coaching specialty will no doubt be driven by the numbers. The urgency to find the best possible dementia-care solutions to improve quality of life and care for each care receiver, improve the quality of work for all care providers, save limited healthcare dollars by providing more education and coaching that is currently available, and finally guide and support better management decisions are all drivers that carry powerful consequences in healthcare today.

Determining Coaching Fees

Starting a coaching business is not easy, but it is made easier by having a service niche and a certificate in that niche. Healthcare businesses are struggling to remain profitable and have been adapting current practices to dementia care. It is not working, and problems keep increasing for management, especially for care receivers.

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Alzheimer/dementia coaching is a specialty, a niche in coaching. To establish fees, coaches need to know some basic information:

- Basic expenses (travel, postage, office, virtual assistant, etc.).
- Liability insurance if in private practice.
- Marketing (attending conferences, printing, and advertising).
- Taxes (federal, state, and provincial).
- Yearly income desired.

They need to come up with a formula which incorporates all possible expenses and desired income to arrive at an hourly fee. If coaches are internal in an organization, they should negotiate a professional wage that is equivalent to that of a vice president of a department and dependent on their coaching experience, training, and certifications.

The coach can create a table recording desired income from different sources and all foreseeable expenses for their business. Barb Zeigler developed a unique value-based formula to calculate the hourly coaching fee. For more information, coaches can contact Ms. Zeigler at [linkedin.com/in/barbz](https://www.linkedin.com/in/barbz) and take the time to properly set their fees based on her formula.

All books written by Alan Weiss are wonderful investments. A/DC training teaches coaches how to adopt his proposal forms to maximize a positive outcome when submitting contracts. Mr. Weiss is renowned for his extensive consulting and coaching success. His information is used in every business school and graduate program.

Part 6



CAREGIVING ROLES

“Find a place inside where there’s joy, and the joy will burn out the pain.”

— Joseph Campbell

Each problem has hidden in it an opportunity so powerful that it literally dwarfs the problem. The greatest success stories were created by people who recognized a problem and turned it into an opportunity.

— Joseph Sugarman

INTRODUCTION

Earlier, I mentioned Joan Tronto's four elements of care: attentiveness, responsibility, competence, and responsiveness. She defined *attentiveness* as the detection of the care receiver's needs. This requires that the caregiver step out of his/her own personal preference system or comfort zone.¹

The second element is *responsibility*. Beyond accepting that the care receiver depends on the care provided to strive and to survive, responsibility is part of the caregiver's moral and ethical fabric. Dictionary.com defines responsibility as a "state of being responsible, answerable, or accountable for something within one's power, control, or management, such as saying 'the responsibility for not completing the task is yours.'"²

Tronto's third element of care is *competence*. This element refers to the individual's abilities and competence to do the work that is required, including being able to communicate effectively with the care receiver. In the case of a person living with Alzheimer's, the caregiver should be able to speak Alzheimer, recognize the stages of the condition, and assist in providing the best possible care in their area of expertise.

For example, if a nurse continues to give medication that is ineffective for the care receiver, this would fall under

incompetence on the part of the nurse and the administration that supports such practices. To give only one example, the Food and Drug Administration (FDA) and *JAMA Psychiatry* have been warning, for over a decade, the deadly effects of administering antipsychotic drugs in patients with dementia. This is commonly the practice with individuals who demonstrate behavioral behaviors such as aggression and hallucinations. Instead, proper dementia care training and education easily suffice to effectively deal with any and all dementia cases. Hospitals, especially surgical departments, often will resort to antipsychotic drugs instead of proper dementia care protocol and training. Antipsychotic drugs of any type can contribute to a higher risk of premature death, most often from cardiovascular conditions like heart failure or pneumonia, in persons living with dementia. The American Nurses Association (ANA) posited, "Continual professional growth, particularly in knowledge and skill, requires a commitment to lifelong learning."³

Tronto's final element of care is *responsiveness* of the care receiver. This element refers to the level of sensitivity demonstrated by the caregiver or care provider. Does the caregiver come into the room with a smile on their face and depart with a departing statement? How responsive is the health care team to the family caregiver, who is also a care receiver? Are they offered a cup of tea or coffee?

The level of responsiveness is directly related to the level of abuse and neglect of a care receiver. Paternalism that restricts the freedom and responsibilities of subordinates is one of the most common forms of lack of responsiveness. This behavior is often under the veil of providing what is *for their best interest* but always spills over into the direct care of the care receiver.

The key way to exemplify responsiveness is through respect. No matter what level of cognitive ability the care receiver possesses, it is vital to promote choice whenever possible. As humans, we strive to be autonomous from early childhood. The ability to make daily choices in our lives such as what to eat, what to wear, what music to listen to, and what activities to participate in is something we grow to take for granted throughout our independent adult years. Without the ability to make choices, depression can fester and the will to live can decline. The best way to respect and support a care receiver is to promote choice and independence as much as possible throughout each day.

Maturity is an essential trait in all caregivers. Maturity is best defined as the ability to do what must be done, at the time it must be done, whether we like it or not. The care receiver is often not able to appreciate the steps necessary to improve their health and safety. The caregiver must be able to make that distinction quickly and effectively and possess the confidence to communicate those needs and take action to safeguard the care receiver.

In dementia care in the home, there must be a pack leader, often the family caregiver who has the blessing of other family members, or at the very least from the healthcare team of doctors, nurses, and aides. The entire interprofessional healthcare team must look to the pack leader for direction and leadership. The pack leader must look to the interprofessional healthcare team for support and assistance in accomplishing all that needs to be accomplished to improve the quality of care and life of the care receiver. On the other side of the coin, the working conditions will be much better and the job satisfaction level will measurably increase.

FAMILY CAREGIVING (INFORMAL CARE)

Family caregivers are brave individuals who commit to a larger-than-life responsibility when they accept the responsibility of providing dementia care at home. Stories abound of interrupted marriages, lives turned upside down, and memories being relived over and over again.

The ability of a family caregiver to maintain and succeed in their caregiving journey includes:

1. Educating themselves about the symptoms and stages and adopting a care model.
2. Treating caregiving like a job with regular breaks, sick leaves, and vacations.
3. Adopting a routine and sticking to it no matter what.
4. Asking for help from family members, friends, and home care agencies.
5. Visiting their doctor regularly, taking supplements, and exercising regularly.

When a spouse or life partner is diagnosed with Alzheimer, two people are affected. The person living with Alzheimer does not lose anything, they simply misplace the information.

Sometimes, they are able to find it, and everyone can rejoice, even if it is only for a moment. The mind may forget; the heart never will. Meet three couples out of millions who are living with Alzheimer.

The Duncans

Young men will fall in love. Norman met Elsie in London when he was serving in the US Army during WWII. They had a whirlwind courtship, and after six months, they were man and wife, Mr. and Mrs. Norman Duncan, with a great future ahead of them. Elsie's dad, a loving and protective father, made sure Norman was the right person for his only daughter. He promptly got Norman to guarantee him that when Elsie went to the United States with him he would cherish her for life. After seventy-one years of marriage, Norman, as her caregiver, became even more attached than ever to his loving wife, Elsie, forever keeping his promise to his father-in-law he made all those decades ago.



Elsie took her last breath on Sunday, August 2, 2015, after she no longer could remember to swallow food and liquids.

She died in Norman's arms with all the family present. Norman was quoted as saying, "God bless her soul. She was a diamond and my total reason for living. I am ninety-six, and I cannot wait to join her." Norman is a health advocate and the first chairman of the board at ICA. His grandson is Dr. Christopher Barden. Dr. Barden is a recognized Alzheimer's researcher and leader in his field in Canada.

The Amrines

On Sunday, May 25, 2014, *CBS Sunday Morning* broadcast the moving story of Melvyn and Doris Amrine, married sixty years. Just prior to Mother's Day, Melvyn walked out of their home and got lost. He is living with Alzheimer's. The Amrines represent millions of couples with one common life experience: living with Alzheimer's. Doris believed she had lost her husband to Alzheimer's, and her marriage was interrupted by this condition. You can see the touching Amrine story at <http://www.cbsnews.com/videos/when-love-becomes-an-instinct/> or on YouTube at <https://www.youtube.com/watch?v=JbqUJNAlqIY>.



This weekend, while watching the Memorial Day ceremony in Washington, DC, on television with my husband, who is living with Alzheimer's, by my side, I noticed his head was reclined and his eyes were shut. All of a sudden, the Navy's military marching song came on. He lifted his head and opened his eyes, and then began to weep. His father was in the Navy in WWII, and that memory flooded his senses— proof that in spite of Alzheimer's, the heart stores the most important memories and feelings.

Love is an instinct that even a person who is living with Alzheimer's retains to the end. We just need to look into their hearts to find it. Maybe living with Alzheimer's and looking into the person's heart is what it is all about!

The Bakers

Donna was diagnosed with early-onset Alzheimer's at age forty-eight, and her husband, Nick, became her caregiver. They live in Phoenix, Arizona. Donna was quoted as saying, "People have to pay attention. It's not an old person's disease. It destroys who you are and everyone around you. It kind of takes away everything that makes you, you. I am losing my life with my husband. He's losing me a little at a time." For the complete story, please go to <http://archive.azcentral.com/12news/news/articles/20120121alzheimers-disease-early-onset.html>.



Donna and Nick Baker

The article describes the first signs of Alzheimer, findings ways to cope with Alzheimer, and how Alzheimer means more than memory loss or forgetting. Most of all, it is about adapting to Alzheimer because this condition defines the present, alters the future, and makes the past the solace for families.

The Deeper Meaning of Dementia

I have often wondered about the reason why there are so many cases of dementia in the world with seemingly no end in sight. So many families and so many individuals are affected by this condition that can be as short as a few months or lasts for several decades. Is it possible that a greater power is protecting these souls from the increasing devastations due to terrorism, wars, and conflict around the world? With dementia, only the present matters and even then, it is not always a clear present as we know it. The past and future fade as the condition progresses. When the end comes, as everything has a beginning and an end— it has been said that many will perish and many

will fear the end— these people with dementia have already been exempt from fear of a catastrophic end to life on earth.

Instead, people living with dementia often live in a safe place, a safe world of their own. They communicate with spirit and travel all the universes. They know no boundaries and experience life in several dimensions— not just three as most humans do. They are here to inspire us and guide us spiritually to the true meaning of life— to love and be loved; to protect and be protected; to provide and be provided for. This is very different from the greed among our people and nations today. The gap between the super rich and the super poor only serves to create a black hole for the working middle class that feeds the rich and provides for the poor.

From Family Caregiver to Pack Leader

Here is a story I wrote that was published on LinkedIn on August 27, 2014, based on a real situation, and how the pack leader's decision played an important role.

A True Story about the Quality of Life A man had been waiting for two hours to speak with his wife's new physician at the nursing home. The new physician was a young doctor from India who had practiced medicine in a few large cities in the United States prior to coming to the State of Maine.

Looking straight at the man, who was his wife's family caregiver, the doctor introduced himself and immediately began to speak. He asked general questions about his wife's physical conditions and current ailments, for which she was being treated, in addition to her dementia. The doctor was only interested in learning the more typical

medical history, but was not interested in specific techniques, treatments, or supplements the husband had discovered and researched. He had convinced the previous doctor to include these supplements along with her other medications to improve his wife's quality of life. Some of these treatments even resulted in the reversal of some of her dementia symptoms (speech, movement, engagement, etc.) which allowed her to complete sentences, ask questions and follow simple instructions from her care providers.

The new doctor then looked at the man's wife who was peacefully and comfortably sleeping in her wheelchair after her breakfast and began to explain that people at that late stage of Alzheimer's are incapable of being aware of, or appreciating, what we think of as quality of life experiences, adding, "Any supplements you may want to give her may in fact be incompatible with some of her other medications." As a final point, the doctor told the husband, "We don't like family members dictating what their loved ones should or should not be taking."

Needless to say, this young attending physician got the surprise of his life when the husband responded, "I am her pack leader. As her pack leader, I am of equal importance to you and all others on her interprofessional healthcare team. Pack leaders do not accede to doctors the final decisions or directives about supplements that may work for the person who is living with dementia." He finished by saying, "You, doctor, may not be the right doctor on her team."

Pack leaders do play an important role and do have a lot of say in the day-to-day care of the care receiver. One should

never cow down in the face of a doctor's demonstration of ego or anyone else on the healthcare team. The expert on this team is the care receiver! And you, as their advocate, must be ready to stand up for the very quality of life of the individual who is vulnerable and without recourse on their own. It is important to establish such a position upfront and to the administration from the first day a person is admitted into a facility or long-term care.

When Alzheimer's Medications Fail

It's not a secret that science is not perfect, and neither are we. Many have commented and written about the ineffectiveness and even the dangers of prescription drugs over the past decade. For example, Peter Whitehouse in *The Myth of Alzheimer's*, and Peggy Sarlin in *Awakening from Alzheimer's*.

Grassroots movements are finding ways to help those living with Alzheimer's in enough cases to make a difference. Individuals in these movements have even found ways to reverse the symptoms, allowing people to have a better quality of life. The ICA promises to change the course of dementia care, and to that end, it promotes innovative dementia-care solutions.

There are nine maverick doctors who are reversing symptoms of Alzheimer's and other dementias in some individuals (See *Awakening from Alzheimer's* by Peggy Sarlin). If there is a chance, it is worth taking in order to improve the quality of life for the care receiver.

Dr. Mary Newport discovered the benefits of coconut oil, one of which is renewing the ability to speak coherently in many (these results are not experienced by everyone) living with this condition. (See *The Coconut Oil & Low Carb Solution*:

For Alzheimer's, Parkinson's, and Other Diseases [2015] by Mary Newport, and Appendix 10, "Virgin Coconut Oil: A True Miracle Food" by Barbara Charis.)

Natalia Bragg is a sixth-generation herbalist living and working in Maine in the USA, who welcomes inquiries about her "four horses or 4-horsemen considered the planks of health which is a combination of D3, garlic oil, Super B Complex, and selenium in specific amounts" through her website, <http://www.uniquemainefarms.com>. She reports, "These four elements are the platform, on which we hang our health, whether we know it or not. Each and every one of these elements affects the heart, brain, and every system in the body. Unless you see it with your own eyes, you will find it hard to believe—both that it is so simple and yet also complex at the same time." Look into these options:

- > The *4-Horsemen* to support the person living with dementia (free PDF upon request at lordethelle@gmail.com or knotiibragg@outlook.com)
- > Lowering blood pressure with a natural blood pressure tonic and drinking three cups of hibiscus flower tea daily (one per meal).
- > Learn to use natural preparations such as Bacopa tincture and Lion's Mane, to name only a few that can make dramatic improvements in the quality of life and therefore care of those living with dementia.
- > How coconut oil can help someone living with dementia start functioning better within twenty-four hours after first intake? (See *The Coconut Oil & Low-Carb Solution: For Alzheimer's, Parkinson's, and Other Diseases* [2015] by Dr. Mary Newport, and Appendix 10, "Virgin Coconut Oil: A True Miracle Food" by Barbara Charis.)

- > How to contain dementia and reverse the symptoms of dementia thereby immediately improving quality of life and reducing caregiver stress (see Appendix 1 and *Awakening from Alzheimer's* by Peggy Sarlin).

The problem with waiting for traditional research to uncover regimens for relief from symptoms is that there is no resolution in sight as to the cause, and therefore the cure for Alzheimer. In the meantime, there are over 9.9 million new cases of dementia diagnosed each year worldwide or 3.2 new cases every second of the day. It is estimated three care providers are needed in each case. This totals an estimated 30 million care providers. It is an enormous cost to humanity as well as healthcare budgets (<http://www.alz.co.uk/news/world-alzheimer-report-2015-reveals-globalcost-of-dementia-set-to-reach-usd-1-trillion-by-2018>).

What Every Family Caregiver Needs to Know

Alzheimer progresses in stages (mild, moderate, and severe) and ideally, the mild stage should be a planning stage. Several topics should be discussed and important tasks tended to by both the person who has the condition and their family caregiver or pack leader. Doing so is enormously useful as the condition progresses and helps avoid family conflicts.

Mild Stage of Alzheimer

First, legal documents must be in order. An *elder law attorney* is the best person to consult about documents such as a last will, power of attorney, and advanced health care directive. Second, financial planning is critical in order to have sufficient funds for the care of the person with Alzheimer and for the person who is left behind when the condition has

run its course. Finally, banking issues and protecting financial sources for the needs of the care receiver and the expenses of providing care should be considered.

Take lots of pictures of events—or simply for no reason. Alzheimer is very hard on the body of the person who is suffering from it, and their appearance changes. Pictures allow us to look back on these memories during the later stages of the condition.

Record the voice of the person diagnosed with Alzheimer. A small recorder is inexpensive, and the recording will last one hundred years. In the advanced stage of Alzheimer, the voice of the care receiver may be silenced. Hearing the voice of the person after they can no longer speak is enormously calming and rewarding. A history of the person's life can be created using a recorder. Find out more about this at <http://www.instructables.com/id/Record-Your-Familys-Oral-History-before-it-dies->.

Moderate Stage of Alzheimer

At this stage, it is important to create activities that keep the person interested and vibrant. Schedule regular visits by family, friends, and acquaintances to play cards, read to the person, or take a walk outside. It is now difficult to travel by plane or bus, and car rides become scary because of disorientation due to a failing perception problem. The pack leader must now treat caregiving as a full-time job. It is important to take regular daily breaks, eat three meals a day, exercise regularly and get respite on a weekly basis— just as if they had a regular office job.

Severe Stage of Alzheimer

It is still possible to keep the person at home. As a matter of fact, it is preferable for their quality of life unless the caregiver has graduated to pack leader and is ready to assume the responsibilities of a team member on the larger interprofessional healthcare team in a care facility.

At this stage, the pack leader must plan on getting away from the home at least three or four days a month for respite. That is in addition to providing full-time care only five days a week as explained above. Therefore, more home care is needed in the severe stage of Alzheimer and requiring more assistance for the primary family caregiver.

As the condition progresses from the mild stage to the severe stage, so do the responsibilities of the caregiver. It is a reversal of progression in that at first the care receiver seems to suffer more because they know how much they are forgetting. On the other hand, the progressively increasing responsibilities and the weight of emotional and physical demands on the caregiver become extreme.

Remembering that no two people experience the same effects of this condition, it is not possible to predict exactly how and when each stage will last, what problems the caregiver may encounter, or how good each outcome will be.

Planning ahead regarding these suggestions saves both stress and sadness later on. I know... I've been there!

Isolation

Isolation is a common, almost expected, result of providing care on a full-time basis to a family member or a close friend in

need. These wonderful caregivers devote twenty-four hours a day, seven days a week to protect someone in need of care and provide assistance with their activities of daily living whether such care is due to dementia or another life-threatening illness.

To all those who suffer in silence, feel abandoned by family and friends, and wish they could receive a call or invitation to tea, you are not alone. Please call the free line at the Alzheimer's Association in your country to talk with someone at times when you feel isolated— it does not matter the time of day or day of the week:

In the United States, 1-800-272-3900

In Canada, 1-800-616-8816

In the UK, 0845 300 0336

Get Basic Dementia-Care Training

Speaking Alzheimer can make the life of the care receiver much easier and the work of the care provider even better and much less stressful. It only takes about twenty minutes to learn to speak Alzheimer and a little longer to understand why it works that way. Home caregivers should teach others who come to the home how to speak Alzheimer so they won't quit visiting because they are afraid to upset the caregiver or the care receiver.

Learn how to easily and effortlessly prepare the care receiver to take a bath or shower. People living with Alzheimer naturally fear getting into the shower or bath, and they can even develop a fear of water.

There is also a method for transferring someone from the home to a long-term care facility in an effortless way, so they

don't know they have been transferred. Many cannot adapt to long-term care if they are simply moved from the home without a plan. Avoid complications, stress, and much sadness for everyone by planning ahead and making a seamless move.

The ICA specializes in training people in all the above and much more. ICA finds a distinction between knowing, understanding, and practicing. To that end, all our training programs use a layered approach to guide our learners towards mastery in dementia care. ICA provides introductory access to a variety of basic but essential dementia care topics, often with one single idea, technique, or concept that learners are able to apply on their own as they leave the training session. This may be as simple as getting them to understand why their caregiving style might be at the root cause of the resistance they experience and report from their care receivers.

Furthermore, ICA recognizes that true mastery in dementia care of effective techniques and strategies takes time. Reflection and mentoring of trained staff are never a given. Therefore, ICA offers a range of services such as webinars, in-person training, coaching, mentoring, and certification in Alzheimer/dementia coaching. ICA knows that proven training that meets the unique needs and timeframes of our clients and learners is critical to changing the course of dementia care.

ICA's ultimate goal is to empower all care providers so that they are sufficiently confident to adapt to the needs of each care receiver in the moment of service. Such confidence is based in knowledge, training, experience, and a deep understanding of not just the "what" or "how" of dementia care, but the "why" behind what ICA teaches.

DR. ETHELLE LORD

The effectiveness of dementia care services will one day be measured by the number in the workforce who put ICA's (International Caregivers Association) standards of care into daily practice.

—Ethelle Lord, M.Ed., DM; President at ICA

How to Tell Another Family Member about the Progression

The letter below was written by me to my mother-in-law (ninety-four years old as of this writing) who had difficulty understanding why she was not healthy enough to travel from the West Coast to the East Coast to visit her son, who is in the severe stage of vascular dementia. It was published on LinkedIn on December 8, 2014. The collage below was framed and sent to her along with the letter.



Pearl Potter 1922-2017

Dear Mom,

I am writing this letter from my heart and to thank you for raising such a wonderful son. Larry may not have always done the right thing in his life, but I know he always wanted to do his

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very best. I know this from loving him and living with him for almost 20 years. He loves you very deeply and his love for you is eternal. You are his mother. You conceived him, carried him for nine months, gave birth to him on March 6th, 1942, and raised him to become the man he is today. No easy task!

Although you did not know this in 1942, it was because of you that I met and fell in love with him in 1995 (the purest love I'll ever know). For that, I will be eternally grateful to you and Dad, whom we all miss so much. I can still hear Dad say, "That's my boy!"; "Well, I'll be darned!"; and so many other sayings. I remember meeting you in the airport in Palm Springs, California, and how in your hearts you wished Larry could find love. That is a wish every parent has for their child, isn't it?

Larry had just met me with a white rose, a warm kiss, and a large group of people clapping for us. It was very special because I had recently been widowed on October 5th and it was just December when I first met you.

Larry's transfer from Presque Isle to Caribou this past summer has been extremely hard on him. It is expected of anyone living with dementia to make such a move. Caring for him at the most vulnerable time in his life is my priority and also my responsibility. He is an amazing person with a heart of gold; the same heart that you saw in him as a little boy and young man before he left home to go to university. Every time I think of him or visit with him now, I feel extremely blessed to be with him and I want to help him in any way I can. I do not want him to ever feel he is alone or not loved.

I want you, as his loving mother, to know that it is important for you to remember him as he was in the enclosed picture and in the nice canvassed photo of him in your living room. He has difficulty keeping his eyes open now because it is too difficult for his brain to process the stimuli around him. He often is silent for long periods of time because it takes a lot of energy and concentration to live in the present. Most of the time he chooses to be in a world of his own where he finds solitude, things to explore, and much love.

The love you and Larry have for one another is eternal. Such love cannot possibly be for only the few years we spend on earth, right? There is no beginning and no end to a mother/child love. I ask that you remember Larry for what he was and not so much for the condition he is experiencing now. The picture that accompanies this letter needs no words; it speaks a thousand words with his right arm securely around you. That is your Larry and you are his loving mother!

Lovingly and with much appreciation,
Éthelle

The Health of the Caregiver

The following is a reprint of an article that can be found at http://www.mdthink.com/mpanel_document/severe-depression-linked-to-brain-inflammation/.

A new study published in *JAMA Psychiatry* suggests that clinical depression is associated with a 30% increase in brain inflammation.

Inflammation is a natural response of the body to infection or disease; however, too much inflammation can be damaging. Some recent evidence suggests that inflammation may drive depression or depressive symptoms such as low mood, anorexia, and insomnia. Researchers of the new study wanted to find out if inflammation is a driver of clinical depression independent of other physical illness; and therefore, to study, researchers scanned the brains of 20 patients with depression and 20 patients without depression with PET scans. They particularly measured the activation of microglia. The findings on PET scan showed significant inflammation in the brains of people with depression. The inflammation was seen to be most severe in participants

with the most severe depression. The brains of people who showed signs of clinical depression exhibited an inflammatory increase of 30%.

Earlier in a similar research in 2012, researchers from Duke University Medical Center had tested the association between depression and CRP in adolescents; and found that the number of cumulative depressive episodes was associated with elevated levels of CRP. Their findings had suggested that depression is more likely to contribute to inflammation in the body.

The debate whether inflammation is a consequence of or contributor to major depression was always going on. The new discovery will hopefully help in discovering therapies for people who have depression. Currently, antidepressants are prescribed to treat depression, but not many benefit from it. Therefore, future research can perhaps focus on anti-inflammatory therapies to treat depression. If this proves successful, it would truly be a unique way of treating depression.

I have never met a perfect human being. We all are here on earth to actually perfect ourselves and become more tolerant of one another. As a caregiver, you will have many moments you wished you could take back or times when you are 100 percent frustrated and exhausted. Perhaps you will say something you regret or refuse to do a task that you believe should be done. Learn to forgive yourself and others. Do this as often as you need to on a daily basis in order to keep moving forward and not holding your regrets.

The Importance of Fitness

It is not advisable for a caregiver to provide long-term care for someone they love without first looking at their own mental and physical fitness. Smiling and laughing out loud takes less muscle energy than being sad and having a sad look on your

face. Caregivers can build the habit of viewing at least one comedy a week, either at a theater or at home. They can read the comics in the daily newspaper. Something funny that the care receiver says, or does like breaking into a song or dance, can also bring a smile to their face. It's a good idea to keep a journal of things they're grateful for and events that brought a smile to their face or the face of the care receiver.

Physical fitness is important in order to be able to endure a 24-7 schedule. Here is a list of quick but effective ways to stay physically fit that require only a few minutes:

1. Lifting small weights while watching TV.
2. Using a new portable aerobic stepper called Original Sand dune for only a few minutes a day.
3. Jumping on a trampoline for a few minutes a day.
4. Using a treadmill for a few minutes a day or placing Energy Pads® under your feet to completely relax muscles, reduce stress, and increase energy levels while simply sitting down.
5. Walking on a treadmill or best still is to walk outside while on a break.
6. Chi Machine for both the care provider and the care receiver.

There are many other pieces to the fitness puzzle covered in ICA's A/DC training program.

PROFESSIONAL CAREGIVING (FORMAL CARE)

Where Are They?

Doctors receive very little training, if any, in dementia care. Because science has not yet been able to complete this puzzle, it is difficult for doctors to readily adapt right-brain solutions to a left-brain health problem.

Professional caregivers should make a commitment to read journal articles and follow dementia stories that are life-changing and offer insight into the day-to-day, hour-by-hour care of people living with dementia. When nurses and aides do not know how to speak Alzheimer, they too often document that the individual refused treatment or food to cover themselves legally when they are unable to treat or feed a resident. This is unnecessary and a truly unacceptable practice. A great many people living with Alzheimer or another dementia are undernourished and eventually starved in care facilities partly due to the lack of analytics to measure the number of people in the care of each professional caregiver. Why?

Personal attendants who work in the home care industry are even less prepared to provide dementia care. Some states

in the United States hire immigrants who are not fluent in English. This creates yet another serious problem for the care receiver and family who depend on home care services. Many buy into a home care franchise only to find they are left on their own with insufficient resources and an untrained pool of employees.

Too many professionals rely only or heavily on DVDs for dementia-care training. One important reason employers miss the mark, however well intended, is that they narrow the learning experience and limit the practical portion of the training on these videos. They also tend to skip mentoring the learner. Training requirements are covered but the learning has not been assimilated as it needs to be— especially in dementia care.

There is a fifty-year history of inter-professionality in health care, which refers to collaborative teams that improve the quality of care and life for those receiving care. Inter-professionality has never been more important or more needed than it is now with dementia care.¹ Team-based care in dementia care saves money and time and improves on the delivery of care. Families can rest better at night knowing there is an entire team providing care and that they are also part of the team. Here are a few examples of organizations and initiatives advancing interprofessional practice and education provided by the National Center for Interprofessional Practice and Education:

- Interprofessional Education Collaborative
- National Academies of Practice
- Institute of Medicine Global Forum on Innovation in Health Professional Education

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- Alliance for Continuing Health Professions Education
- Institute for Healthcare Improvement Open School
- Health Resources and Services Administration, Bureau of Health Professions
- Geriatric Interdisciplinary Team Training Program
- Collaborating Across Borders Conferences I, II, III, and IV

Nobody wants to be overlooked in dementia care; most of all, the care receiver and his/her family. Every person living with dementia deserves to be treated with a great deal of respect and dignity, which is more available within an interprofessional team than in isolation.² Communication is primary to any health care or treatment plan. Nurse leaders, practitioners, and educators must especially avoid isolation when it comes to dementia care because an individualized treatment approach is required in order to provide better nursing services.³ This is not to say that interprofessional healthcare teams are without conflict. This was especially true in a qualitative study of formal health care professionals and caregivers in the home.⁴ Conflict is a reality in any workplace where more than one person is employed.

DEMENTIA CARE FACILITIES

Teamwork and Inter-professionality— A Pair

In inter-professional (IP) workplaces, clearly defined team roles and coordination of tasks enable the entire healthcare team to have more confidence in explaining their roles, which are supported by a much larger team. Doctors, nurses, and everyone else involved “can become points of reference in health care system changes by providing support in IP nursing practice environments, describing the work of nurses, demonstrating positive outcomes linked to nursing practice, and participating in research that documents and expands how nursing is theorized in interprofessional situations.”¹

Executive and management teams benefit from adopting an IP workplace.² Since family caregivers are part of an IP team in dementia care, and, hopefully, an Alzheimer/ dementia coach coordinates the functions of the team, management is able to benefit from a family caregiver’s knowledge and experience like that of no other person serving on the team.

The care partnership agreement program (CPAP) that I created supports the lifework of Dr. Riane Eisler, who

introduced cultural transformation theory (CTT) in 1987 and supported resources “for the cultural transformation from domination to partnership.”³ Few books can inspire and transform more than *The Chalice and the Blade: Our History, Our Future* by Riane Eisler.

Hospitals

Dementia-friendly hospital care requires the adaptation of the interprofessional approach to such specialized care. Often dementia patients are given drugs that actually kill them—accidentally or at least sets them back to a point that they never recover completely, but nevertheless, these outcomes are tragic for the patient and their family. The number of individuals living with dementia worldwide was estimated at twenty-four million in 2007, with the number expected to double by 2021.⁴ In the World Alzheimer Report 2015, the Global Voice on Dementia reported around 46.8 million people living with dementia, with the number expected to rise to 74.7 million by 2030 and 131.5 million by 2050. With over 9.9 million new cases of dementia being diagnosed yearly, or 3.2 cases every second of the day, the need to change the course of dementia care is critical (<http://www.alz.co.uk/>).

Some dementia patients are admitted to psychiatric wards when that is totally unnecessary, again illustrating that attending physicians are undertrained in dementia care. I know of one family caregiver, a trained ER nurse, whose father was admitted to a psychiatric ward, given medication to subdue him and even restrained in his bed. He died only a few days later after having walked into the hospital on his own two feet. The psychiatrist reprimanded the daughter in the hall of the hospital, out of sheer frustration, stating, “I’m tired of

telling families their dementia family member can be helped,” and walked away leaving her standing there. This event took place in the State of Massachusetts in the United States, not in a third-world or foreign country.

Leadership must adopt better dementia care at all levels, from admission to how these individuals are spoken to and treated. A person living with Alzheimer should never be left alone in a hospital. Someone should be there who can speak for the patient, who should not be treated as though he/she is refusing care but instead as though he/she is simply confused. Acute care settings are ill-equipped to even accept, let alone treat someone with dementia. It is very hard to find someone in a hospital who can speak Alzheimer.

Dementia care is not only the largest health care problem in the history of mankind; it is a major public health problem in acute care hospitals. In addition to a diagnosis of dementia, more and more patients are now admitted to a hospital with other serious conditions such as anxiety, depression, diabetes, COPD, and heart disease. In “Dementia and Serious Coexisting Medical Conditions: A Double Whammy,” Katie Maslow concluded that “Nurses and other healthcare professionals should expect to see these relationships in their elderly patients... They should expect the worsening of cognitive and related symptoms in acutely ill people with dementia.”⁵

Currently, there is no standard of dementia care available to doctors. Therefore, there are no existing parameters of practice followed in the field. Although people in all the medical specialties are rushing to find the best possible ways to deal with dementia care within their respective specialties, much is unknown in the field of dementia care from a scientific and technological viewpoint. Since there are so many different

types of dementia, it is doubtful the medical community will ever be able to master each and every situation.⁶

Gary Joseph LeBlanc, who was his father's primary caregiver for about a decade, took his father to the ER. The ER doctor knew that Gary's father was living with Alzheimer. He looked at Gary's father while he was lying down, did not address Gary, who was sitting next to his father, but asked his father, "Do you know who the president of the United States is?" Gary's father stayed silent. When the ER doctor left the examining room, he heard Gary's father say to Gary, "Now, did you hear that?

He's the doctor, and he's asking me who the president of the United States is!"⁷

When patients living with dementia are released from a hospital stay, a better continuum of care is in order so that the family caregiver is supported and the care receiver receives the best possible care. The use of Reiki, to name only one complementary treatment modality, is outstanding in keeping a person calm and providing doctors and nurses the opportunity to treat them. It is used with dental care, hospital procedures, and much more. Reiki is ideal in that it is noninvasive and supports the entire individual receiving treatment. For an example of Reiki and dementia, please go to <http://www.thehealerwithin.com/complementary-therapies/>.

We are told by the Alzheimer's Association that Alzheimer was discovered over one hundred years ago. There is evidence in history that dementia was always present in one form or another. With the baby boomers, it is even more prominent as thousands are reaching the age of sixty-five on a daily basis. Both HIV and Ebola have been known for much less time, yet the medical community is on top of these two health care problems. Why governments and health agencies have not

made Alzheimer's care and research a priority before now is a mystery to millions of sufferers. I think we can do better, don't you?

Pain, colors, lighting, and the size of the television, furniture, and other equipment in a hospital room are different for someone living with dementia than for anyone else. An Alzheimer/dementia coach is trained to consult with and advise management in regard to the proper equipment and protocols to use in the care of those living with Alzheimer and other dementias.

Assisted Living

Early-onset dementia diagnoses are increasing in number, and these cases alone are changing assisted living facilities and nursing homes. Younger individuals are not able to adapt to such facilities that were designed for the elderly. And the elderlies of today are typically more active than elders in decades past, and they're looking for facilities that provide more active meaningful engagement activities.

Baby boomers look for assisted living facilities that offer dignity for all residents, affordable rates, and activities, other than bingo at 2:00 p.m. and 7:00 p.m. They want a care/social community worthy of their hard-earned dollars with appropriate living accommodations to last them until the very end of their lives— and why not?

Such communities must now adapt to serve all ages, not just the elderly. Elderly folks love to see and hear children's laughter because it offers hope that the next generation will carry on. They also love to hear and see dogs and cats. They are accustomed to such animals being part of the family. They want to get their hands dirty and laugh out loud. That is just

part of living with dignity and enjoying a quality of life that is respectful.

Each individual is unique— just like a snowflake. Yet current assisted living facilities often impose one-size-fits-all accommodations, activities, and participation in decision-making. Because Alzheimer can strike a person as early as in their twenties, we need to plan for decades of caregiving. Without the support of family members, friends, and acquaintances this is truly impossible to sustain for anyone who provides such care.

When the family caregiver is burned out after three or four years (See the “Caregiver Fatigue Timeline” in Appendix 7), the care receiver must usually be moved out of the home and into a care facility. Some are moved into assisted living facilities to benefit from the greater socialization there, which is very important for anyone living with Alzheimer, and others are moved into assisted living facilities because their home can no longer provide safety and care and meet their needs.

Owners, managers, and attendants in assisted living facilities receive no dementia-care training. Leaving someone who has been diagnosed with Alzheimer in an assisted living facility is like playing Russian roulette. The odds that they will simply walk out of the building following someone who is leaving are extremely high. Their quality of life and even their longevity is at high risk. The assisted living workforce has expressed an interest in dementia care training, but owners and managers have not yet set such training as a priority or requirement in spite that a large number of residents living in their facilities with a form of dementia. Assisted living facilities are like ticking bombs!

Nursing Homes

Proper training at the top of the management team on down to the custodians is grossly lacking in nursing homes. Family members are truly the ones who become the warriors and insist on changing the course of dementia care in these facilities. Government rules and regulations have gone from one extreme to another. In the not-too-distant past, residents were sometimes tied to their chairs or beds and drugged to keep quiet, and early deaths were run-of-the-mill. Today, the pendulum has gone to the other extreme where the rules and regulations do not allow even a safety belt in a wheelchair to prevent falls. Fall prevention needs to be a priority because falls further add to already high health care costs and actually kill people.

The Merry Walker has been all but banned from hospitals and care facilities when in fact such equipment allows a resident to be more mobile, increases physical fitness, and prevents falling to the floor. The Merry Walker is perfect for home, assisted living facilities, and nursing homes. Alzheimer affects the ability to keep your balance, resulting in shuffling of the feet, which naturally increases the risk of unnecessary falls.

A large majority of nursing homes assign too many residents to a single certified nurse assistant, and many do not have a registered nurse on staff around the clock. At home, a care receiver enjoys one-on-one care and is sometimes assisted by more than one person. In nursing homes, a resident with the same needs as the person in the home is cared for by the same staffer as ten or more other residents with similar health care needs.

By the time a person living with Alzheimer is admitted to a long-term care facility or a nursing home, that individual needs assistance with most of their activities of daily living including dressing, hygiene, eating, and mobility. Many have incontinence of both urine and feces.

It is not surprising that certified nurse assistants (CNAs) burn out in months or a few years because they are assigned too many residents. You might hear that CNAs are assigned to only five to seven residents, but it does not take a calculator to know that a facility with forty residents and three or four CNAs, in fact, must struggle to provide the best possible services they can at a ratio of one CNA to ten residents.

Both nursing homes and assisted living facilities need to implement analytics in their facilities in order to properly staff for services. This is a service that A/DC can bring into an existing organization in order to upgrade services and staff in accordance with data rather than to meet government regulations. Data not only help efforts to upgrade and expand, but also to meet the health care needs of patients, residents, and families. (For more information about analytics, see Wills, MJ, "Decisions through Data: Analytics in Healthcare." 2014. *Journal of Healthcare Management*. 59(4) pp. 254–262.)

Families must beware, stand guard, and be proactive with managers of these facilities and the governments to whom they pay their taxes. The responsibility of providing care, meeting needs, and protecting a person living with Alzheimer does not start and end at home. It starts with diagnosis and ends on the day of the person's last breath. Families are part of the larger interprofessional health care team and must insist on taking on that responsibility to assure the best possible outcome. Everyone can do better when it comes to dementia care.

Falls in a nursing home often result in hospitalization. The cost of such a fall is estimated to be twelve thousand dollars per fall, but much greater is the loss of confidence on the part of the resident. Unintentional, or unsupervised falls that result in hip fractures and other fall-related fractures in older adults, speed up placement in long-term care facilities.⁸ Falls are preventable, and more needs to be done to protect the independence of those living with Alzheimer and other dementias.

Dementia Villages and Care/Social Communities

On the outskirts of Amsterdam in the town of Weesp, Netherlands, is a *dementia village*. It was recently profiled by Dr. Sanjay Gupta at CNN. The segment is available at <http://www.youtube.com/watch?v=LwiOBlyWpko>. The dementia village is a wonderful answer to the outdated and largely inadequate nursing homes of today.

However, care/social communities are even better arrangements that accommodate families living with their loved ones. They are for all generations, not just the elderly living away from their loved ones. They are homes where there is family, so the care receiver is never alone, does not feel isolated, and is not tempted to *escape*.

Remembering 4 You personnel originally designed the care/social community plan, named it the CPAP, and purchased a logo to identify care/social communities. All that is needed is curiosity, interest, and investors.

It will take decades to upgrade care homes. Baby boomers should speed up the process as millions become caregivers and care receivers on a yearly basis. Most retirees cannot meet all

the responsibilities of maintaining a private dwelling. They are welcoming community living that can remain current to their needs and how they view their golden years.

Summary

In 2006, over eight hundred thousand people were living in assisted living settings in the United States.⁹ Clearly, the need for care/social communities will continue to rise. Since WWII, we have known when the baby boomers will reach retirement age, yet we have not planned for the housing demand. Economists advising governments should have been working toward resolving this housing crisis since the 1960s.

Analytics in health care not only offer insights but lead to outcomes that change the course of health care. Alzheimer is challenging to everyone, from families to undertakers. We are all pressed to reduce the costs, improve teamwork and outcomes, provide much better care for less, and develop a standard for dementia care that everyone can adopt. Yet evidence is mounting that we are not moving fast enough to accommodate those affected, which affects bottom lines in many different industries. Analytics competency is grossly lacking in care facilities, yet this is what can help such organizations “harness ‘big data’ to create actionable insights, set their future vision, improve outcomes, and reduce time to value.”¹⁰

Let's work smarter in health care, not harder. There is a solid certificate program for Alzheimer/dementia coaches backed with training that makes them ideal healthcare partners in dementia care. These professionals are ready to bring the highest possible value to any management team and family ready to find the best possible solutions for dementia care.

Part 7



TEAMWORK

“The positive thinker sees the invisible, feels the intangible, and achieves the impossible.”

— Winston Churchill

INTRODUCTION

Simply put, teamwork allows an organization or group of people to do much more in less time and for less money— a group of people works collaboratively to achieve a common goal. In dementia care, the goal is to provide the best possible care for and support all the needs of the person living with Alzheimer or dementia.

In a systems approach, teamwork helps avoid blaming one person for a mistake or error. The entire system is responsible for mistakes and errors. By taking a system's responsibility, it is possible to change the system to avoid future errors of the same kind. Therefore, it is imperative and pressing for the system (organization) to find a solution as soon as possible. Teamwork within a systems approach values each member and looks to the organization to solve mistakes and problems. No individual gets the blame for a system's error or weakness.

So, even if one employee or more starts slacking off or even undermining the team in some way, it is still a systems problem because the team allowed such a situation to occur. The source of the problem might go as far back as the hiring process. Perhaps it is in the training or mentoring programs.

In the news recently, there was much to do about the systemic problems of patient access and patient care in veterans' affairs in the United States. By failing to correct such problems at a systemic level, nothing actually gets resolved and problems continue to exist.

TEAMING FOR STRENGTH

Each team member brings personal traits and skills to the team that no other member can. It is important to determine individual preferred or natural strengths in each team member. The easiest and best way is to do assessments.

The first assessment needs to be evaluating each member's preferred communicating style. This allows other members an appreciation of other communication styles and motivates them to be more tolerant of how others learn and work. Once each team member has completed their own self-assessment and scoring, the team can have an open discussion about how to use this information for themselves and in living and working with others. I recommend the neuro-linguistic programming assessment. (See Appendix 2.)

The second assessment is the Belbin evaluation for preferred team roles that was discussed in chapter 6. Belbin is the expert in team formation and team roles. The process takes about twenty minutes, and the results are analyzed by a Belbin expert. For more information on this assessment, simply go to <http://www.belbin.com/rte.asp?id=400> and for analysis go to <http://www.belbin.com/rte.asp?id=10>.

Questions that are commonly asked concerning the Belbin team roles are:

- Is Belbin's test a psychometric test?
- What can be gained by identifying people's team roles?
- What are *observer assessments*?
- Why use *observer assessments*?
- Do team roles change?
- Should I let people know my preferred team roles?
- What is an *allowable weakness*?
- What is a *team role sacrifice*?
- When I know my strongest team roles, what should I do about it?
- Why *plant*?
- I thought there were eight team roles?
- Is there a self-scoring version of the inventory?
- What about the reliability and validity of *inter-place*?
- Do I need to be accredited to use the reports?

The more we can learn about team roles, the better we can communicate and solve problems at work. You can read *Team Roles at Work*, *The Belbin Guide to Succeeding at Work*, and *Management Teams: Why They Succeed or Fail* for greater understanding of the value of this information

TEAM FORMATION AND ROLES

In order to have an ideal team formation, all team positions should be represented. Three shapers (See Appendix 5) on one team can stress out the entire team, and it might not be able to accomplish all tasks.

Anyone can play more than one team role, but each individual has a preference, just as each individual has a preferred communicating and learning style per NLP. Uncovering each person's favorite team position allows the team leader to allocate work to the individual with the appropriate skills and capabilities. Knowing the allowable weaknesses for each team member based on their role is valuable to the entire team.

Now imagine that each individual living with Alzheimer's also has a preferred way to communicate and a preferred team role. There is no end to the value of such information in helping the entire healthcare team improve communication and avoid conflict or resistance.

For a self-evaluation Belbin inventory, please go to: [http://www.belbin.com/content/page/8967/BELBIN\(UK\)-Self-PerceptionInventory+CompletionGrid.pdf](http://www.belbin.com/content/page/8967/BELBIN(UK)-Self-PerceptionInventory+CompletionGrid.pdf)

INTER-PROFESSIONALITY IN HEALTHCARE IS BACK!

Inter-professionality requires two or more people from different professions learning about one another and enabling one another to effectively collaborate and improve on health outcomes.¹ More and more, Alzheimer is forcing the hands of everyone, from families to the medical community, to create a better system of care and performance of care in order to improve the quality of life. How can we work together in changing the course of dementia care if we do not learn together?

Inter-professionality ideally begins in medical schools and nursing schools. However, with Alzheimer it needs to begin in the field with an Alzheimer's/dementia coach standing at the hub of the healthcare wheel, or center stage; family caregivers, doctors, nurses, aides, home care agencies, etc. as the spokes; and the care receiver as the rim of the wheel.

There are four core competencies in inter-professionality:

- ✓ values/ethics
- ✓ interprofessional communication

✓ roles/responsibilities

✓ teamwork

Dr. George E. Thibault, an MD from Canada, who is president of the Josiah Macy Jr. Foundation stated, “We have good evidence that health care delivered in teams is more efficient and more effective...To meet the public’s needs, health professions educators must teach and model collaborative practice and team-based models of care.”

(For more information and greater details about interprofessional collaboration, please go to the Canadian Nurses Association at <http://www.cna-aic.ca/en/on-the-issues/better-care/interprofessional-collaboration>)

The following timeline is reprinted from “Interprofessional Education and Collaborative Practice” by Abraham Flexner:

History of Interprofessional Education:

- 1910:
 - Abraham Flexner, acclaimed reformer of medical education, criticizes the splintering of education for healthcare professionals and especially medicine
 - After the Progressive Era, interest in IPE waned but continued in Canada
- 1940s:
 - The importance of interprofessional collaboration in rehabilitation is recognized
- 1970s:
 - First systematic review of studies about the impact of team delivery of care in rehabilitation by Laura Halstead MD

- Birth of Interprofessional Education as a field of study.
- Increased global concern regarding the delivery of healthcare and the role of interprofessional teams in reducing safety errors. The UK and Canada assume leadership roles.
- 1980s:
 - Center for the Advancement of Interprofessional Education established in the United Kingdom —Journal of Interprofessional Care established in 1986
- 1990s:
 - Canadian Interprofessional Health Collaborative established
 - Institute of Medicine (IOM) report To Err is Human (1999) details extent of preventable errors in medicine and strategy to improve quality and safety of care
- 2000s:
 - IOM report Crossing the Quality Chasm: A New Health Care System for the 21st Century calls for fundamental changes to the health care system to close the quality gap.
 - Educational institutions develop IPE initiatives
 - First Collaborating Across Borders conference held on the University of Minnesota-Twin Cities campus, a US-Canadian collaboration (2007)
 - World Health Organization Study Group on Interprofessional Education & Collaborative Practice developed
 - World Health Organization report published in 2010, Framework for action on interprofessional education and collaborative practice²

Part 8



LOOKING AHEAD

“You never change things by fighting the existing reality. To change something, build a new model that makes the existing model obsolete.”

— R. Buckminster Fuller

You will make it through this and you will know what to do and when to do it. Somehow, we all figure it out along the way. We may handle things differently than others, but we do the best we can with the hand we're dealt.

— M. J. Smith



Before looking ahead, it is important to look back and reminisce. The majority of family members say, “I knew there was a problem, and I just could not put my finger on it.” Alzheimer is a slow-developing condition affecting millions of people and therefore millions of families. It is a life-changing condition that is very much predictable and progressive in the degree of disability.

One of the first signs that dementia, or as I once heard someone call it, “Al Zeimer,” has moved in with you is when your loved one keeps promising things but cannot be relied on to do what they promised. It can be as simple as going to the store to get a few supplies only to return home empty-handed, yet with an excuse.

Another telltale sign is the stories the individual with Alzheimer tells to their family members or in social gatherings. The stories begin to not make sense to family members, who are familiar with them, while to strangers they might seem perfectly logical and reasonable. The voice is the same but the memories are shifting slowly. The individual searches their brain for the facts and recalls the facts available to them at that moment, which are not necessarily related to the story. To give you an example, the story might have to do with recalling a recent trip to visit a friend. The person with Alzheimer might say, “We went to see Bob and played cards last weekend,”

when the fact may be that they went to visit Mary who was in the hospital. Unless the caregiver is there to set the story straight, a stranger would conclude that it is factual—there is no indication of memory loss. Here are other signs that family members are first to notice:

1. Forgetfulness that actually disrupts daily life

People living with Alzheimer's can forget recently learned information or simply go out to do an errand and forget to do the errand. This includes forgetting important dates or events, asking for the same information over and over again, telling the same stories over and over again, expressing a fear of driving long distances alone, recognizing the face of a famous actor but being unable to remember their name, and relying more and more on family members, friends, and associates to remember things for them.

2. Inability to plan and solve problems readily as before.

People living with Alzheimer's can have difficulty following a plan or routine that was otherwise well established. Following a simple recipe or the routine for mowing the lawn or keeping track of monthly expenses is now obviously challenging.

Concentration on familiar tasks becomes a problem, so more and more errors are noticed by others.

3. Trouble completing familiar daily tasks at home or at work

Daily tasks become a real burden. Driving to and from work or to the market, managing the family budget, and remembering the rules of a familiar

card game are examples to watch for. Perhaps the individual takes longer than is reasonable to make a simple decision. They might forget how to tie their shoes or button up a garment.

4. Confusion about times, dates, and places. People living with Alzheimer's lose track of dates, seasons, and the passage of time. They have difficulty understanding something that is not happening in the present. They might even forget where they are and how they got there.
5. Difficulty seeing a visual image or determining spatial distances.

It is not uncommon for people living with Alzheimer's to experience sudden vision problems. This is evident when reading, judging distance, and determining color or contrast. It is even possible for them to see themselves in a mirror but have difficulty identifying with their own reflection, thinking it is someone else.

6. Difficulty finding the right word or dealing with a writing task.

Difficulty finding words and writing makes those living with Alzheimer's not want to join in conversations because they have difficulty following what is being said or written. They often stop speaking in the middle of a sentence, unable to complete their thought, or they simply repeat themselves. They can struggle with vocabulary, have problems finding the right word, or call something by the wrong name (for instance, a dog might be called a teddy bear).

7. Misplacing things and losing the ability to retrace steps.

People living with Alzheimer's often put things in unusual places (for instance, an empty coffee mug in the refrigerator or freezer). They can lose things and be unable to retrace their steps to find those things. Sometimes they actually accuse another person of stealing things they have misplaced. When they find something in the wrong place, they quickly accuse someone else of putting it there.

8. Poor judgment and reasoning.

People living with Alzheimer's exhibit poor judgment (for instance, wanting to trade a good car for a brand new one without prior discussion or planning). They are quick to donate to political organizations or charities without consulting their spouse or partner. They pay less and less attention to grooming and personal hygiene.

9. Withdrawal from activities at work and at home
People living with Alzheimer's have a tendency to remove themselves from hobbies, social activities, work projects, and sports. They may no longer be able to keep up with a favorite sports team or remember how to complete a favorite hobby. They avoid social situations because of the changes they are experiencing (for instance, someone who liked to joke is no longer able to find the punch line).

10. Mood and personality changes

The mood and personality of an individual living with Alzheimer's often change. Because they are confused, suspicious, depressed, fearful, or anxious,

it becomes extremely difficult for them to maintain their normal persona. As a result, they can be easily upset with others, especially in places where and at times when they feel out of their comfort zone (especially away from home).¹

Eventually, the person living with Alzheimer is able to detect that what they are saying makes absolutely no sense and is in fact erroneous. They may actually voice, “What I just said does not make any sense. Please forget what I said and forgive me for saying it.” There is much sadness, crying even, over the loss of memory in the early stage of this condition. The person living with Alzheimer might cry out loud while many of the family caregivers cry silently (hiding their tears that are flowing inside their hearts) so as not to further traumatize their loved one. Family members put up a front of leadership in moments like this.

That leadership position is extremely important for several reasons and lasts until the end of the life of the individual living with Alzheimer. It is most prevalent in the pack leader, who is usually the one who coordinates family interactions and news. The pack leader also intervenes on behalf of their loved one with doctors, nurses, and the many professionals the individual has a need for over the progression of this condition.

No two people experience Alzheimer in the same way, as it is very much an individual condition dependent on several factors such as personality traits, intelligence, and cultural background. Those who seem to do the best and live independently the longest usually have one thing in common: a high intelligence quotient (IQ).

Planning ahead for the course of this condition includes developing medical, legal, financial, and care plans to put in

place as needed. In the beginning, the person diagnosed with Alzheimer's is able to comprehend more than they are often given credit for. That is the time to examine their wishes and have them sign legal documents that will guide appropriate care as the condition progresses. Over time, judgment declines to a point at which they truly should not be making life-changing decisions or even driving a vehicle, let alone be allowed to wander in public places alone, as they will get lost easily and start to panic. Some Alzheimer sufferers have been shot dead because others viewed them as intruders who might harm them.

Visits to a doctor's office or hospital are to be expected. That is why it is important that the pack leader has a power of attorney to be able to easily speak to health care professionals and make decisions. It is important to record health changes on a day-to-day basis in a diary and bring it to all doctor appointments so the doctor can see the progression of a health problem and address it more easily.

Questions to ask the doctor during the first appointment:

- a. What are the tests needed, and how long will it take to get a diagnosis?
- b. Will you refer us to a neurologist?
- c. Could the medicines the patient is taking be causing their symptoms?
- d. Does the patient have any other conditions that could be causing their symptoms or making them worse?
- e. What should we expect if the patient has Alzheimer?
- f. What treatments are available for Alzheimer?
- g. What are the risks, benefits, and possible side effects of such treatments?

- h. Are there any nonpharmaceutical treatments the patient can take for Alzheimer?
- i. Is there anything else we should know right now?
- j. When should the patient come back for another visit?

Possible tests to be done by a neurologist (No single test can identify Alzheimer)

- a. Memory testing including pencil and paper exercises
- b. A detailed patient history
- c. Neurological exam of the nervous system and brain functioning
- d. PET scan (does not assure accurate diagnosis of Alzheimer)

Preparing for a visit to the medical doctor or neurologist

Keep a log of events and complaints to share with the doctor. What symptoms have you noticed? When did you first notice changes? How have the symptoms changed the person's everyday life? List questions you and your loved ones have so you remember to ask them when you see the doctor.

Take all the patient's medications to the doctor's office and information about how often these medications are taken. Take insurance cards as well.

The doctor will want to gather a detailed patient history and complete several medical tests, such as:

- a. Physical exam
- b. Mental status exam
- c. Lab tests of blood and urine

- d. Neurological exam of the nervous system and brain functioning
- e. Psychiatric assessment of mental health and cognitive skills

What to expect during a visit to the neurologist

The neurologist will likely perform a series of tests and exams as follows:

- a. Physical and mental health history review
- b. Mental status exam
- c. Cranial nerves exam
- d. Motor system exam
- e. Sensory system testing
- f. Deep tendon reflexes testing
- g. Coordination exam
- h. Gait observation
- i. Lab tests and other diagnostic procedures²

Legal Issues

There is an urgency to tending to legal matters in order to seize the small window of opportunity at the beginning by signing legal documents such as a health care proxy/ living will, power of attorney, and other legal papers important to the person living with Alzheimer's and the protection of assets for the pack leader or family. Elder attorneys specialize in elder law, but now that we have individuals being diagnosed in their twenties and thirties with Alzheimer, it is best to select an estate attorney appropriate for the situation. Here are some items that should be brought to the attorney's office:

- a. an itemized list of assets such as bank accounts, contents of safe deposit boxes, vehicles, real estate, etc.
- b. copies of all estate planning documents, including wills, trusts, and powers of attorney
- c. copies of all deeds to real estate
- d. copies of recent income tax returns
- e. life insurance policies and cash values of policies
- f. health insurance policies or benefits booklets
- g. admission agreements to any health care facilities
- h. list of names, addresses, and telephone numbers of family members
- i. list of names, addresses, and telephone numbers of financial planners and/or accountants³

Financial matters are more important now than ever before. By protecting the assets of the person diagnosed with Alzheimer, everyone in the family is protected. The cost of providing specialized dementia care is extremely high and can last for decades. In 2014, the average cost of home care in the United States was about twenty-five dollars per hour, which is rather costly for most families. It is less in Canada if you can access Baluchon Alzheimer's ([http:// www.baluchonalzheimer.com](http://www.baluchonalzheimer.com)), a respite service based in Montreal, which is fifteen dollars (for four-to-fourteen-day contracts for twenty-four hours each day). Other than Baluchon Alzheimer's in the Province of Québec, the average hourly home care in Canada is estimated at about twenty dollars per hour.⁴

At these hourly rates, it does not take long to calculate the hundreds of dollars a day that families would pay for respite care or daily care. Many primary caregivers will first reduce the

hours they work outside the home. Then they must consider quitting or retiring from full-time employment outside their home to devote all their time, energy, and savings to taking care of the person living with dementia. Finally, many of these primary caregivers will spend all their savings, retirement fund, and need to sell their home in the course of supporting the person living with dementia.

A wonderful new reference for home care since 2019 is Nurse Mother Caregiver providing information and resources in every state in the United States for those seeking home care, home health care, and hospice care providers (nursemothercaregiver.com).

Estimates of Dementia Care Costs in the United States and Canada in 2013

According to John Hancock's 2013 Cost of Care Survey, other senior care options are even more expensive than nonmedical care:

- The 2013 average cost of a private nursing home room (\$258 a day/\$94,170 annually) has risen an average 3.6 percent per year.
- The 2013 average cost of a semiprivate nursing home room (\$227 a day/\$82,855 annually) has risen an average 3.6 percent per year.
- The 2013 average cost for a month in an assisted living facility (\$3,427 a month/\$41,124 annually) has risen an average 2 percent per year.
- The 2013 average cost of adult day care (\$71 a day/\$18,460 annually) has risen an average of 1.6 percent per year.

- The 2013 average cost for a home health aide (\$19 hourly/\$29,640 annually) has risen an average 1.3 percent per year.⁵
- For 2019 costs in the United States go to dementiacarecentral.com/assisted-living-home-care-costs/ and payingforseniorcare.com/memorycare

In the United Kingdom in 2013, “...average cost of a residential home is now £580 a week— and more than £700 a week for a nursing home— it is people’s assets can be quickly exhausted if they spend their last few years in care.”⁶ It is extremely important to remember that approximately 80 percent of persons living with Alzheimer will eventually reside in long-term care facilities, especially when the condition exceeds four or five years of home care and when support needs exceed family caregiver capabilities. Families that wait too long to make this change reach burnout stage and can put themselves and their loved one with Alzheimer at risk.

Those who are transferred from home to long-term care fair just as well as they did at home, and perhaps even better because of the regularity of medical intervention, meals, and care provided. It is never recommended that the pack leader resigns from his/her position before the person with Alzheimer is actually deceased. If at all possible, the pack leader should continue as the primary caregiver, becoming part of the larger interprofessional healthcare team to help with the more and more complex requirements as the person living with Alzheimer ages.

Having savings set aside can prove to be a lifesaver when it is time to transfer to long-term care, as many facilities readily accept someone who has the ability to pay for the services (even if only for a few months) but cannot so readily accept someone

who has to qualify for Medicaid or other financial assistance. Medicaid pays much less than private insurance companies for long-term care, so facilities are not able to accept as many Medicaid residents as they might like to.

Pack leaders have a predictable and progressive track. Familiarity with statistics and information about this process can be lifesaving. It is estimated that a large percentage of pack leaders contemplate, plan, and even carry out suicide. The specific family caregiving stages should be learned and observed carefully. The only way to lengthen this process and avoid burnout or even suicide is to treat this responsibility as you would any other full-time job, with proper, regular breaks and respite, vacations, and asking for help along the way.

Burnout is defined as a state of physical, emotional, and mental exhaustion that can be accompanied by a change in attitude. Acute burnout can lead to a feeling of hopelessness that can, in some cases, result in homicidal or suicidal tendencies. To paraphrase a popular quote about depression, burnout “is not a sign of weakness; it’s a sign that you have been strong for too long.” This is the challenge shared by people who provide care for those living with long-term degenerative diseases such as Alzheimer, multiple sclerosis, brain injury, stroke, and some forms of mental illness.

It is important for a caregiver to recognize the symptoms of burnout and seek professional help when they need it. In many ways, they are also living with Alzheimer or dementia.

The Caregiver Fatigue Timeline (See Appendix 7) reflects the results of numerous health and wellness research studies on full-time caregiving. People living with Alzheimer may require years, even decades, of specialized care by a family member. Therefore, knowledge of the signs of caregiver fatigue is a

commonsense thing for family members and family doctors to look out for. A timeline similar to that of the caregiver fatigue timeline can be used for healthcare professionals.

One to Eighteen Months (after the diagnosis of dementia)

Assess for signs of anxiety.

At Twenty-One Months

Assess for signs of physical and mental fatigue and need for sleep aids.

Twenty-Four to Thirty-Two Months

Assess for facial signs of total exhaustion and burnout.

At Thirty-Two Months

Assess for signs of self-medication for muscular pain and to reduce irritability, fatigue, isolation.

By Thirty-Eight Months

Assess for lack of appetite with weight decrease (or weight increase in spite of lack of appetite), lack of physical energy due to chronic fatigue, hypertension/ colitis, lack of energy to brush teeth every day and manage tasks in the home, and feelings of guilt and grief.

After Fifty Months

Assess for fatigue due to 24-7 responsibilities, isolation, and depression with or without suicidal thoughts or plans.

Sometimes, the family or interprofessional team has to intervene to place the pack leader themselves in care to save their life.

It is extremely important for the pack leader to identify a support team from the very beginning. They should share the caregiver fatigue timeline with them and make their doctor (as well as the doctor for their loved one) aware of it so they

can receive the medical support they need. Whenever a person is diagnosed with Alzheimer, a diagnosis for the caregiver should be given at the same time because they are at risk for depression, weight gain or loss, sleep deprivation, anxiety, and a variety of ailments. Also, of concern is the age of the pack leader. It is estimated that 20 percent of pack leaders predecease those they care for.

Death is a certain fate for all of us from the moment of birth, but few of us discuss our eventual moment of death and how we would like our lives to be remembered. Confronting Alzheimer seems to push that stage to the forefront. When looking at decades of dealing with Alzheimer, it is a subject that truly needs to be discussed.

Realistically facing death helps us create memories and honor each other for our contributions to our families, our work, and society in general. For example, it can be as simple as creating a story for the person diagnosed with Alzheimer about a bridge being built in their honor to receive them once their life is over, and that there are special people already there holding a space for them and ready to welcome them with gifts and a unique celebration. These people and their welcome gifts can be named. Developing such a story early in the timeline can help everyone involved grasp the reality that life is finite but love is eternal. Depending on individual faith and upbringing, such stories can sustain the emotional need to be connected and support everyone's basic human needs. The following list of basic human needs is based on the work of Abraham Maslow, a prominent American psychologist and author who developed a hierarchy of human needs, and is adapted for persons living with Alzheimer and other dementias:

Physiological

Examples include basic medical attention, food, shelter, hydration, socialization, auditory/visual/kinesthetic activities, physical affection, rest/sleep, and physical exercises including walking/yoga/physical therapy. In one word, this is referred to as self-care.

Safety

Examples include avoiding unnecessary falls; never being left alone to wander except in a safe area; moral/ ethical conduct by all caregivers; being surrounded by family and family pictures and memories; regular medical appointments for optimal health including eye exams, dental exams, and physical therapy; and meeting responsibilities according to abilities versus disabilities.

Love/Belonging

Maintain friendships through regular visits with friends, and let friends know how to speak and approach the person who is living with Alzheimer. Allow families to visit and help with specific responsibilities including doing activities with their relative who is living with Alzheimer. Sexual intimacy is important in that it helps maintain normalcy for that individual, yet sexual intimacy may be limited to touching, especially in the severe stage of Alzheimer. Always approach the individual with a smile and a soft tone of voice that denotes love and acceptance.

Esteem

All humans should be treated with dignity and respect. Those living with Alzheimer or another dementia have a need to feel confident in their abilities and have the respect of others. Their feeling realm increases as their condition progresses, so they are more able to sense whether they are respected or disrespected by others, especially by their care providers.

Self-Actualization

Moral, ethical behavior, and conduct are extremely important when it comes to delivering care to someone living with Alzheimer. Be creative by following the individual rather than leading them. They are their own best expert in this area, and they demonstrate spontaneity in the best possible ways. When problem-solving, it is important to present only two solutions at a time and make it possible for them to respond with a simple yes or no. Always accept everything they say; never disagree with a person who has been diagnosed with Alzheimer. (For more information, see James, Oliver, *Contented Dementia*, 2010, Ebury Publishing, London, England.)

Families with several siblings often struggle to reach a compromise, so the pack leader should be identified or appointed as soon as there is a diagnosis. This ensures that the medical, legal, and care responsibilities are attended to appropriately.

Because Alzheimer is a brain disease, and most people look as healthy physically after diagnosis as before, family and friends often comment that the person does not look sick. Because people living with Alzheimer are able to fake conversations and reactions and keep up as much as possible

with others, family and friends might even comment that they think there is nothing wrong with them. In addition to being a source of stress and frustration for the person living with dementia, it is also a major source of stress and frustration for the person providing the twenty-four-hour-a-day, seven-day-a-week care (24-7). Comments from family and friends such as “He looks so good and seems to feel good too” or “I can’t tell that she is not well,” take on a negative aspect rather than providing positive support. Once family and friends leave, the person with Alzheimer relaxes and behaves as usual in front of the 24-7 caregiver. The person with Alzheimer might even ask the caregiver, “Who were those people? I’m glad they left.”

Families are important to people living with Alzheimer, as they want to keep the connection and sense of normalcy as much and long as possible but are having a hard time holding on to the memories finding the right words and making sense of their environment. People living with dementia are equally important to family caregivers and care partners because family members can reduce the level of isolation, provide support, and reassure the caregivers that someone else cares as much as they do.⁷

Enrolling family members to help with specific chores and responsibilities at specific times is extremely important for the quality of life of both the pack leader and the care receiver. One of the best resources for how to do this is *Coach Broyles’ Playbook for Alzheimer’s Caregivers* by Frank Broyles. His recommendations are applicable anywhere in the world and this is a must-have resource for any pack leader. There’s a free download at <http://www.smaaa.org/documents/AlzheimersPlaybook.pdf> and it’s available in print in 140 languages. It covers all stages of Alzheimer’s in a user-friendly way.

Use *The Playbook* as a tips and strategy training manual for any family member or hired care provider to assure the maximum understanding and respect for the person who is living with Alzheimer's. It can also help a pack leader design a suitable daily routine. Such a routine in spreadsheet format can allow any family member or outside provider to complete daily tasks that the pack leader has identified as necessary to stay ahead of demand of this specialized care responsibility.

Family members should take time to accept the diagnosis and let it sink in, and then start planning. The longer the people involved live in denial, the longer it takes to get the right treatment. Planning includes medical care, legal matters, and learning about the three stages of Alzheimer's and their sublevels. Once the reality of the diagnosis is accepted, family caregivers will find the courage they need, can experience joy in providing care to their loved one, and discover the hope that the quality of life is the best possible one while research keeps looking for treatments or even possible cures.

Caregivers should learn the five stages of caregiving/care partnering below in order to protect their own health and welfare (These stages follow the caregiver fatigue timeline. For a copy of the caregiver fatigue timeline, request one at lordethelle@gmail.com).

- Early Stage: Getting the news and looking for support and information
- Middle Stage: Feelings of guilt, resentment, and frustration
- Late Stage: Feeling depressed and needing more respite
- Bereavement Stage: Letting go and reframing the meaning of life as a survivor

- Rehabilitation Stage: Mental health therapy for situational symbiosis and to reenter society

Caregiver Support

Establishing a daily routine helps the caregiver rest more and reduce stress. Melatonin is particularly helpful in boosting the immune system and has other benefits as well (Read *Melatonin* by Dr. Russel Reiter and Jo Robinson). Consider replacing a multivitamin with the 4 Horsemen formula. Take the 4 horsemen daily to support your health and consider herbal supplements to address specific need you might have (internationalcaregiversassociation.com/botanical-energy.html). Aromatherapy, Reiki, and massage therapy are all helpful in reducing stress. The use of essential oils is particularly helpful for both the person with Alzheimer and the caregiver.

Purchasing and installing a day-of-the-week clock, sometimes known as dementia clock, can reduce stress for the person with Alzheimer and their caregivers. It tells the time of day (morning, afternoon, etc.) and the day of the week (Sunday, Monday, etc.), skipping the exact time, which has little meaning for someone with Alzheimer. It seems to naturally reduce caregiver stress as well because the caregiver is not limited to minutes and hours but enjoys the flexibility of morning, afternoon, evening, and nighttime.

Oral hygiene can be a problem for anyone who is getting older. Gel-Kam, Flo-Gel (not toothpastes but to be used after brushing), and Crest Healthcare are especially beneficial for people living with Alzheimer because they treat gums, whiten teeth, and make the teeth stronger and more resistant to decay caused by acid and bacteria. Use Gel-Kam or Flo-Gel after brushing teeth with regular toothpaste, leaving the gel on the

teeth for about a minute before rinsing. When the person can no longer spit toothpaste out after brushing, do not use any toothpaste, only the brush to clean the teeth.

Dietary changes have been shown to improve the symptoms of Alzheimer. Fruits, vegetables, and protein in the form of chicken, freshwater fish, and nuts are recommended. According to the Alzheimer's Association, a brain-healthy diet requires foods high in antioxidant levels such as dark-skinned fruits and vegetables like kale, spinach, broccoli, etc. An individual living with Alzheimer requires more calories than someone who is healthy.⁸

I mentioned family relatives recruited to help for specific responsibilities at designated times. I also mentioned hired caregivers to help on an as-needed basis. There is also phone help through the Alzheimer's Association and the Alzheimer's Society that can prove to be life-saving in the middle of the night when the pack leader is unable to sleep and worrying about the next day.

It is sometimes advisable for the pack leader to have an emergency phone line installed in their home so they need only push a button to reach emergency services. These are usually available through local hospitals or in the United States through the Area Agency on Aging.

The Systems Approach

The systems approach to dementia care begins with the pack leader setting a solid foundation that is useful and appropriate for any situation, including the transition from home to long-term care. The pack leader begins with simple systems in the home that help coordinate tasks and services. One is to keep

a to-do list of daily responsibilities so anyone can step in and complete it.

Below is a sample of a to-do list for home care. It changes over time as the needs of the care receiver change and as more outside help is brought in to assist the pack leader.

Make copies for each day, date each list, and retain copies.

Start/Finish	Tasks to do in morning	Notes on tasks accomplished or yet to do	Initials
	Up, cleaned, dressed		
	Breakfast and medications/teeth		
	Activity		
	Laundry and cleaning bedroom		
	Dishes		
Start/Finish	Tasks to do in afternoon	Notes on tasks accomplished or yet to do	Initials
	Lunch and medications/teeth		
	Nap time		
	Activity		
	Laundry and dusting/moping		
	Dental appointment	2:30 p.m., Dr. Desjardins	

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Start/Finish	Tasks to do in evening	Notes on tasks accomplished or yet to do	Initials
	Dinner and medications		
	Teeth brushing and bedtime		

Today, there are electronic programs, smart phones, tablets, etc. to maintain such records. These records become a way for the larger healthcare team to understand the elements that influence the health outcomes and quality of life of the care receiver. Caregivers can also help the team draw conclusions about or relationships between the daily elements of care and alter their design or approach in order to provide the best possible dementia care right at home. They can monitor costs of care and efforts needed to provide such care.

Four elements are needed in the systems approach:

1. Identification (ability to identify the multiple elements of care needed).
2. Description (ability to describe how elements of care operate independently and interdependently).
3. Alteration (ability to change the design and continuously improve processes that promote learning and adaptation to stages).
4. Implementation (ability to integrate new approaches and services needed to facilitate people, processes, facilities, equipment, and organizations in delivering the best possible care).

The process remains the same whether providing care at home or at a long-term care facility. Ideally, the pack leader retains their position and responsibilities as the primary

caregiver to the very end. This allows the pack leader and the care receiver to complete the circle of life, and the pack leader to complete the long but rewarding circle of care for their loved one.

There is no magic to being a pack leader. In fact, there is much heartache, learning, sadness, reward, and human value. Think about a baby developing in its mother's womb, vulnerable as to basic functions of nutrition and sleep; the same is true of someone diagnosed with Alzheimer's or another dementia—they enter the most vulnerable part of their life, and how the pack leader eats and sleeps determines their quality of life.

Caregiver Partnership Agreement Program (CPAP)

In spite of new technology and guidelines, patient/resident safety and quality of care remain challenging.⁹ One of the most serious problems creating enormous economic and personal costs to society is the ratio of workers to residents/patients. It has become an even more serious problem now with the influx of individuals diagnosed with Alzheimer who require specialized care, including greater interaction and teamwork among clinicians, patients/ residents, families, and other stakeholders; processes (institutional, regulatory, professional ethics, etc.); and technology (instrumentation and mobile devices). These needs require us to formulate the best possible systems approach to dementia care and an Alzheimer's-friendly healthcare experience for each care receiver and their family.

A second and equally serious problem in long-term care facilities is the communication gap between professionals and families. Workers do not have time to get to know their residents well enough to meet their emotional and personal needs. This often results in resisting behaviors on the part of

the person living with Alzheimer or another dementia that are subsequently controlled by drugs. These drugs are deadly and only serve to hasten progression of these conditions. Anytime there is a communication breakdown or cross-communication, the hope for cooperation and coordination is lost instantly.

Long-term care facilities are not ready to receive and provide specialized dementia care... yet. The medical/rehab model of care we have known for centuries has yet to be converted to the care/social model suitable for dementia care. The CPAP originally came out of Remembering 4 You and is a blueprint for a care/social model of care, and it's needed everywhere. It not only meets the basic level of care for these individuals and their families, but it saves time, money, and frustration— and even lives, which are too often left to further progress into a state of chaos, pain, and loneliness.

With the emergence of Alzheimer worldwide, the time has come to adopt a socially innovative healthcare program that demonstrates true cooperation, understanding, and teamwork for the good of the individual. The traditional approach (feeding, dressing, and passing out medications) is no longer acceptable. ICA's CPAP enhances care for the resident, increases job satisfaction for the caregiver, and increases the value of the facility in the eyes of its stakeholders. Countries like China and India that do not have long-term care/social institutions for dementia care may now have to invest earnestly in such facilities in order to accommodate the growing number of cases of Alzheimer in their countries. The cost of private care for the course of this condition is much higher than the cost of institutionalization. Many pack leaders and family caregivers end up leaving their full-time work to stay at home and provide

daily care or supervision of care for their loved ones because the care becomes that demanding.

Here in the United States, much needs to be done to increase the quality of care and reduce costs. CPAP is designed to do just that, but also to beef up team formation and teamwork. The concept includes pack leaders and family caregivers as integral parts of the interprofessional team.

Benefits of Adopting an Alzheimer-Friendly Healthcare Workforce Program

Facility/ Business	Management	Professionals	Families
Organizational development shifts to an Alzheimer/dementia-friendly healthcare system within a care/social model of care.	Solid team formation and teamwork.	Increased job satisfaction through team support.	Remaining the pack leader/ primary caregiver for a loved one.
Continue with existing staff and talent.	Added Alzheimer/dementia coach position on the management team.	Increased dementia care training.	Integral part of a larger interprofessional team as a Care Partner.
Access to a certified Alzheimer/dementia coach, internal or external, to the organization.	Closing the communication gap between staff and families.	More time for one-on-one care of individuals with Alzheimer.	Completing the cycle of life and care with formal partnership agreement.

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Attract new long-term residents and keep them longer.	Ongoing data collection and analysis.	Improved family communication.	Exceptional training/ support and up-to-date techniques as a Care Partner.
Instant increased, positive market exposure and a preferred rating.	Program is easily adapted from one location to another using existing staff.	Decreased job-related stress, burnout, and sickness.	Increased positive feelings and appreciation for placement choice.

CPAP's Vision

CPAP's vision is to change the healthcare culture around the world so that individuals living with Alzheimer always encounter and experience an Alzheimer-friendly level of care.

CPAP's Mission

CPAP's mission is to improve the quality of care for individuals living with Alzheimer within an institutional setting, one facility at a time. To ensure the best level of care, the CPAP leading to an Alzheimer-friendly healthcare workforce which includes family members. Hospitals, nursing homes, and assisted living facilities are all encouraged to adopt an Alzheimer-friendly healthcare workforce for the best value and service to care receivers. This model sustains the best possible way by including the following basic business values: accountability, responsibility, value, training, monitoring, and rewards. The program provides a perfect avenue for families to complete their cycle of care and life with their loved one, and for organizations and businesses to be true servants.

CPAP's Guiding Principles

1. Alzheimer's-friendly healthcare is essential for all individuals living with Alzheimer and dementia.
2. An educated healthcare workforce generates the best possible care.
3. Caregiver partnerships include family caregivers ready to volunteer their services.
4. Teamwork is at the heart of an Alzheimer's-friendly interprofessional team.
5. Respect and dignity for every person living with Alzheimer is essential.

CPAP's Philosophy

CPAP revolves around a simple idea: by developing and implementing a care/social model of care versus imposing an outdated and ineffective medical/rehab model of care on care receivers, and by improving the quality of work through teamwork, we improve quality of life for everyone providing care for individuals living with Alzheimer and other dementias. One of the fundamentals of this program's success is to utilize the existing talents and resources of organizations and facilities to enhance quality of life and quality of work by teaming existing talents with families and resources outward, not inward.

Looking forward, we need healthcare providers with a vision and passion to make a difference.

Dementia care requires right-brain thinking and working for a dementia-friendly workforce rather than the left-brain thinking and working we have employed in the medical/ rehab

model of care. We need to educate the healthcare workforce, beginning with family caregivers. Ideally, dementia care standards will be taught in medical schools, nursing programs, and healthcare management programs.

It is recognized that Alzheimer necessitates a whole new approach to healthcare that includes the reintroduction of the interprofessional model of care with the family caregivers, and the ability to adapt with the progression of this condition. The realization that there is no cure on the horizon, and early diagnosis and following treatment to see which therapies work best, therefore experimental at best, is all that there is available. The large majority of dementia care and treatment today is still considered inhumane with locked-down units and grossly undertrained staff.

Management of these facilities and hospitals are equally ignorant of the most basics in dementia care. This is one of the main reasons, among many others, that ICA's goal is to educate and train the workforce if they want to be involved in the care of a care receiver.

Alzheimer/dementia coaches must be part of the interprofessional team as internal or independent coaches within dementia care. Alzheimer is the largest healthcare challenge in the history of mankind. How do we care for these millions in short-vision healthcare leaders and an already-overworked, undertrained healthcare system?

This comment by Kurt Saunders, MSc, PGCE, OSHCR, MIHM, FCMI, on the state of dementia care in the UK was published on LinkedIn on March 30th, 2014:

How many years have forums like this been going on about improving standards? Nothing has changed.

We have 1.5 million people receiving care, and there is another 2 or 3 million in the pipeline. The government and our society don't value the elderly, so funding will never be given for the staff ratio or qualified staff. Within the NHS treatment is rationed, especially if one is over 65 and with dementia.

Our government claim there is no more money, while at the same time urinate over £11 billion in overseas aid.

In domicile care, social services refuse to fund enough home visits, and many care providers are having to accept between £10.50 and £15 per hour to provide qualified care. Many care homes cannot afford to pay higher rates to attract better staff, and there isn't a legal minimum staff ratio per client. This makes any dementia mapping little more than statements of intent with no delivery. Even where clients are paying £1200 per week for dementia care, the qualifications, training, and assessment of staff competence leaves a lot to be desired.

Despite many people's good intentions in the care sector, our care system for the elderly is not fit for purpose, as we shall all find out ourselves at some point!" (Kurt Saunders, 2014). Specific objectives in dementia care are as follows:

- Improve clinical testing for dementia care
- Improve safety and reduce medical errors in dementia care
- Improve wellness, prevention and reversal of symptoms in dementia care
- Improve interprofessional approach to dementia care
- Reduce the high costs and increase efficiency of dementia care services through education
- Improve pay for family caregivers who work 24/7

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- Improve pay for certified nurse assistants and home care workers by upgrading training

The time is now! We all need to improve both clinical and care effectiveness and member/patient/family satisfaction in dementia care.

It is time to “put a new show on the road”; to ask ourselves, “How did I contribute today to someone’s needs who is living with dementia in a respectful, dignified, and effective way to improve their quality of life?”

APPENDIX 1

USA CASE AND FRANCE CASE — CONTAINING DEMENTIA

GAME CHANGER, CONTAINING VASCULAR DEMENTIA IN THE UNITED STATES

Evidence of remission in dementia

The Good Shepherd Healing System™ (GSHS) may be a game-changer for Alzheimer's and dementia. In a small study, all participants achieved remission. The GSHS is a first in its ability to STOP Alzheimer's and dementia in their tracks by offering optimal health to individuals living with either of these conditions.

The World Health Organization (WOW) in 2017 is developing a Global Dementia Observatory (GDO) to facilitate countries around the world in strengthening their health systems to adequately offer support the millions living with dementia and also offer support to their families. The estimate cost of dementia care is estimated at \$604 billion annually in US dollars (see goo.gl/VqS1Yx).

Dementia research is certainly important and needs to continue. Much like Jeff Bezos at Amazon.com is recognized as a game changer for his leadership, Éthelle Lord, DM, along with Richard Rankin, DC, discovered the power of remission in dementia in a small study that ended in June 2016.

Background

In the fall of 2015, I received a call from a friend in Canada who was receiving treatment at a distance working with Dr. Rankin. Her words were “I think you should talk to Dr. Rankin. He can help your husband. His work is remarkable.” My husband, Major Larry S. Potter, USAF Retired, was diagnosed in 2003 with vascular dementia, and more, following his triple bypass in the fall of 1999.

On the same day I picked up the phone and called Dr. Rankin to discuss this matter further and perhaps even set him straight about helping with my husband's dementia. I really did not know what he would say when I dialed his number.



Gary Sinclair, Soul Link

“Results of working with GSHS requires that I start to make you aware of this service for you and possibly someone you love.

I remain in AWE as well as the staff of where my wife is [living based on] the progress she has made in a month! She is the talk of the place.” ~ Gary Sinclair, mymemorymakeover.com, FACEBOOK post on February 27, 2017

To my great surprise and awe, after a few hours on the phone I proposed a small study of individuals living with dementia. These patients were invited to participate with very little evidence of success, since no such study had ever been done. The GSHS is a new perspective on healing.

Participants who volunteered to be part of this study were from different geographic areas. Their diagnoses varied from Alzheimer's and frontal lobe dementia to vascular dementia. One participant was from France, one from the Caribbean, and the others were from different parts of the United States, East to West Coasts.

As serendipity has it, I wanted to discover a way to reduce my husband's symptoms of dementia. After all, in 2010 and 2014 I was able to reverse his symptoms by following recommendations in *Awakening from Alzheimer's* by S. Sarlin. However, each time the reversals did not hold.

Remission of Vascular Dementia: A case study

Larry had been diagnosed with vascular dementia on January 30th, 2003. His father, Buck Potter, one of 12 children born in Brainerd, Minnesota, also lived with vascular dementia. When researching a genetic cause for Larry's dementia, it was discovered that 8 of 12 children in Buck Potter's family had been diagnosed with dementia.

After moving Larry from one care facility to another in August of 2014, only a short 12-mile trip by ambulance, he

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closed his eyes and stopped moving for almost years. He was very much like a person in a coma: little to no eye movement, no clear speech except for moaning and the occasional yelp, and needing total care in his new facility. His neurological impairment was extreme in 2014 (see the list of pre-remission symptoms below the photos).

Photos of Larry Potter in 2015 prior to his remission



Symptoms prior to remission:

- Had to be primed to drink and eat
- Inability to open his eyes and focus
- Inability to sit comfortably or hold a sitting position in his wheelchair
- Inability to speak (only moans and yelps)
- Inability to feed himself or hold a drink
- Inability to participate in any activity
- Total care and in a sleeping position 24/7 whether in or out of bed

What Dr. Rankin was telling me on the phone on that late fall evening was nothing short of amazing. I gave him my complete attention and paid him to do the very first GSHS application on my husband that same evening. This was all done from Arkansas where Dr. Rankin lives and works. My husband, Larry, was in Maine, over 1,600 miles away.

About the Good Shepherd Healing System

The GSHS is a new form of intuitive healing delivered by a GSHS practitioner. It is delivered by voice command to a specific body system. For those living with dementia it is delivered to the brain and other body systems that are responsible for causing dementia (heart, lungs, kidneys, etc). The GSHS practitioner does not diagnose anyone. The person seeking GSHS applications always comes with their own medical diagnosis.



For the purpose of explaining how energy healing works, *energy* refers to anything that vibrates. Every vibration has a frequency, and vibrations are quantified by numbers. For example, voice and sound are two types of energy communication. When a dog whistle is used, the high-

frequency sound it makes has meaning for a dog; however, humans are not able to react to the dog whistle because they do not hear it and therefore it does not have the same meaning for a human as it has for a dog. Although other research is ongoing in energy healing for dementia (e.g. at Harvard), the GSHS is the first approach to achieve remission in dementia patients.

The GSHS is designed to quickly identify the problems in the person living with Alzheimer's or dementia, eliminate and neutralize those problems, and achieve optimal health for the individual. Because we believe that it is possible to ACHIEVE REMISSION IN ALZHEIMER'S, return meaningful living to millions of individuals living with dementia, improve their cognitive and physical abilities, and enable them to experience optimal health, the GSHS is now available to those who have a desire to STOP dementia from progressing and regain as much as possible, cognitively and physically.



Joe Potocny is living with dementia in California

"I got involved (in the GSHS study) because this was one that had no preconceptions of the disease. It involved the whole person. Not just the normal tau proteins and tangles... (GSHS) approached Alzheimer and dementia more as an auto immune disease, where the entire person needed to be treated. It covered

the chemicals in the brain, blood, organs, emotions, physical being and the spiritual side of the individual. Well I finished my current involvement on 5/6/2016.

The conclusion is that I have a 25% improvement in my overall being. Here is what I can tell you. I have lost about 20 pounds. I am finally, after years, back to walking daily. For the first time, in how long, I actually use power tools. On Monday, I helped my daughter do something that I always enjoyed, we sanded down her oak dining table to get it ready to refinish it... I have more energy and do think a little clear[er], but still get confused and have problems getting things from my head to mouth... My writing is easier. The proof is in the pudding, as they say..." ~ Joe Potocny, Author of LIVING WITH ALZHEIMER'S, (goo.gl/ro1vPc)

Remission for Larry Potter

Needless to say, I could barely wait to visit with Larry the day following the first application of the GSHS by Dr. Rankin. I wanted to see for myself if the GSHS application had any effect on him. To my amazement and that of his care providers, his eyes were now wide open. He was able to look all around and focus on sounds, people, and events. With tears in my eyes I said, "Larry! I've been waiting here for you for so long. Welcome back!" He was present and looking straight at me for the first time in about two years.

Photo of Larry Potter in 2016 after his remission



Larry Potter with his wife, Éthelle

Significance of remission for Larry Potter, a case study

Larry gained the ability to:

- Speak full sentences
- Move, drink without assistance, and eat finger foods
- Ask and answer questions requiring left-brain functions
- Participate in and enjoy activities such as outings
- Listen to and enjoy recorded books, especially the Harry Potter series
- Take phone calls and FACETIME or SKYPE calls
- Attend and participate in his own care-plan meetings

After his remission, Larry has been able to keep his eyes opened during his awakened hours. He also began to speak words, form sentences, ask questions, answer questions, shake hands with others, be coordinated with his hand and arm movements in the wheelchair, and much more. Time was on his side now. *“To hear his voice again with the same intonation and feelings he used to have was nothing short of a miracle to me. I continue to be amazed at his daily progress.”* ~ Ethelle Lord

Summary

The GSHS is a visionary healing application that is a game changer. No longer is there a mystery in what has been afflicting millions of individuals in every country around the globe. According to the World Health Organization (WOW) in a 2012 report, dementia is a public health priority (see goo.gl/jg713).

The significance of remission in dementia and Alzheimer is immeasurable. There are cognitive and physical improvements we can measure. However, the emotional and spiritual benefits are different for every person and their family. Remission offers families who fear their loved one is “disappearing” before their very eyes due to their dementia an opportunity to “re-appear”, live a better quality of life, and experience optimal health.

The high cost of dementia care is bankrupting every nation around the world. By introducing remission, the cost of dementia care is greatly reduced since independence is often returned to the person diagnosed with Alzheimer or dementia. They suddenly are able to start a finish a task without being supervised. Families are able to feel the freedom from having

to suffer from watching their family member slowly losing language and physical abilities.

The GSHS is a recent discovery (2016) and therefore more study needs to be done to see that improvements continue over time. Larry Potter continues to make progress daily and it is expected that he will walk again in the future. Various care providers and family members have expressed their amazement at the progress he had made so far. The future remains bright knowing remission has changed the course of his dementia.

The next article will outline another case study of remission due to the GSHS. The International Caregivers Association continues to be a leader in dementia care and changing the course of dementia care. For more information on this article or to set up a GSHS consultation, please go to www.icareassoc.com

The content provided by the author is offered on an informational basis only and as a service made available to those interested in achieving remission of Alzheimer or dementia. The information provided in this article applies to one participant only and does not address any other circumstances. The Good Shepherd Healing System (GSHS) does not require any herbal or nutritional supplements, any special diet, or any physical activity. The GSHS does not cause unwanted side effects, death, or worsening of an existing condition as it uses energy to create optimal health in each individual.

GAME CHANGER, CONTAINING ALZHEIMER'S IN FRANCE

Introduction

To date there are no effective treatments available for those diagnosed with Alzheimer's or dementia, and this in spite of millions spent on scientific research. Relief for family care providers and the medical community continues to elude us. Everyone is frustrated about what to say and what to do to improve the care of millions of individuals all over the world.

The mission of the International Caregivers Association (www.icareassoc.com) is to change the course of dementia care. Our small, privately funded study demonstrated the GSHS, energy medicine, may hold a key to achieving relief of symptoms and remission of a patient's recovery from Alzheimer's and dementia.

Alzheimer's and dementia is considered an irreversible, progressive disease by the medical community and the Alzheimer's Association. This case study demonstrates that it is indeed possible to achieve remission of Alzheimer's and dementia (also see goo.gl/lg80jP). It is promising news in the field of dementia, however more studies must be conducted using the GSHS.

Background

The study was self-funded by Ethelle Lord, DM, and Richard Rankin, DC. All GSHS treatments were performed by

Dr. Rankin. Dr. Lord was responsible for assisting and record keeping.

Every morning for over a year, Alban Maino, founder of Memory-Lane.tv called his grandmother, Rosette, in France, from Maine, to sing along with her. He made short film about his grandmother when she moved into a care facility in France, <https://www.youtube.com/watch?v=4UaYRJwQhrQ>.

As long as she could sing along and recognize his voice every morning for a period of a few years, Alban was a happy man and grandson. Then she became seriously ill; was hospitalized in France; and I received a call from him.

Rosette, a 93-year-old woman in an advanced stage of Alzheimer's, was one of seven participants in this small study. Every GSHS treatment was done from a distance. She was in France, and the GSHS practitioner, Dr. Rankin, was in Arkansas in the United States.

Consent for participating in the study was received after Alban phoned about her imminent end-of-life condition. She was accepted in the study in February of 2016. The first GSHS treatment was done the following day.

The main phenomenon of this case study is that Rosette's daughter (Alban's mother), her family care provider, had been informed by Rosette's doctor to return Rosette from the hospital to the care home, get hospice services for her, and make her funeral arrangements right away. But after the GSHS was used, Rosette enjoyed quality of life and optimal health up to the time of her death. Sadly, Rosette died of natural causes last summer.

1. An email message from Rosette's daughter to Alban dated February 27th, 2016, reads as follows: "...tres fatiguée

depuis 15 jours, pouvant à peine parler, alitée toute la journée...” (Rough English translation: ...very tired for the past 15 days, hardly able to speak, in bed all day... ”)



Rosette returned from hospital per her doctor's order. Picture was taken by her daughter on February 27th, 2016.

2. Two days after the first email was received, a second email dated February 29th, 2016, was sent to Alban from his mother and reads as follows: “...assise, maquillée, repose, il est 10h du matin, je suis surprise du changement...” (Rough English translation: “...sitting, makeup on, rested, it is 10 a.m., I am surprised by the changes...”)



Rosette, photo taken by her daughter on February 29th, 2016 after her first GSHS treatment for remission of Alzheimer's.

3. A third email to Alban reporting Rosette's GSHS progress dated March 2nd, 2016, reads as follows: "...je la retrouve courbée sur son deambulateur dans le couloir, elle peine mais marche. absente parfois sereine. elle se souvient de certaines choses de sa mémoire immédiate telle que le nom de son amie qu'elle avait complètement perdu." (Rough English translation: "...I found her bent over her walker in the hall, she is barely able to walk, appears absent and sometimes serene, she remembers certain short-term memories such as the name of her best friend, which she could never recall before this time.")
4. Finally, a fourth email to report Rosette's remission progress dated March 4th, 2016, reads as follows: "... dans son fauteuil elle m'attend écoute sa musique, espère venir dimanche à la maison, je la questionne elle répond, toujours absente parfois...le personnel me dit qu'elle trouve son déambulateur trop lourd préfère être dans le fauteuil roulant pour se déplacer."

5. (Rough English translation: "...found her sitting in her armchair, waiting for me and listening to her music, she hopes to come home on Sunday. I asked her some questions and she answered me, appears absent at times... the staff tells me that she finds her walker too heavy to use and prefers to use the wheelchair as her mode of transportation.")



Rosette, a month after her first GSHS treatment. Picture taken by her daughter on March 2nd, 2016.

6. In addition to the email reports from Rosette's primary caregiver, her daughter, Alban, her grandson, reported the following in the last week of March of 2016 when talking about his grandmother: "Good progress this week!" There were no details in his short comment.

Conclusion

One of the requirements of research participants was that an observer report any and all progress, good or poor, each time behavioral, physical, or emotional changes were observed. Several participants reported only occasionally when time permitted. All reported progressive cognitive and physical improvements in activities of daily living.

Rosette's progressive improvements were recorded in a series of emails and published in this article for the reader. As evidenced by the report and photos sent by her daughter to her grandson, remarkable physical, emotional, and psychological changes were recorded over a 30-day period in 2016. All these remarkable changes were observable following the first GSHS treatment and within only 30 days following the treatment.

Summary

Rosette was able to greatly reduce her symptoms, speak and recall lost information, and most of all she was able to once more enjoy her visits with family and friends. Much like one other research participants, she was able to have an awareness of her environment to the time of her death. The GSHS will one day be widely used to achieve remission in those diagnosed with Alzheimer's or dementia. This treatment is equivalent to dropping an atomic bomb into the healthcare system, with dementia as its target.

Because quality of life and quality of care are immediately affected after only one GSHS treatment, this treatment has the potential to greatly reduce the high cost of dementia care for families and healthcare budgets.

Every change reported by the care provider for those in this study was reviewed and analyzed directly afterwards by Drs. Lord and Rankin. Some of the reports were verbal and noted by Dr. Lord for the records.

The content provided by the author is offered on an informational basis only and as a service made available to those interested in achieving remission of Alzheimer or dementia. The information provided in this article applies to one participant only and does not address any other circumstances. The Good Shepherd Healing System (GSHS) does not require any herbal or nutritional supplements, any special diet, or any physical activity. The GSHS does not cause unwanted side effects, death, or worsening of an existing condition as it uses energy to create optimal health in each individual.

For more information, please contact the author at lordethelle@gmail.com or <http://www.internationalcaregiversassociation.com/>

APPENDIX 2

NLP ASSESSMENT

For each of the following statements, please allocate a number to every phrase, ranking them from 4 to 1. Use the following system to indicate your preferences:

- 4 = Closest to describing you
3 = Next closest description
2 = Somewhat describes you
1 = Least descriptive of you

Example:

1. I make important decisions based on:

- 4 my gut feeling and comfort level
3 how the idea sounds to me
1 how it looks to me
2 precise review and study of the issues

1. I make important decisions based on:

- ___ my gut feeling and comfort level
___ how the idea sounds to me
___ how it looks to me
___ precise review and study of the issues

2. During a disagreement, I am most likely to be influenced by:

- ___ the volume and tone of the other person's voice
___ whether or not I can see the other person's point of view

- ___ the logic and rationale of the other person's opinion
- ___ whether or not the other person is sensitive to my feelings

3. When communicating with others what's important to me is:

- ___ the way I dress and look
- ___ sharing my feelings and experiences
- ___ knowing the meaning of my words
- ___ being heard and listened to

4. When someone is asking me to important question, I tend to:

- ___ listen carefully and then ask questions to ensure I understand
- ___ prefer time to think it over and to choose my words carefully
- ___ appreciate being given time to search inside for the answer
- ___ answer quickly, describing it in pictures

5. I would consider myself:

- ___ attuned to the sounds of my surroundings
- ___ able to easily make sense of new facts and data
- ___ sensitive and flexible in my relationships
- ___ creative and able to handle tremendous amounts of information quickly

6. People really know me best when they:

- ___ can relate to what I'm feeling
- ___ can see my perspective
- ___ listen carefully to what I have to say and how it is said

ALZHEIMER'S COACHING

___ are interested in the meaning of what I am communicating

7. When working on a project with other people, I am more likely to:

- ___ want to improve the process with my ideas
- ___ want to be part of the vision and planning process
- ___ want to sequence the events and put things in order
- ___ want to help build good solid relationships

8. When describing things to me:

- ___ showing it to me brings clarity
- ___ I can remember well just by listening
- ___ writing it down helps me to integrate it
- ___ presenting the facts in a logical way makes sense

9. In times of stress, I am most challenged with:

- ___ trusting people, situations, or concepts
- ___ being diplomatic, being too blunt and to the point
- ___ separate my feelings from what other people are feeling
- ___ being flexible and changing the timing of plans

10. I find it easy and natural to:

- ___ receive inner inspirations
- ___ tell where new ideas fit in
- ___ follow the direction of tried and true methods
- ___ organize and plan events

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For information and classes on NLP, visit Linda's website EverydaySoulWorks.com.

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This assessment was obtained, with permission, from Dr. Ethelle Lord at www.Remembering4You.com

207-764-1214.

Info@remembering4you.com

APPENDIX 3

CALCULATING NLP ASSESSMENT RESULTS

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For information and classes on NLP, visit Linda's website
EverydaySoulWorks.com **Step 1 of 2:**

Copy your answers from the test to the lines below.

Example:

1.

__4__K __3__A
__1__V
__2__D

1.

__K

2.

__A

3.

__V

4.

__A

5.

__A

__A

__V

__K

__D

__D

__V

__D

__D

__K

__K

__D

__K

__A

__V

__V

6.

__K

7.

__A

8.

__V

9.

__D

10.

__D

__V

__V

__A

__A

__A

__A

__D

__K

__K

__K

__D

__K

__D

__V

__V

Step 2 of 2:

Copy the numbers that are associated with each letter.

Sample:

	V	K	A	D
1.	1	4	3	2
	V	K	A	D
1.	x	X	x	x
2.	x	X	x	x
3.	x	X	x	x
4.	x	X	x	x
5.	x	X	x	x
6.	X	x	x	x
7.	X	x	x	x
8.	X	x	x	x
9.	X	x	x	x
10.	X	x	x	x
Totals	X	x	x	x
	V	K	A	D

To check, add up $V+K+A+D=100$

For example: $K=29$, $V=27$, $D=24$, $A=20$

V = Visual, K = Kinesthetic; A = Auditory;
D = Auditory-Digital

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APPENDIX 4APPENDIX 4

NLP MATRIX

Understanding Communication Styles Using

NLP—Neuro-Linguistic Programming

Processing Order (Representation System)

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Linda Storey

■Visual—*Sight* ■Auditory—*Hearing* ■Kinesthetic— *Touch/
Feeling* ■Auditory Digital—*Words*

	Visual	Auditory	Kinesthetic	Auditory Digital
Common Characteristics	They memorize by seeing pictures, and are less distracted by noise. They often have trouble remembering	They typically are easily distracted by noise. They can repeat things back to you easily, learn by listening,	They often talk slowly. They respond to physical rewards, and touching.	This person spends a fair amount of time talking to themselves.

DR. ETHELLE LORD

	verbal instructions because their mind tends to wander.	like music, and like to talk on the phone.	They memorize by doing or walking through something.	They memorize by steps, procedures, and sequences.
Describing to them	They are interested by how the program looks.	Tone of voice and the words used are important.	They will be interested in a program that <i>feels right/ gut-feelings.</i>	They will want to know if your program makes sense.
Commonly used words	See Look Appear View Show Imagine Crystalize	Hear Listen Sound Tune in/out Click Ring a bell	Feel Touch Touch base Get hold of Comfortable Catch on Play, Together	Sense Experience Think Understand Think Process Decide I Know
Speech Pattern	Quickly grouped words	Lots of interruptions and “uh”, “um”, “ah”, etc.	Deliberate phrasing	Long complicated sentences
Processing Patterns	Quickly with minimum of detail	Will let you know unconsciously when they understand by changing the subject	Extensive detail	Will not give indication of understanding unless you ask
Often Asks	When? When does it start, when does it finish?	Where? Where can I use this model? Where can I make it better?	Why? Why would you do it this way?	What if? What would happen if...

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Provided to you with permission by Dr. Ethelle Lord at
Remembering4You: www.Remembering4You.com,
telephone: 207-764-1214,
e-mail: info@remembering4you.com

APPENDIX 5

BELBIN TEAM ROLES

Team Role	Contribution	Allowable Weaknesses
Plant	Creative, imaginative, freethinking, generates ideas, and solves difficult problems.	Ignores incidentals, too preoccupied to communicate effectively.
Resource Investigator	Outgoing, enthusiastic, communicative. Explores opportunities and develops contacts.	Overoptimistic. Loses interest once initial enthusiasm has passed.
Coordinator	Mature, confident, identifies talent, and clarifies goals. Delegates effectively.	Can be seen as manipulative. Offloads own share of the work.
Shaper	Challenging, dynamic, thrives on pressure. Has the drive and courage to overcome obstacles.	Prone to provocation. Offends people's feelings.
Monitor Evaluator	Sober, strategic, and discerning. Sees all options and judges accurately.	Lacks drive and ability to inspire others. Can be overly critical.

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Team Worker	Cooperative, perceptive, and diplomatic. Listens and averts friction.	Indecisive in crunch situations. Avoids confrontation.
Implementer	Practical, reliable, efficient. Turns ideas into actions and organizes work that needs to be done.	Somewhat inflexible. Slow to respond to new possibilities.
Complete Finisher	Painstaking, conscientious, and anxious. Searches out errors. Polishes and perfects.	Inclined to worry unduly. Reluctant to delegate.
Specialist	Single-minded, self-starting, and dedicated. Provides knowledge and skills in rare supply.	Contributes only on a narrow front. Dwells technicalities.

APPENDIX 6

ANSIR MATRIX OF STYLES

Styles	Thinking Realm	Working Realm	Emoting Realm
Diligent	*Follow the standard *Know rules *Facts and figures	*Processing details *Spot errors *Reliability, thy name is Diligent	*Detached *Not influenced by emotions *Not touchy-feely
Eccentric	*Play with ideas *Bright and clever *Too weird to work	*Bend the rules *Gravitate to the cutting edge *Get a rise out of you	*Searching for the spark *Noncommittal *Independent
Empath	*Make everybody happy *Exceptional concern for others	*Caring, nurturers *Find comfort in helping others	*Need to <i>connect</i> emotionally *Easily laugh or cry *Can remain in hurtful relationships
Evokateur	*Imaginative dreamers *Not the most attentive	*They are creative originals	*Life of fantasy *Introverted and often lonely

ALZHEIMER'S COACHING

Extremist	*Results over consequences *Tacticians and strategists	*The risk-taking daredevil!	*Bold and adventurous *Relationship difficulties *No strings
Healer	*Long-term over short-term *Focus on the source	*Workplace chameleon *Everything affects the whole	*Hearts to love all *Jealous mates
Idealist	*Quick to judge and conclude *Be smarter and win	*Money + power = power *End justifies the means *Quickly up the corporate ladder	*Perfectionist *Strive to win in high society *Sacrifice their relationships
Kinsmen	*Poll opinions *Able talkers *Masters of the common ground	*Compromise is a powerful tool *World-class people skills	*Hard on themselves *Need to feel helpful *Prone to <i>the blues</i>
Philosopher	*They need to understand—holistically *Natural researchers	*Anti-establishment *Rules inhibit individuality *Seek accreditation	*Sense change *Shake up the stagnant *Often unpopular
Realist	*Right and wrong *Honest *Stubborn, thy name is Realist	*Mr./Ms. Fix-it *Love to build *Smart hands	*Protector and provider *The strong don't cry
Sage	*Acquiring over application *Solve complex problems	*Their interest is where they start *It just doesn't matter	*Detached and intellectual *Forgotten or misunderstood *Observers only

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Scintillator	*Optimistic *Infectious humor *Keep things moving	*Morale boosters *Create shortcuts *Fidgety, entertaining	*Sleeping giant *Deflect fear with humor *Physically feel emotional pain
Sentinel	*Expect the predictable *New ideas get scrutiny	*Enforcers of fairness *Pride and integrity	*Private and often lonely
Visionary	*The ultimate <i>idea</i> person *Prized in a business setting	*Inner tick-tock drives them *Natural leaders	*Stagnant or mundane = “Good-bye”

APPENDIX 7

CAREGIVER FATIGUE TIMELINE

[from International Caregivers Association]

There is a common challenge that people share, when providing care for people living with long term and degenerative diseases such as Alzheimer's, multiple sclerosis, brain injuries and stroke, and some forms of mental illness. Because of its long-term nature, these caregivers are at high risk of burnout. Burnout is defined as a state of physical, emotional, and mental exhaustion that may be accompanied by a change in attitude. At its most acute stages burnout may also lead to a feeling of hopelessness that can in some cases result in homicidal/suicidal tendencies.

To paraphrase a popular quote on depression... burnout *“is not a sign of weakness. It's a sign that you have been strong for too long”*.

It is important to recognize the symptoms of burnout and to seek professional help when you or the caregiver needs it. Remember that you are in many ways also living with Alzheimer's and dementia. The caregiver fatigue timeline below reflects the results of numerous health and wellness research studies on full-time caregiving. People living with

Alzheimer's may require years, even decades, of specialized care by a family member. Therefore, knowledge of the signs of caregiver fatigue is a common sense thing to look out for by family members and family doctors.

Caregiver Fatigue Timeline

At 1-18 Months

(after the diagnosis of dementia)

Caregiver finds themselves

- anxious to provide the best possible care for loved one
- managing the person with dementia
- taking on responsibility for maintaining the house, garden, car
- attending to family relations
- “keeping up appearances”
- helping person with dementia through social situations
- remaining optimistic, caring, supportive
- operating as “superwoman/superman”
- attending to personal care

At 21 Months

Caregiver may begin to show signs like those below that suggest they are becoming more and more fatigued:

- Begin to take medication, usually for sleep/headaches
- Find it harder and harder to keep on top of things
- Find some help from family still available
- Loosing or gaining weight

24-32 Months

Caregiver's fatigue had grown to where their

- Emotional and physical resources are clearly drained
- contact with their own personal doctor, dentist, minister, friends, is less and less

Caregiver burnout may express itself as

- Experiencing feelings of powerlessness
- realizing that their caregiving now consumes their whole day and night
- outside help dwindles away

At 32 Months

Caregiver may find that

- their stress becomes harder to conceal
- they may begin taking tranquilizers to cope with fatigue
- they may begin using medication for musculoskeletal pain
- Sleep is continually disturbed
- they are gradually becoming more and more irritable
- they have less and less contact with others

By 38 Months

Caregiver

- feels unhealthy
- Finds it hard to get up

- Never feels rested
- May have hypertension/colitis
- may exhibit symptoms of chronic fatigue
- Caregiver loses the will to take care of themselves
- Is unable to manage the household
- Rarely socializes with others
- Feels helpless, guilty, a failure

After 50 Months

Caregiver may experience

- a chronic state of fatigue
- a state of “un-wellness”
- an inability to ask for help
- feelings of isolation
- inability to access resources for information or help

If you or someone in your family is a family caregiver, please share this list with family and your family doctor. Your family doctor will be able to monitor your health along with the health of the person you provide care to.

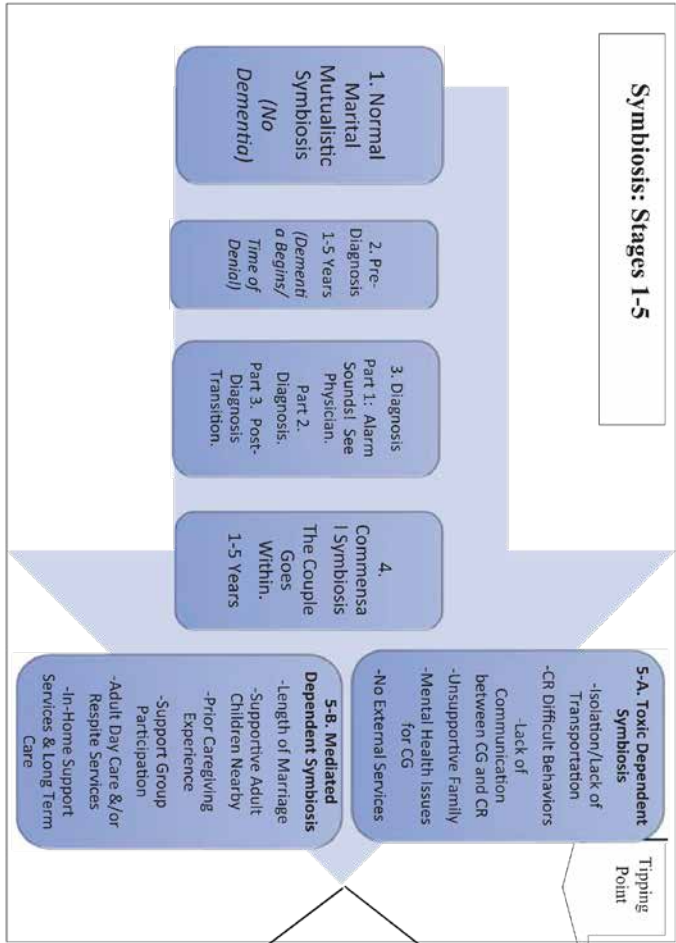
Source:

<http://www.snocare.org/kit/03CaregiverFatigueTimeline.pdf>

Join the International Caregivers Association to stay informed & see what is changing the course of dementia care (i.e., containment and remission of dementia is now possible) www.icareassoc.com

APPENDIX 8

STAGES OF SYMBIOSIS



APPENDIX 9

INTERNATIONAL COACH FEDERATION

Core Competencies

The following eleven core coaching competencies were developed to support greater understanding about the skills and approaches used within today's coaching profession as defined by the International Coach Federation. They will also support you in calibrating the level of alignment between the coach-specific training expected and the training you have experienced.

Finally, these competencies and the ICF definition were used as the foundation for the ICF credentialing process examination. The ICF defines coaching as partnering with clients in a thought-provoking and creative process that inspires them to maximize their personal and professional potential. The core competencies are grouped into four clusters according to those that fit together logically based on common ways of looking at the competencies in each group. The groupings and individual competencies are not weighted—they do not represent any kind of priority in that they are all core or critical for any competent coach to demonstrate.

ALZHEIMER'S COACHING

A. Setting the foundation

1. Meeting ethical guidelines and professional standards
2. Establishing the coaching agreement

B. Co-creating the relationship

3. Establishing trust and intimacy with the client
4. Coaching presence

C. Communicating effectively

5. Active listening
6. Powerful questioning
7. Direct communication

D. Facilitating learning and results

8. Creating awareness
9. Designing actions
10. Planning and goal setting
11. Managing progress and accountability

A. Setting the Foundation

1. **Meeting Ethical Guidelines and Professional Standards—**
Understanding of coaching ethics and standards and ability to apply them appropriately in all coaching situations.
 1. Understands and exhibits in own behaviors the ICF Standards of Conduct (see list, part III of ICF Code of Ethics).
 2. Understands and follows all ICF Ethical Guidelines (see list).

3. Clearly communicates the distinctions between coaching, consulting, psychotherapy, and other support professions.
 4. Refers client to another support professional as needed, knowing when this is needed and the available resources.
- 2. Establishing the Coaching Agreement—** Ability to understand what is required in the specific coaching interaction and to come to agreement with the prospective and new client about the coaching process and relationship.
1. Understands and effectively discusses with the client the guidelines and specific parameters of the coaching relationship (e.g., logistics, fees, scheduling, and inclusion of others if appropriate).
 2. Reaches agreement about what is appropriate in the relationship and what is not, what is and is not being offered, and about the client's and coach's responsibilities.
 3. Determines whether there is an effective match between his/her coaching method and the needs of the prospective client.
- B. Co-Creating the Relationship**
- 3. Establishing Trust and Intimacy with the Client—** Ability to create a safe, supportive environment that produces ongoing mutual respect and trust.
1. Shows genuine concern for the client's welfare and future.
 2. Continuously demonstrates personal integrity, honesty, and sincerity.
 3. Establishes clear agreements and keeps promises.

ALZHEIMER'S COACHING

4. Demonstrates respect for client's perceptions, learning style, and personal being.
 5. Provides ongoing support for and champions new behaviors and actions, including those involving risk taking and fear of failure.
 6. Asks permission to coach client in sensitive, new areas.
4. **Coaching Presence**—Ability to be fully conscious and create a spontaneous relationship with the client, employing a style that is open, flexible, and confident.
1. Is present and flexible during the coaching process, dancing in the moment.
 2. Accesses own intuition and trusts one's inner knowing—goes with the gut.
 3. Is open to not knowing and takes risks.
 4. Sees many ways to work with the client and chooses in the moment what is most effective.
 5. Uses humor effectively to create lightness and energy.
 6. Confidently shifts perspectives and experiments with new possibilities for own action.
 7. Demonstrates confidence in working with strong emotions, and can self-manage and not be overpowered or enmeshed by client's emotions.
- C. Communicating Effectively
5. **Active Listening**—Ability to focus completely on what the client is saying and is not saying, to understand the meaning of what is said in the context of the client's desires, and to support client self-expression.

1. Attends to the client and the client's agenda and not to the coach's agenda for the client.
 2. Hears the client's concerns, goals, values, and beliefs about what is and is not possible.
 3. Distinguishes between the words, the tone of voice, and the body language.
 4. Summarizes, paraphrases, reiterates, and mirrors back what client has said to ensure clarity and understanding.
 5. Encourages, accepts, explores, and reinforces the client's expression of feelings, perceptions, concerns, beliefs, suggestions, etc.
 6. Integrates and builds on client's ideas and suggestions.
 7. "Bottom-lines" or understands the essence of the client's communication and helps the client get there rather than engaging in long, descriptive stories.
 8. Allows the client to vent or "clear" the situation without judgment or attachment in order to move on to next steps.
- 6. Powerful Questioning**—Ability to ask questions that reveal the information needed for maximum benefit to the coaching relationship and the client.
1. Asks questions that reflect active listening and an understanding of the client's perspective.
 2. Asks questions that evoke discovery, insight, commitment or action (e.g., those that challenge the client's assumptions).
 3. Asks open-ended questions that create greater clarity, possibility, or new learning.

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4. Asks questions that move the client toward what they desire, not questions that ask for the client to justify or look backward.
7. **Direct Communication**—Ability to communicate effectively during coaching sessions and to use language that has the greatest positive impact on the client.
 1. Is clear, articulate, and direct in sharing and providing feedback.
 2. Reframes and articulates to help the client understand from another perspective what he/she wants or is uncertain about.
 3. Clearly states coaching objectives, meeting agenda, and purpose of techniques or exercises.
 4. Uses language appropriate and respectful to the client (e.g., nonsexist, nonracist, nontechnical, and nonjargon).
 5. Uses metaphor and analogy to help illustrate a point or paint a verbal picture.

D. Facilitating Learning and Results

8. **Creating Awareness**—Ability to integrate and accurately evaluate multiple sources of information and to make interpretations that help the client to gain awareness and thereby achieve agreed-upon results.
 1. Goes beyond what is said in assessing client's concerns, not getting hooked by the client's description.
 2. Invokes inquiry for greater understanding, awareness, and clarity.
 3. Identifies for the client his/her underlying concerns; typical and fixed ways of perceiving himself/herself

and the world; differences between the facts and the interpretation; and disparities between thoughts, feelings, and action.

4. Helps clients to discover for themselves the new thoughts, beliefs, perceptions, emotions, moods, etc. that strengthen their ability to take action and achieve what is important to them.
 5. Communicates broader perspectives to clients and inspires commitment to shift their viewpoints and find new possibilities for action.
 6. Helps clients to see the different, interrelated factors that affect them and their behaviors (e.g., thoughts, emotions, body, and background).
 7. Expresses insights to clients in ways that are useful and meaningful for the client.
 8. Identifies major strengths vs. major areas for learning and growth, and what is most important to address during coaching.
 9. Asks the client to distinguish between trivial and significant issues, situational vs. recurring behaviors, when detecting a separation between what is being stated and what is being done.
9. **Designing Actions**— Ability to create with the client opportunities for ongoing learning, during coaching and in work/life situations, and for taking new actions that will most effectively lead to agreed-upon coaching results.
1. Brainstorms and assists the client to define actions that will enable the client to demonstrate, practice, and deepen new learning.

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2. Helps the client to focus on and systematically explore specific concerns and opportunities that are central to agreed-upon coaching goals.
 3. Engages the client to explore alternative ideas and solutions, to evaluate options, and to make related decisions.
 4. Promotes active experimentation and self-discovery, where the client applies what has been discussed and learned during sessions immediately afterward in his/her work or life setting.
 5. Celebrates client successes and capabilities for future growth.
 6. Challenges client's assumptions and perspectives to provoke new ideas and find new possibilities for action.
 7. Advocates or brings forward points of view that are aligned with client goals and, without attachment, engages the client to consider them.
 8. Helps the client "do it now" during the coaching session, providing immediate support.
 9. Encourages stretches and challenges but also a comfortable pace of learning.
- 10. Planning and Goal Setting**—Ability to develop and maintain an effective coaching plan with the client.
1. Consolidates collected information and establishes a coaching plan and development goals with the client that address concerns and major areas for learning and development.
 2. Creates a plan with results that are attainable, measurable, specific, and have target dates.

3. Makes plan adjustments as warranted by the coaching process and by changes in the situation.
4. Helps the client identify and access different resources for learning (e.g., books, other professionals).
5. Identifies and targets early successes that are important to the client.

11. Managing Progress and Accountability— Ability to hold attention on what is important for the client and to leave responsibility with the client to take action.

1. Clearly requests of the client actions that will move the client toward his/her stated goals.
2. Demonstrates follow-through by asking the client about those actions that the client committed to during the previous session(s).
3. Acknowledges the client for what they have done, not done, learned, or become aware of since the previous coaching session(s).
4. Effectively prepares, organizes, and reviews with client information obtained during sessions.
5. Keeps the client on track between sessions by holding attention on the coaching plan and outcomes, agreed-upon courses of action, and topics for future session(s).
6. Focuses on the coaching plan but is also open to adjusting behaviors and actions based on the coaching process and shifts in direction during sessions.
7. Is able to move back and forth between the big picture of where the client is heading, setting a context for what is being discussed and where the client wishes to go.

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8. Promotes client's self-discipline and holds the client accountable for what they say they are going to do, for the results of an intended action, or for a specific plan with related time frames.
9. Develops the client's ability to make decisions, address key concerns, and develop himself/herself (to get feedback, to determine priorities and set the pace of learning, and to reflect on and learn from experiences).
10. Positively confronts the client with the fact that he/she did not take agreed-upon actions.

Source: <http://www.coachfederation.org/icfcredentials/core-competencies/>

APPENDIX 10

***VIRGIN COCONUT OIL: A TRUE MIRACLE FOOD**

Properties include: Anti-Aging, Weight Loss, Destroys Bacteria and Viruses, Enhances the Immune System, and Many More Benefits.

One of the questions I recently received: Is coconut oil as good as they say it is? I actually loved this question. Thank you for asking, Dana... because it gave me the opportunity to go back and remember as to why it is part of my own daily diet, and I hope it will become one of yours as well.

Coconut oil has been used for thousands of years— and those who have used it are healthier than those who have used the polyunsaturated oils, such as canola, flaxseed, corn, safflower, and sunflower since the 1950s. Why did this change occur? People were told that these unsaturated fats were better, but results have proven otherwise— weight gain, heart disease, and thyroid problems increased.

Before the 1950s, heart attacks were not common in Sri Lanka, where coconut consumption was ultra-high.

However, coconut consumption dropped because of the saturated fat scare, and hospital admissions for heart attacks rose drastically from 1972–1992. The 1978 edition of the Demographic Yearbook of the United Nations reported that Sri Lanka had the lowest death rate from ischemic heart disease, while coconut oil was their main dietary fat.

So how does this happen to those in the tropics that use coconut oil? How can people on their traditional high-saturated fat coconut diet keep trim and healthy?

Researchers have known for quite some time that the secret to health and weight loss associated with coconut oil is related to the length of the fatty acid chains contained in coconut oils. Coconut oil contains what are called medium-chain fatty acids or medium-chain triglycerides (MCTs). These medium-chain fatty acids are different from the longer-chain fatty acids found in other plant-based oils. Most vegetable oils are composed of longer-chain fatty acids or triglycerides (LCTs). LCTs are typically stored in the body as fat, while MCTs are burned for energy. MCTs burn up quickly in the body.

Coconut oil is nature's richest source of MCTs. Not only do MCTs raise the body's metabolism leading to weight loss, but they have special health-giving properties as well. The most predominant MCT in coconut oil, for example, is lauric acid. Lipid researcher Dr. Jon Kabera states, "Never before in the history of man is it so important to emphasize the value of lauric oils. The medium-chain fats in coconut oils are similar to fats in mother's milk and have similar nutraceutical effects. These health effects were recognized centuries ago in Ayurvedic medicine. Modern research has now found a common link between these two natural health foods—their fat or lipid content. The medium-chain fatty acids and

monoglycerides found primarily in coconut oil and mother's milk have miraculous healing powers."

Outside of a human mother's breast milk, coconut oil is nature's most abundant source of lauric acid and medium-chain fatty acids.

Much of the research done on coconut oil and lauric acid, the most predominant fatty acid chain found in coconut oil, has centered around the antimicrobial and antiviral properties of this unique fatty acid. Today, many strains of bacteria are becoming resistant to antibiotics, and antibiotics are generally ineffective in treating virus infections. When lauric acid is consumed in the diet, either in human breast milk or in coconut oil, lauric acid forms a monoglyceride called monolaurin, which has been known to destroy several bacteria and viruses, including *Listeria monocytogenes* and *Helicobacter pylori*, and protozoa such as *Giardia lamblia*. Some of the viruses that have been destroyed by monolaurin include HIV and cytomegalovirus. There is also evidence now that the MCTs in coconut oil kill yeast infections such as Candida.

How did coconut oil get such a bad reputation? It started in the 1950s when public opinion toward saturated fats in general, and then later toward coconut oil in particular, began to turn negative. This history of the edible oil industry in the United States has been well documented by Dr. Mary Enig, PhD, and can be read at www.coconutoil.com or at the Weston Price Foundation website (www.westonprice.org). Her articles, "The Oiling of America" and "Coconut: In Support of Good Health in the 21st Century" provide in-depth analysis of the saturated fat research, and the negative campaigns that have been waged against coconut oil.

What we know today, but was not well known in the 1950s, is that hydrogenated and partially hydrogenated vegetable oils create trans-fatty acids that have been linked to heart disease.

Dr. Mary Enig states: The coconut industry has suffered more than three decades of abusive rhetoric from the consumer activist group Center for Science in the Public Interest (CSPI), from the American Soybean Association (ASA), and other members of the edible oil industry, and from those in the medical and scientific community, who learned their misinformation from groups like CSPI and ASA. According to one of CSPI's own press releases, "In 1984, CSPI organized the first national campaign to pressure fast-food restaurants and food companies to stop frying with beef fat and tropical oils, which are high in the cholesterol-raising saturated fats that increase the risk of heart disease. After six years of public pressure— including full-page newspaper ads placed by Nebraska millionaire and cholesterol crusader Phil Sokolof—the industry finally relented in 1990."

Congress held hearings in 1988 to discuss the safety of tropical oils. Dr. George Blackburn, a Harvard medical researcher, testified that coconut oil has a neutral effect on blood cholesterol, even in situations where coconut oil is the sole source of fat. Surgeon General C. Everett Koop dismissed the entire attacks on coconut oil as "foolishness" and continued to say "but to get the word out to commercial interests terrorizing the public about nothing is another matter." However, with no strong political influence in Washington from the coconut-producing countries, the ASA and CSPI prevailed and soon coconut oil almost vanished from the American diet, today it has been replaced by the so-called "healthier" vegetable oils.

What have replaced saturated fats are now liquid vegetable oils, also known as polyunsaturated oils. Unfortunately, these oils are not stable and are prone to oxidation. These commercial vegetable oils are a recent addition to our diet since World War II, when manufacturers developed a process to make them shelf stable by using hydrogenation. Hydrogenating, or partially hydrogenating these oils, also makes them more solid (mimicking saturated fats) and useful for baking and deep-frying.

Research now shows that the processing of these polyunsaturated oils creates a whole new subclass called *trans-fatty acids*. These trans-fatty acids aren't found in nature and are very toxic. Studies are now showing that trans-fatty acids are linked to cardiovascular disease, diabetes, and cancer, among others. What are the polyunsaturated commercially processed in the United States-containing trans-fatty acids? Soy, corn, cottonseed, and safflower are the most common. Ninety percent of all margarines in the United States today are made from soy oil and are loaded with trans-fatty acids.

One reason why saturated coconut oil has a long history of use in traditional cultures— it does not easily oxidize (turn rancid). Virgin coconut oil will not go rancid at room temperature in the tropics for over two years. Conversely, the refined oils that many Americans use are very unstable and can turn rancid very rapidly.

In addition, saturated fatty acids constitute at least 50 percent of cell membranes. They give the cells necessary firmness and integrity. They play a vital role in the health of our bones. They lower a substance in our blood that indicates proneness to heart disease. They protect the liver from the toxic effects of alcohol and certain drugs. They enhance the

immune system. They are needed for the proper utilization of essential fatty acids. Elongated omega-3 fatty acids are better retained in the tissues, when the diet is rich in saturated fats. Short and medium-chain saturated fatty acids have important antimicrobial properties. They protect us against harmful microorganisms in the digestive tract.

In addition, there is a thyroid-stimulating, anti-aging effect in coconut oil. The cholesterol-lowering properties of coconut oil are a direct result of its ability to stimulate thyroid function. In the presence of adequate thyroid hormone, cholesterol (specifically LDL cholesterol) is converted by enzymatic processes to the vitally necessary anti-aging steroids, pregnenolone, progesterone, and DHEA. These substances are required to prevent heart disease, senility, obesity, cancer, and other diseases associated with aging and chronic degenerative diseases.

The research for this information came from Ray Peat, PhD. And Mary Enig, PhD and if you click on: www.coconutoil.com you will find a list of references available.

How can you best utilize this valuable oil? I would suggest using it raw, unheated. Twice a day you can use a teaspoon in a blender drink or add it to your homemade salad dressing. You will see the word “virgin”— meaning not heated. I checked out different brands, and they are not all processed the same way. If you are interested in a specific brand, ask me to check it out. Pure Planet and Jarrow are two brands that checked out well.

Coconut oil solidifies at temperatures under seventy-six degrees. At temperatures over seventy-six degrees, it becomes liquid.

I use coconut oil on my face and skin daily. It nourishes the skin... no chemicals, whatsoever. I drink light coconut milk in my smoothie every morning and love the taste.

Coconuts have been used by cooks for thousands of years with good results. Speaking of cooks, check out coconut cookbooks on Google Search. Bruce Fife, ND has one called *Cooking with Coconut Flour* and another on *The Miracle of Coconut Oil*, so add virgin coconut oil to your dietary program and it could prove to be a miracle food for you too.

Healthfully Yours,
Barbara Charis

*Charis, Barbara; “Virgin Coconut Oil: A True Miracle Food”; July 21, 2010; retrieved March 2015 from <http://www.charisholisticcenter.com/?s=coconut+oil>.

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GLOSSARY

(ADLs) activities of daily living. Not limited to mobility, self-care, and continence, but includes housework, shopping, and leisure activities provided to the care receiver. Such responsibilities can be performed in the home or in long-term care (Author).

Aggression and dementia. Observable aggressive language and/or behaviors by someone living with a dementia. This requires specialized training for care providers to prevent emotional and physical harm to others.

Alzheimer's/dementia coach (also known as A/DC). A specialty in coaching. It is defined as a professional coach, often possessing a life coach training certificate prior to the A/DC certificate, who has successfully completed a coach training program in this specialty. The A/DC is considered front and center of any dementia care program because of the extensive training in most areas of dementia care (Author).

Alzheimer's coaching. Defined as a coaching specialty/ niche dealing with many aspects of coaching (management coaching, life coaching, business coaching, team coaching, training, team formation, teamwork, and Alzheimer's café,

to name a few). Training to become an Alzheimer's coach requires a basic coach formation training accredited by the International Coach Federation in addition to a specialized Alzheimer's coach training program (Author).

Alzheimer's type. Defined as a type of dementia that accounts for an estimated 60 to 80 percent all types of dementia (Alzheimer's Association).

Assisted living. Refers to housing designed for persons who require some assistance with nursing care, housekeeping, meal preparation, and activities. Today's assisted living facilities accept individuals living with dementia who may require assistance with mobility, incontinence or other life challenges (for an extra hourly fee). Such extra fees can add up quickly and therefore an individualized plan needs to be considered in each and every case (Author).

Bereavement stage. Refers to the grief a person experiences when there is a great loss. It is a human emotion brought on by the diagnosis of Alzheimer's. At the physical level, there is real pain, sadness, depression, and much grief. At the spiritual level, it is an opportunity to learn and discover; to strengthen our nurturing abilities; and to find joy and satisfaction in making a difference in another person's life (Author).

Care of Alzheimer's. Refers to the skills needed to provide such specialized care to individuals diagnosed with Alzheimer's. Without the skills care is not only difficult but impossible. Care receivers suffer with poor care and may be driven to total frustration with their care providers to the point of becoming combative (Author).

Career coaching. Refers to all aspects of work from résumé writing to interview practice and exploring all options to fulfill career aspirations (Author).

Cause of Alzheimer's. Refers to the source of the particular Alzheimer's-type diagnosis. Only vascular dementia has a known cause, can be reversed, and even prevented altogether (Alzheimer's Association).

Command coaching. Relates to the coach taking control of the coaching session and commanding steps to enable the client to shift. It enables the coach to establish safety and disciplined parameters. An example is in sports or weight control (grassrootscoaching.com).

Cure of Alzheimer's. Refers to a cure for Alzheimer's. Since there is no cause, there is no cure. Research continues but is rather hopeless at the moment for anyone living with a diagnosis of Alzheimer's (Author).

Dementia. Umbrella term for memory loss and memory problems. The largest form of dementia is Alzheimer's, followed by vascular dementia. However, there are well over one hundred different types of dementia under this umbrella (Author).

Dementia care. Refers to skills needed to address the specialized care and services for someone living with dementia. Some of the skills needed is training in speaking Alzheimer's, types of dementias and symptoms to look for, knowledge of brain changes, and personality of individuals living with dementia (Author).

Dementia care program for home care. Refers to the adoption of systems to simplify care in order to minimize exhaustion on the part of the care provider. Familiarity with programs

such as *Coach Broyles' Playbook for Alzheimer's Caregivers* and the stages of Alzheimer's to anticipate changes that are predictable but progressive (Author).

Dementia care program for long-term care. Refers to dementia care specific training for professionals who work or serve this population. Examples are administrators, dementia unit managers, nurses, social workers, physicians, corporate trainers, patient advocates, geriatric workers, activity directors and assistants, etc. (Author).

Early-onset Alzheimer's. Defined as a diagnosis given to someone who is less than sixty-five years of age (Alzheimer's Association).

Family caregiver or carer. Defined as the individual who provides most of the day-to-day care for family members living with dementia or some other chronic condition requiring assistance with some or all activities of daily living such as personal hygiene, dressing, feeding, activities, transportation, and such (Author).

Formal care. Care provided by an individual who is trained in their field of expertise. This care can be provided in the home but is mostly provided in hospitals and long-term care by professional healthcare workers such as physicians, nurses, aides, and social workers (Author).

Formal care provider. Defined as an individual who is professionally trained and certified to provide the level of care he/she is providing to a care receiver (Author).

Healthcare leaders and executives. Defined as the chief executive officer at a hospital, the chief nursing officer at a hospital, the administrator at a nursing center, and the manager at an assisted living facility. There may be other

classifications not listed here but the individuals must be leading the organization or responsible for a large interprofessional healthcare workforce (Author).

Home care. Refers to care provided in the home and also as domiciliary care. Social care that is designed to support the needs of the care receiver while living at home. Such care is provided by a family caregiver who may be assisted by professional care attendants or healthcare professionals hired by the family to provide the best possible care for the person living with Alzheimer's. The family caregiver is not on a payroll and often must leave their full-time employment, benefits, and lifework to stay at home with the person living with Alzheimer's (Author).

Informal care provider. Care provided by an individual who is untrained in the care they are providing but may have had to learn on the job or self-study, experience, or workshop training. This care can be provided in the home or in long-term care by a family member or by someone the family may have hired for a specific task such as feeding the care receiver or assisting with activities of daily living (Author).

Inter-professionality. Defined as a group of individuals from different disciplines working and communicating with each other. In the interprofessional learning (and working) environment, each member provides his/her knowledge, skills, and attitudes to augment and support the contributions of others (Hall and Weaver, 2001, from <https://www.ttuhsce.edu/qep/teamwork.aspx>).

Interprofessional teamwork. Defined as a collaborative interaction among interprofessional team members to provide quality, individualized care for patients (IOM, 2003, from <https://www.ttuhsce.edu/qep/teamwork.aspx>). In the

context of Alzheimer's/dementia coaching, such teamwork includes the primary caregiver or pack leader (Author).

Intervention coaching. A specialty in coaching ideal to transform individuals, couples, businesses, and organizations. It is ideal in career coaching and helping clients help themselves to change (Anthony Robbins).

Laser coaching. A method of coaching individuals using metaphors followed by coaching questions that are aimed at the core of the matter. It is a method invented by Thomas Leonard, founder of Coach U. Mr. Leonard was a genius, a wizard, and has been referred to as the father of coaching (Thomas Leonard).

Life coaching. Defined as a synergistic relationship between a client and a coach that help in major transition phases of life. Life coaching is the largest of the many specialties in the coach industry (Author).

Long-term care. Refers to care services and support of activities of daily living over a long period of time. This can be in the home but is often used in the context of a care facility. When the primary care provider is no longer able to provide such long-term care in the home, the care receiver must go into a care home (Author).

Long-term health insurance plan. Refers to an insurance plan designed to cover long-term services and support that may apply for home care or long-term care facility care. Such an insurance plan generally covers the costs of long-term care thereby offering flexibility of services and place of services for the care receiver. The policy generally will pay a daily or monthly rate of benefits. It may protect and secure a healthy financial future for any long-term medical condition (Author).

Metaphor. Refers to a figure of speech in which a word or phrase is applied to an object or action to which it is not literally applicable (e.g., Living with Alzheimer's is like being afloat in the middle of a raging sea without hope of finding the shore (Author).

Metaphor in Alzheimer's coaching. Refers to a tool in coaching to give insight into the client's perception of their situation and goals. In *Man and his Symbols*, chapter 1, Carl Jung wrote about the point we reach which conscious knowledge cannot go beyond. The use of metaphors form symbols and images to help the client move past his/her unconscious wisdom. This allows us to have a more comprehensive understanding of ourselves and others (Author).

Metaphor of symbiosis with Alzheimer's. Refers to the symbiotic relationship in the relationship of a dementia couple, their social identity as a couple, and also the changing social identity of the two individuals within that symbiotic relationship. The movement is from mutualistic symbiosis, to commensal symbiosis, and ultimately leading to parasitic symbiosis which is a toxic, dependent symbiosis (parasitical symbiosis) resulting in stress in both parties but mostly for the caregiver who may suffer serious emotional issues. Such a symbiosis threatens the social identity and personal identity of both (Carolyn M. Gallogly).

Professional care provider (see formal care).

Pack leader. A term by the author and defined as the individual who leads in all areas of care and treatment of the care receiver. This may include selecting the treating doctor, team members, and methods of treatment as part of the larger interprofessional healthcare team for that individual. The individual has the power of attorney and perhaps

guardianship of the care receiver, but previously started as the primary care provider and therefore is familiar with a variety of likes and dislikes of this individual for which care is provided (Author).

Primary caregiver or carer. Defined as the individual, usually a family member, responsible to provide the primary day-to-day care of the care receiver. This individual will possess a power of attorney and/or a guardianship and has the power to make a medical decision for the well-being of the care receiver (Author).

Residential care homes. Refers to a small, more homelike family setting where an individual living with Alzheimer's receives dementia care. These homes are also referred to as personal care homes. Care provided to a small number of persons living with Alzheimer's (usually six to eight individuals) assisted by no more than a ratio of one to four (one professional caregiver to four residents). This form of care may also be provided within the confines of a home designed for this purpose (Author).

Residential memory care units. Refers to locked-down units to prevent residents from accidentally escaping the facility and harming themselves by walking out. Such units are often needed because the facility is understaffed for dementia care. This is one way to house the more combative residents away from other residents and visitors to the facility (Author).

Stages. Refers to a point, period, or step in a process or journey of living or caring for someone living with dementia.

Stages of Alzheimer's. A series of seven stages identified by the Alzheimer's Association as the Global Deterioration Scale (GDS) and also known as the Reisberg Scale. Such stages

are predictable and progressive with Alzheimer's. Stage 1: No impairment, Stage 2: Very mild decline, Stage 3: Mild decline, Stage 4: Moderate decline, Stage 5: Moderately severe decline, Stage 6: Severe decline, and Stage 7: Very severe decline (Alzheimer's Association).

Stages for primary caregiver or pack leader. Refers to the progression of stages in providing dementia care. There are five stages: early stage, middle stage, late stage, bereavement stage, and rehabilitation stage (Author).

Stages of a formal care provider. Refers to the stages of professionally trained care providers that specialize in dementia care. There are four stages: early stage, middle stage, late stage, and bereavement stage (Author).

Symbiosis. Classified into three forms: mutualism, commensalism, and parasitism. The interactions may have positive, neutral, or harmful results (wizznotes.com).

Symbiosis in dementia care. Refers to the process beginning with mutualistic symbiosis, moving to commensal symbiosis, and ending with parasitic symbiosis or relationship which is a toxic dependent symbiosis resulting in changes at a personal and social level (Carolyn M. Gallogly). In the rehabilitation stage of caregiving for the care provider, such symbiosis must be addressed. It is necessary to successfully reenter society and heal from the caregiving journey (Author).

Teams. Defined as healthcare professionals (formal and information for the purpose of Alzheimer's coaching) working interdependently to achieve specified, shared objectives. The achievement of the shared objectives required team members to share multiple sources of information, systematic communication, coordination, and cooperation

(Salas, DiazGranados, Weaver, and King, 2008, from <https://www.ttuhsc.edu/qep/teamwork.aspx>).

Teamwork. Defined as the interaction and relationships between two or more professionals (formal or informal) who work interdependently to provide safe, quality care. Teamwork includes the interrelated set of specific knowledge (cognitive competencies), skills (affective competencies), and attitudes (behavioral competencies) required for an interprofessional team to function as a unit (Salas, DiazGranados, Weaver, and King, 2008, from <https://www.ttuhsc.edu/qep/teamwork.aspx>).

Therapeutic coaching. Defined as a model of coaching that draws from several disciplines such as sociology, psychology, human development, career counseling, and mentoring (Mary Beth Tracy).

Workforce. Defined as the people engaged in or available for work, either in a country or area or in a particular company or industry (Business Dictionary).

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